Women's Healthcare Associates LLC

WHA CAD 100

TRAINING MANUAL

Patient Services
Women's Healthcare Associates

Table of Contents

Training Checklists 6

Notes 13

Notes 14

Notes 15

Notes 16

Epic Fundamentals 17

Appointment Desk 19

Registration 22

Registration – Checklist Settings 23

Scripting on How to Ask Registration Questions 25

Registration – Demographics 27

Televox Text Reminder 30

Permanent Comments 31

Registration – Adding a PCP 33

Registration – Adding WHA Care Team Members 34

Registration – Adding an Emergency Contact 37

Registration – Adding Employer Info 38

Registration – Adding a Guarantor/Insurance 39

Registration – Editing Established Patient Information 47

Patient FYI's on Appt Desk 48

Patient Messages on Appt Desk 50

Registration – New to WHA but already in Epic 51

Patient Chart Advisories 54

Mark Patients for Merge 55

Known Non-Duplicate (KND) 57

Term of Care 58

Term of Care Letters 60

Staff Messages 61

Managing Staff Messages 63

Telephone Encounter and Medication Refill Request 65
Creating Smart Phrase Speed Buttons 70
OB / GYN Scheduling 71
Ultrasound Guide 72
Traditional OB Timeline 73
Pregnancy Wheel 75
Scheduling – Basics 76
Solutions View for Scheduling (*Auto-Search*) 78
Schedules View for Scheduling (*Manual Search*) 79
Warnings for Appt Release during Scheduling 81
Appt Review / Completing the Scheduling Process 82
Chaperone Info & Scripting for Schedulers 83
Cancel vs. Reschedule 85
Appointment Warning Overrule Reasons 86
Using the Change Appointment function on the Appt Desk 87
Searching Across Multiple WHA Depts 89
Scheduling - Appointment Notes 90
Scheduling – Depo Injections 93
Scheduling – Est Wellness with Problem visit type 94
Requirements for the Est Wellness with Problem 97
Scheduling – Est Office Visits 99
Scheduling – Advanced Visit Types 102
Scheduling – Visits with a Procedure Room 106
Scheduling – Visits with Pools 110
Scheduling – Series of Appointments 112
Scheduling – NST Visits 114
Scheduling – Visit Types with Subgroups 115
Scheduling – From an Active Request 116
Scheduling – Manually Linking Active Requests to a scheduled visit 117
Scheduling – Linking and Unlinking Appointments 118
ER Follow Ups 119

Referrals - Where to find Active Referrals on file 122
Referrals - Reading notes within a Referral 123
Referrals - Assigning a Referral and FAQs 125
Prescription Benefit Verification 127
List of Visit Types 129

Training Checklists

Name: Start Date: Date Completed:

CAD 100 Central Training Checklist

Segment 1	
WHA and Provider Overview	Trained
☐ **Clinic Overview - type, location, & associated Hospital**	
☐ **Types of Roles within the clinic**	
• MDs, CNM's, NP's, Behaviorists	
• MA, LPN, Med. Rec., PSR etc.	
• Clinic Supervisor Roles	

Segment 2	
Sharepoint Overview	Trained
☐ **Most common icons on the left and upper toolbars**	
☐ **Location of Patient Services tip sheets on Sharepoint**	

Segment 3	
Epic Fundamental	Trained
☐ **Hyperspace Toolbar - quick access to frequently used activities**	
☐ **Editing Hyperspace Toolbar**	
☐ **Demonstrate how to use the Search bar**	
☐ **Home Workspace Tabs - "hover to discover"**	
☐ **Activity Toolbar - specific to the active workspace**	
☐ **EPIC Button - access additional activities and pin recent**	

Segment 4	
Registration	Trained
☐ **Appointment Desk Overview**	
•Storyboard, Interactive Facesheet, Sidebar, Patient Msgs/FYIs	
☐ **Registration Overview**	
•Navigation Pane and Checklist	
☐ **Register New Patient**	
•How to search and enter by Name, DOB, Street Address	
•Demographics - Legal name, preferred name, aliases, phonetic name, patient pronouns, address, phone, text opt in/out, e-mail, MyHealth sign-up	
•Census Questions - Language, interpreter, written language, marital status, religion, ethnicity, race, country of birth	
•Adding PCP and Care Team member	
•Adding an Emergency Contact	
•Employer Info - employer name and employment status	
•Guarantor Account - Guarantor account overview, search for an existing account, create new account	
•What to do if patient is not the Guarantor	
•Coverage - Adding new insurance, adding subscriber, using the awaiting response tool	
•Verify Checklist - goal is the big green checkmark	

- ☐ **Entering/Editing Patient Message Options**
  - •FYI's
  - •Patient Messages
- ☐ **Register the Minor of a Registered Patient**
  - •When Guardian is Emergency Contact and Guarantor
  - •Adding coverage that is attached to the Guarantor
- ☐ **OHP & Self-Pay Patients / Insurance Referrals**
- ☐ **Adding a Secondary Insurance**
- ☐ **Register New Patient to WHA but <u>already in EPIC</u>**
  - •How to tell if they are a WHA patient (Patient Sidebar - no Guarantor Account
  - •Verify that all Registration Info. Is up to date
  - •Utilize Coverage Copy if possible or add insurance

Segment 5

<u>Messaging within EPIC</u> Trained

- ☐ **Staff Message**
  - •Sending a new Staff Message
  - •Sending future Staff Messages
  - •Managing In-Baskets
  - •Notifying our FC team of Termed Insurances
- ☐ **Telephone Encounter**
  - •Medication Refill - Utilizing Smart Phrase
  - •Utilizing Telephone Call Button on Hyperspace Toolbar
  - •How to create Speed Buttons & Smart Phrases (NOB & Rx Refill)
  - •Using F3 to enlarge encounter Documentation box
  - •Using F2 to "move through" encouter
  - •Contacts, Reason For Call, Routing, Documentation

SCHEDULING TRAINING

<u>OBSTETRICS CARE</u> Trained

- ☐ **OB Scheduling Overview**
  - •Traditional
  - •Review Ultrasound Guide
  - •Genetic Testing - NIPT Scheduling Date
  - •Global Billing
  - •Postpartum Scheduling
- ☐ **OB Intake Documentation**
  - •Creating new Telephone Encounter
  - •Using Smart Button to fill in questions
  - •Verifying Care Team
  - •Discuss NOB Workflow
- ☐ **OB Scheduling**
  - •Date Range
  - •How to find EDD & GA

- •OB Workflow
- •Discuss Options Counseling

☐	**Pregnancy Wheel**	
☐	**Scripting "Wrap up" topics for Telehealth & In person visits**	

Segment 6

GYNOCOLOGY CARE		Trained
☐	**GYN Scheduling Overview**	
	•Office Visit, Wellness Visit, Telehealth visits, Mammogram, IUD, BH, Procedures, Birth Control types	
	•Utilizing Scheduling Timeframe Guide	
	•Medicare GYN patient scheduling	
☐	**Schedule Injection**	
☐	**Schedule Established Office Visit**	
☐	**Schedule New Patient Office Visit**	
☐	**Schedule Wellness Visit**	
☐	**Schedule Mammogram with Wellness Visit**	
☐	**Schedule BH Intake**	
☐	**Schedule IUD insertion**	
	•New Patient Procedure Scheduling	
☐	**Schedule Est Patient Procedure: Colposcopy & Leep**	
Canceling/Rescheduling Appointments, Waitlist		
☐	**Cancel Appointment**	
☐	**Reschedule List**	
☐	**Waitlist**	

Segment 7

Wrap-up of Patient Services		Trained
☐	**Review HIPAA 'Business Need'**	
	•Don't access **own personal** chart/info in Epic	
☐	**Review Term of Care patients**	
☐	**Demonstrate using View Schedules**	
☐	**How to utilize the Credentialing Wiki**	
☐	**Marking patients for Merge**	
☐	**Using an Interpreter in our office & on phones**	
☐	**How to find the WHA Dictionary**	
☐	**Late Patient Policy**	

Women's Healthcare Associates LLC

CHECK-IN TRAINING	
Segment 8	
Department Appointment Report (DAR)	Trained
☐ Explination of each column on DAR	
•DAR Icon Review	
•Settings and saving a custom DAR	
Segment 9	
Workflows at the Check-In desk	Trained
☐ Patients with GTT appointments	
☐ Patients using the Restroom prior to appt	
☐ Guest Policy	
☐ PPE for Check-In staff	
☐ Late Patient Policy	
Segment 10	
Check In Process	Trained
☐ Sign In	
•Ask for 3 patient identifiers	
•Review if there are any icons on DAR (such as Appt Note)	
☐ Check In	
☐ Verify Patient Information	
☐ Taking Patient Photo	
☐ Scanning patient's ID and Insurance Cards	
☐ Documents	
•E-Sign	
•Non-E-Sign - including ABN Waivers	
•The importance of collecting Consent form signatures	
☐ Permanent Comments	
•Permission to Communicate Contacts	
•Voicemail Permission Comment	
☐ Collecting Copays	
•The importance of collecting copays	
☐ Collecting Patient Outstanding Balance	
☐ Collecting Pre-Payments	
☐ Status change	
•Present, Arrived, Checked Out	
☐ Patient Label from printer	
☐ Patient Verifying Info / Initial Labels	
☐ Payments outside of Check In	
Segment 11	
End of the day wrap-up for Check-In staff	Trained
☐ Cash Drawer	
•Printing Cash Drawer	
•When and how to close the Cash Drawer	
•Re-Printing Cash Drawer	
☐ User Batch Report	
•When you will use User Batch Report	

Name: Date Completed:

CAD 100 Local Training Checklist

Segment 1

****To Be Trained/Observed/Competent within the *first 10 Days***

Set Up:	Trained	Observed	Competent
☐ Computer Desktop Set-up			
☐ Five9 Settings Set-up (See Sharepoint for guide)			
•Priority Lines			
☐ Scanner (if needed)			
☐ Adding all needed office printers/label printers			
☐ Favorite/Save PhTech/CIM OHP Member Look-up			
☐ Favorite/Save OneHealthPort Insurace Website			

Where to Find:			
☐ Office Supplies			
☐ Copy/Fax Machines/Label printers			
☐ Mail (outgoing and incoming)			
☐ Scanning (HIM)			
☐ Inner Office Mail / Courier Schedule			
☐ Visitor Workflow / Visitor Sticker Badges			

Review with PSS:			
☐ Attendance and sick call procedures			
☐ Break Requirements			
☐ Lunch			
☐ Parking Policy at your clinic			
☐ PTO requests			
☐ Food/cell phone work policy			
☐ Dress code			
☐ Central Park/SharePoint			
☐ MyHR			
☐ TimeStar			
☐ Patient Complaint Process (RL Datix on Sharepoint)			
☐ HIPAA			
•Don't access **own personal** chart/info in Epic			
•Staff who are patients, need to be treated like patients			
☐ Find Teams Schedules (phones and check-in desk)			

Segment 2

To Be Trained within the ***first 10 Days*

To Be Observed within the ***first 30 Days*

To Be Competent within ***the first 60 Days*

Clinic Schedule(s) Review	Trained	Observed	Competent
☐ Providers and Specialties			
☐ Site Schedule(s)			
☐ Reschedules			
☐ Same Day Scheduling			
☐ Scheduling Restrictions			
☐ EPIC "Hold" time slots vs. "Unavailable" time slots			
☐ Call Scheduler on Sharepoint			

Segment 3

To Be Trained/Observed within the ***first 30 Days*

To Be Competent within the ***first 60 Days*

Review with Financial Coordinator or Preceptor:	Trained	Observed	Competent
☐ Types of Coverage (OHP, Medicare, Commercial, etc.)			
☐ Double Coverage			
☐ Self Pay Process (Staff Message FC Team)			
☐ Bad Debt			
☐ Term of Care (financial & causes types)			
☐ OB Billing (global) / OB Benefits			
☐ Collecting Copays			
☐ OHP GYN Auth.			
☐ Referral Workflow			
☐ Notes on the DAR			

Segment 4

To Be Trained within the ***first 10 Days*

To Be Observed within the ***first 30 Days*

To Be Competent within the ***first 60 Days*

Patient Services Workflows:	Trained	Observed	Competent
☐ WHA Wiki			
☐ Scheduling - GYN and OB appts			
☐ Scheduling - Telehealth Visits			
☐ Reschedule List and Waitlists			
☐ Scheduling from an Order			
☐ Reschedule vs Canceling a Visit			
☐ Canceling Triage Appointments Workflow			

☐	Reschedules			
	•HCG Quants Workflow including Advice Nurse Contact			
	•Appt Request Workflow			
☐	Dot phrase list set up			
☐	NOB Workflow			
☐	PP3			
☐	PP8 (wellness visit)			
☐	IUD			
☐	PP8 or Wellness with IUD			
☐	ER Follow Up Workflow			
☐	Surgery Scheduling - Where to send messages			
☐	Surgery Blocks			
☐	US Scheduling (orders, follow up appts, AMA)			
☐	Depo Calendar / Scheduling			
☐	Nexplanon / Finding a provider that does them			
☐	Mammograms w/ and w/o implants			
☐	Patient Instructions for Mammo and US			
☐	Interpreters / Linguava			
☐	Attaching paperwork/forms to visits			
☐	MyHealth- Paying bills, sending msgs & loading ins info			
☐	Check-in Consents / Forms (Forms vs. Visit Types)			
☐	Medicare High Risk Form / Where to find it in chart			

Segment 5

****To Be Trained within the *first 30 Days***

****To Be Observed within the *first 60 Days***

****To Be Competent within the *first 90 Day***

Review with Preceptor/PSS:		Trained	Observed	Competent
☐	Extended Hours			
☐	Televox related to patient care			
☐	Release of Info Form in person and via Myhealth			
☐	FMLA/STD paperwork process - Form Fee			
☐	Returned Mail			
☐	Certified Mail			
☐	Clinic Front/Back Phone Line			
☐	Routing Calls (Advice, PODS, Pools, & Providers)			

Notes

Notes:

Notes

Notes:

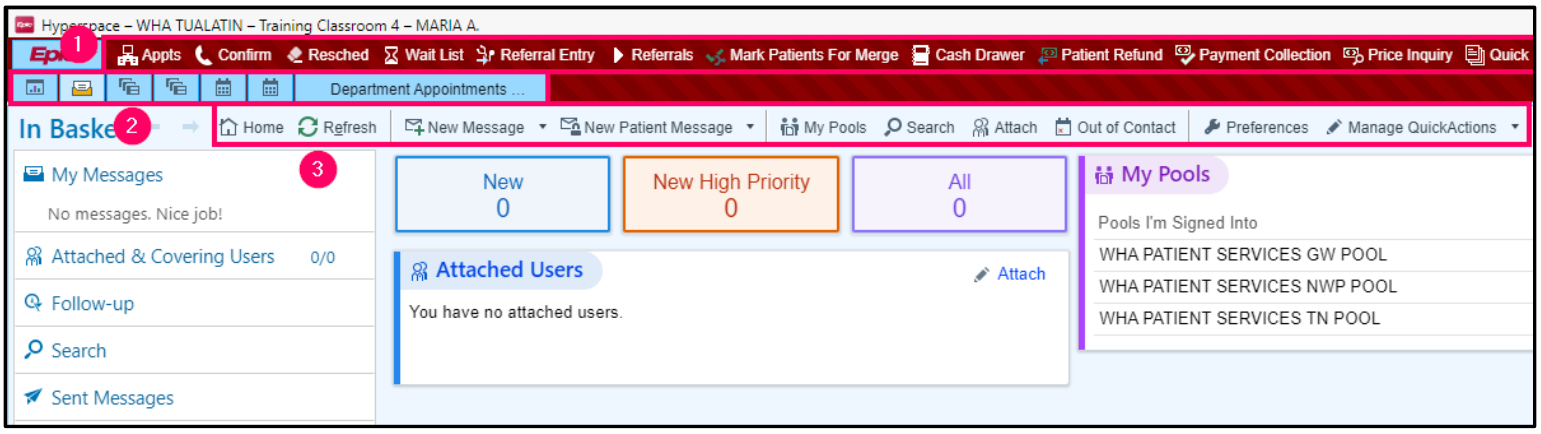
Notes

Notes:

Notes

Notes:

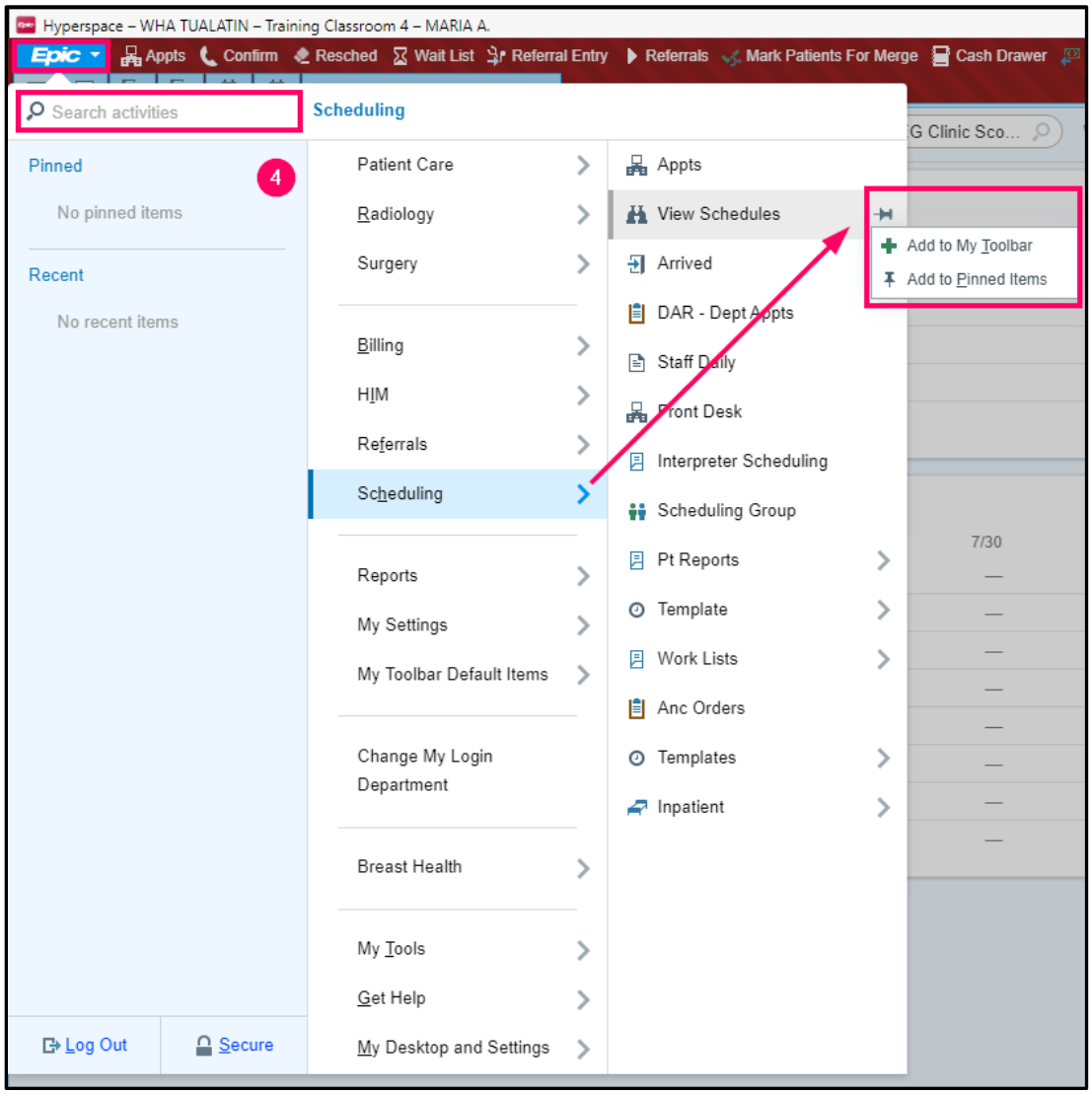
Epic Fundamentals



1. **Hyperspace Toolbar**
   - One-click access to frequently used tasks
   - Customizable
   - The color of this toolbar will vary depending on what environment you are in

2. **Home Workspace tabs**

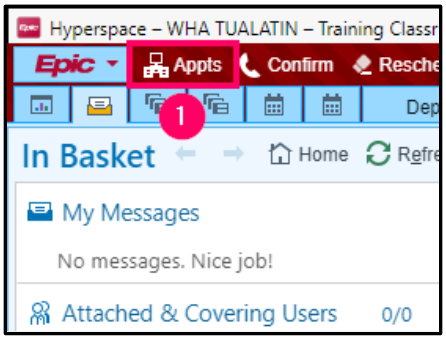
3. **Activity Toolbar**
   - Specific to the workspace you are in

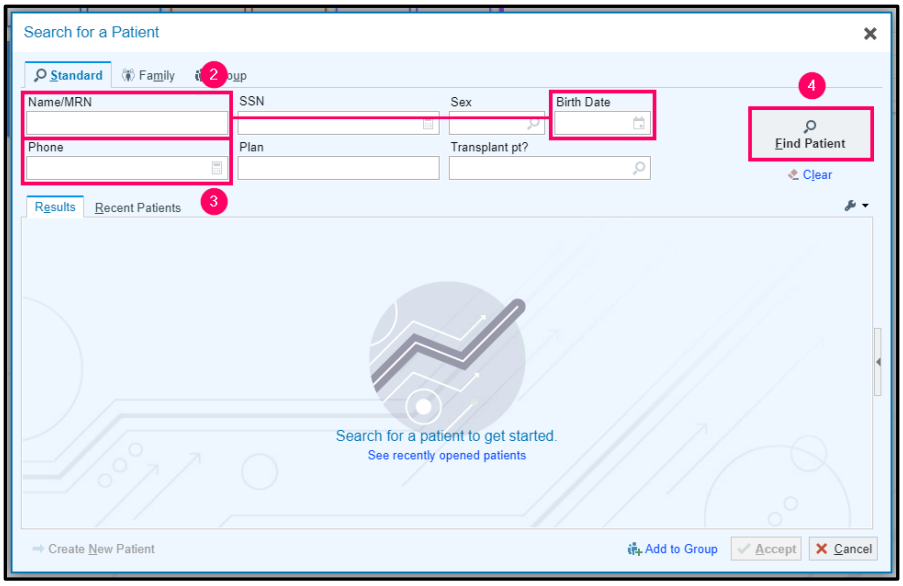


4. **Epic Menu**

- **Search** for activities in Epic
- Add tools to the Hyperspace Toolbar with the + **Add** option or to the Epic dropdown by **pinning** it

Appointment Desk





1. Open the Patient's Appointment Desk by clicking on the **Appts** icon on the Toolbar

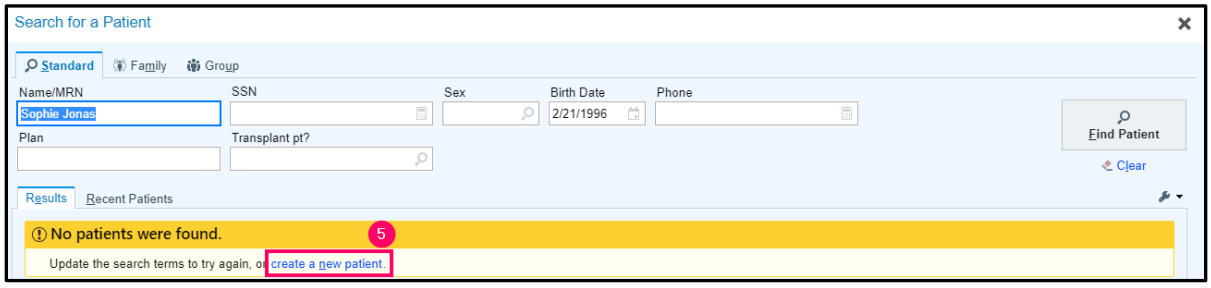
2. To search for a patient, put in the patient's **first and last name** (*first last > last, first > partial name)* and the **Birth Date** (You do not need to add punctuations as long as single digits have a zero ahead of them. *Example: 010286*)

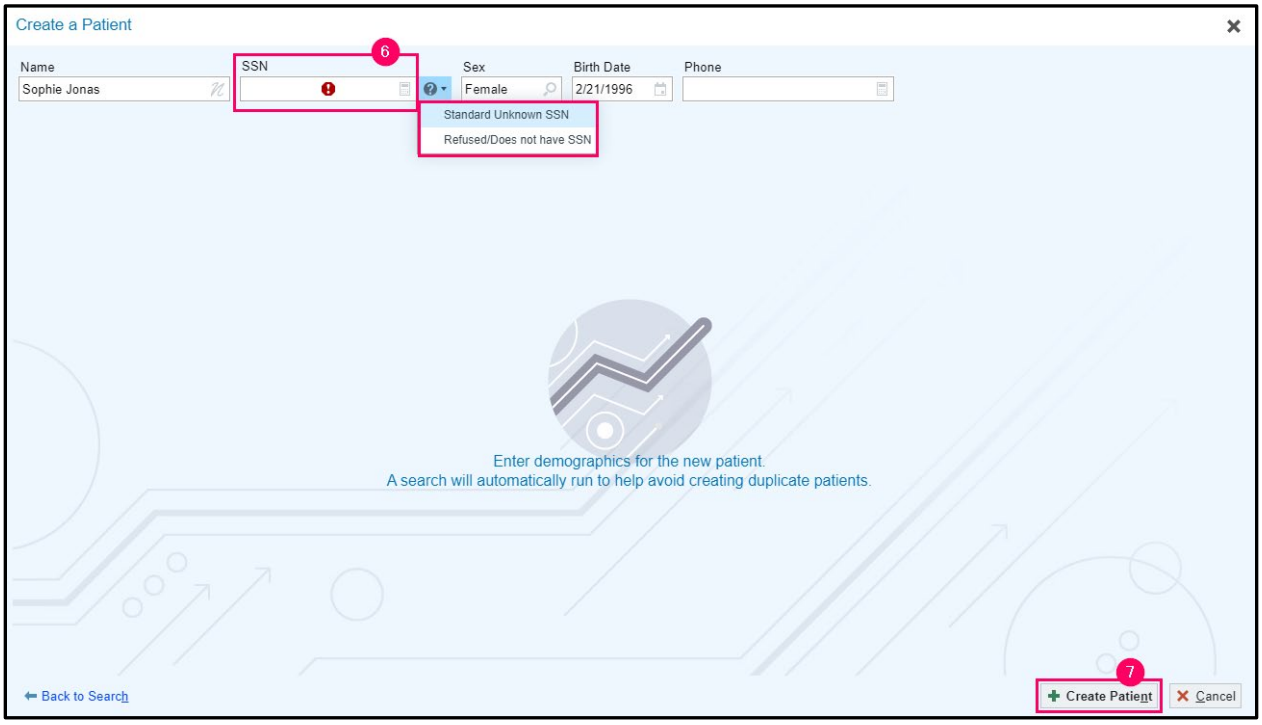
3. You can also search for a patient via their **Phone Number**

4. Click **Find Patient** to start your search

NOTE: Always look up the patient by ***at least three identifiers***

** Notice that you cannot Create a New Patient until you have first searched to see if the patient is in the Epic database. We **do not** want duplicate accounts *(See chapter titled Mark Patients For Merge if you come across a duplicate chart)*

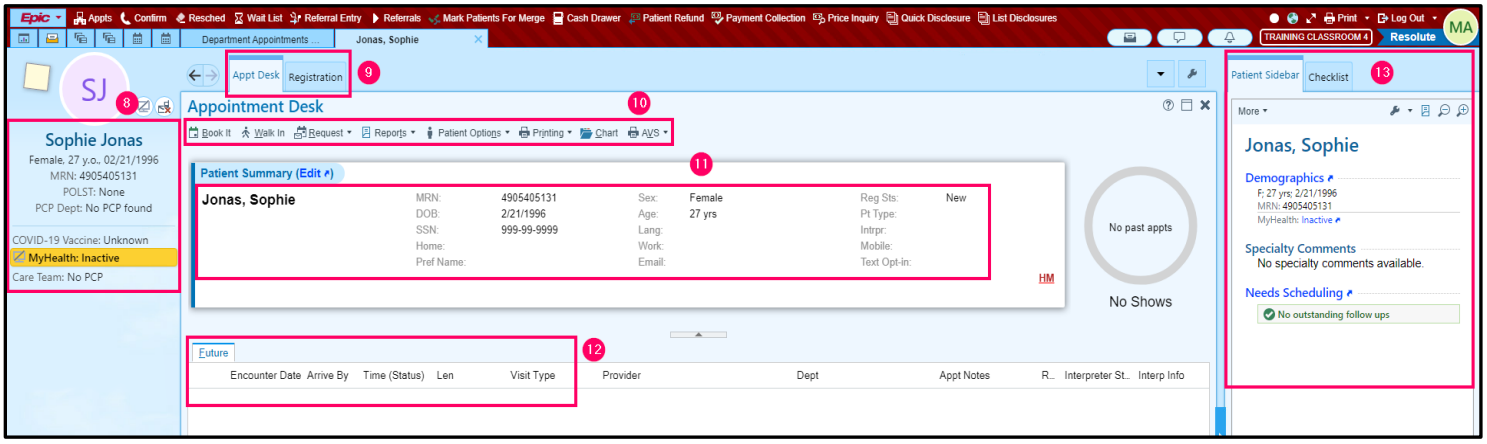




5. Click **Create a New Patient**

6. You will need to enter their SSN (*add Unknown 0's or Decline 9's*) and Sex of patient. Sex should be whatever is listed on their **ID & insurance card profile** due to insurance/billing issues that can occur (*See Page 19 for more info regarding Gender Identity & Pronouns*)

7. Click **Create Patient**



8. **Storyboard** has quick at-a-glance information about the patient (*hover to discover!*)

9. **Activity Tabs** that can be toggled through as needed (*Registration, Financial, etc.*)

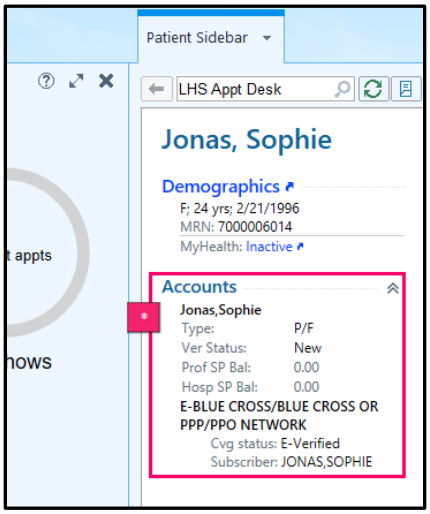
10. **Activity Toolbar** where you'll find activities such as Book It and Registration

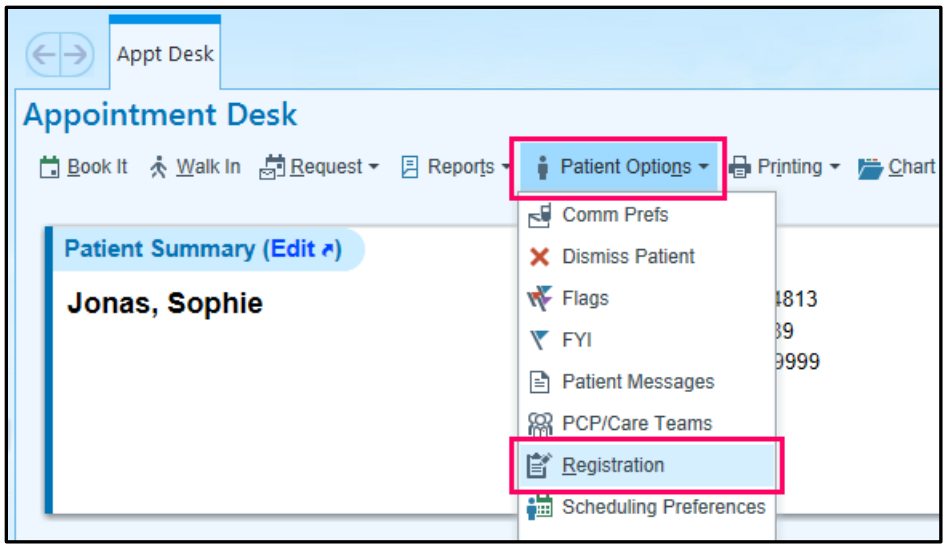
11. Brief patient **Demographic information**

12. **Workspace Tabs** will always show a Future tab (*currently scheduled visits*) but will also show Past (*appts*), Referrals, Active Requests, Wait List, etc. if applicable

13. **Patient Sidebar** will show the same information as Storyboard without needing to hover to expand the info

Registration

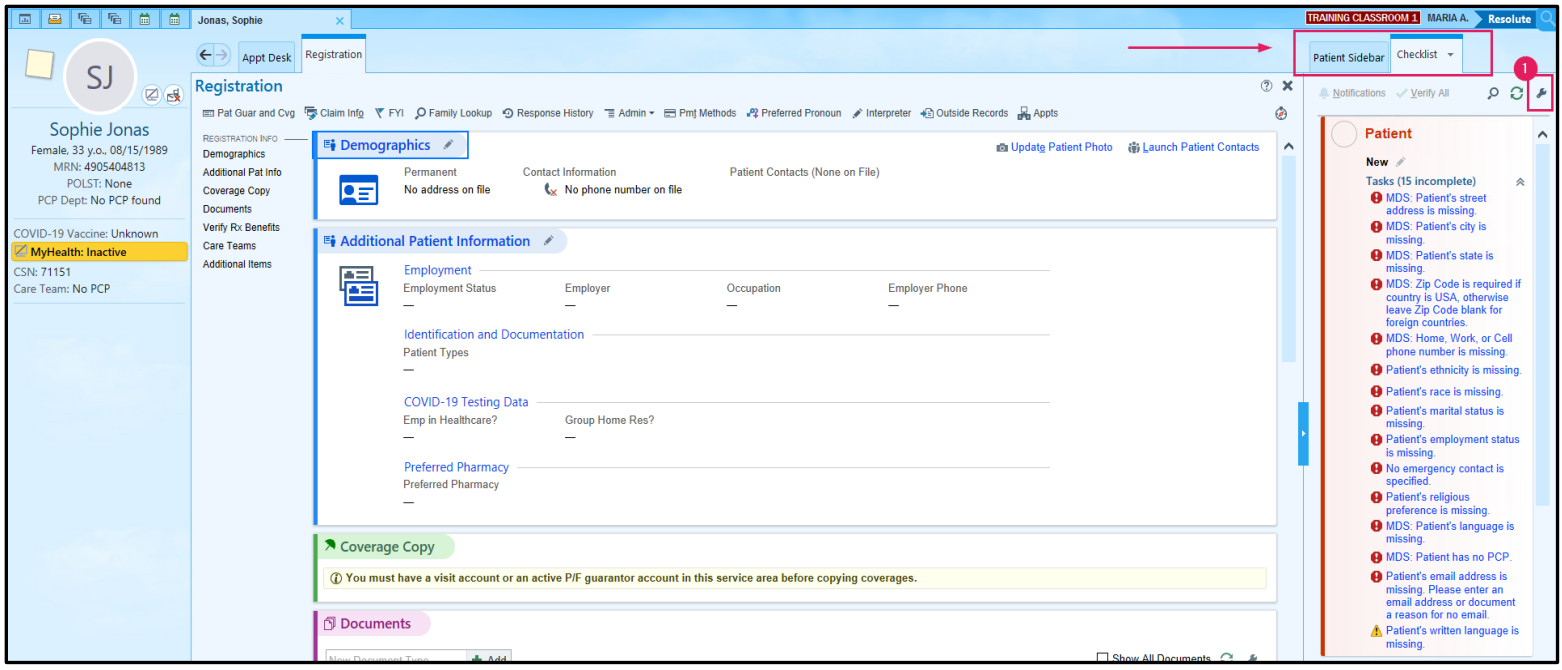




The Patient Sidebar helps verify if there is a Guarantor Account created or not yet for this patient. If they are new to WHA, there most likely will not be one listed here.

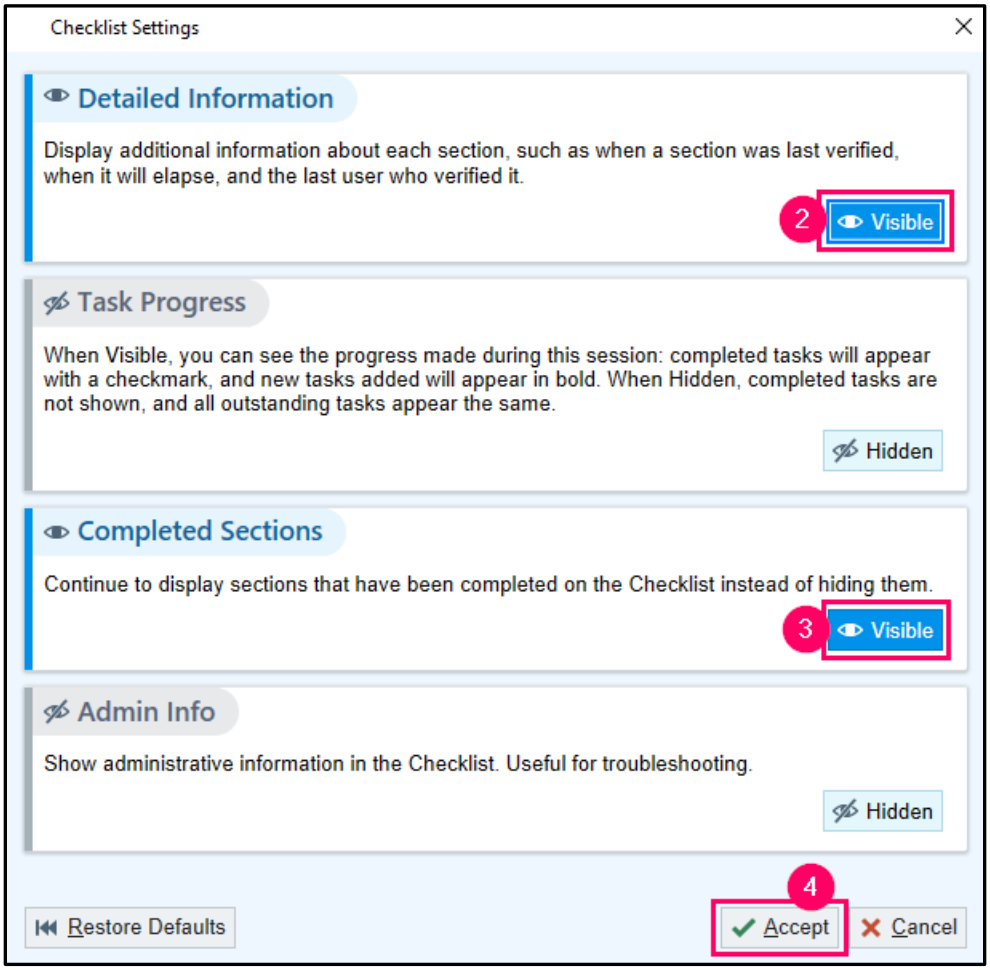
To open the patient's Registration, click on **Patient Options** drop down menu and then choose **Registration**

Registration – Checklist Settings



Each Registration will have a **Checklist** in the sidebar to the right. The Checklist will have warnings and hyperlinks to help each user complete the patient's Registration without missing information.

1. To be able to utilize this Checklist properly, each user will need to edit the Settings the first time logging into Epic (it will save thereafter). To navigate to the Settings screen of the Checklist, click the Wrench in the top right hand corner



2. Click this button to turn it from Hidden to Visible

3. Click this button also to turn it from Hidden to Visible

4. Click **Accept**

Scripting on How to Ask Registration Questions

Patient Demographics/General Information:

- What is the name on your license or state ID?
- What is your date of birth?
- What is your social security number?
- What is the listing of sex or gender on your license or state ID?
- Is there a name you would like us to use for you in our clinic? (a preferred name)
- What is your mailing address? Do you have an Apt or Unit number?
- What is the best phone number to contact you at? Was that a mobile or home phone?
- Do you have a secondary phone number that you would like us to have?
- Would you like to opt-in for Text Appointment Reminders?
- Would you like to provide an email address so we can get you set up with MyHealth, our Patient Portal? On the portal, you can fill out new patient paperwork, pay your bills online, receive lab results and communicate with your provider.

Census Questions:

"I am going to ask you a few census-based questions, you are free to decline if you wish…"

- What is your preferred language (or the language you use in the home)?
  - If preferred language is not English let the patient know:
    "*We offer free certified medical interpretation for all phone calls and medical appointments"*
- What is your primary Written Language?
- What is your marital status?
- Is there a religious preference that you would like for us to respect in our office?
- What Ethnicity and Race do you best identify with?
- What country were you born in?

Employer Info/Patient Relationships/Care Team:

- Are you currently employed or are you a student?
- Do you work full time, part time, self-employed, unemployed or other?
- If employed, what is the company name that you work for?
- Do you have a Primary Care Provider? (Example: someone you would go to if you had a bad sore throat)
- Which provider would you like to see at our clinic?
- Is there an Emergency Contact that I can put on file for you? (does not have to be in Oregon) "We would only reach out to your emergency contact if you had an emergency in our office or if we had an urgent need to get in touch with you and your phone was not working"
    - What is their name?
    - How do you know them/name?
    - What is a good phone number for them/name?
    - Is that a home phone or cell phone?
    - What is their preferred language? And if not English, do they need an interpreter?

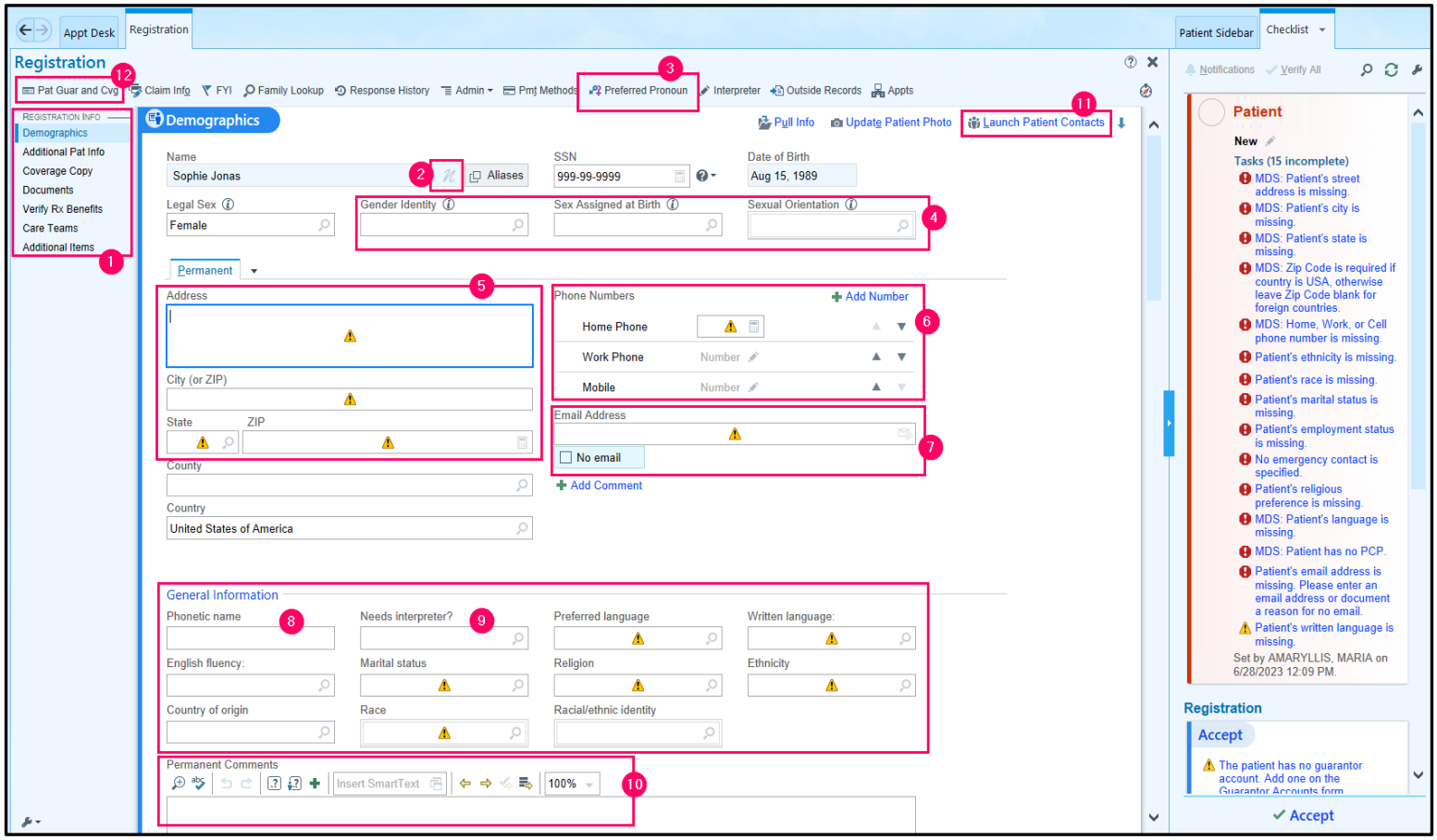
Guarantor:

- Who will be responsible for the bills? – Only ask this if patient is under 18. If the **patient is over 18** and not a dependent adult then you do not need to ask – If patient is 18+, the **patient is always their own guarantor**

Creating Coverage/Insurance:

- What insurance company do you go through?
- Do you know what plan you have with your insurance?
- Are you the main subscriber on the insurance? (Is it through your employer? Or another family member's employer? Who is that main policy holder?)
- What is the ID Number? (May also be called: Subscriber Number, Policy Number, Insured ID Number)
- What is your Group Number?
- Do you have a secondary insurance? (If yes, repeat the questions above)

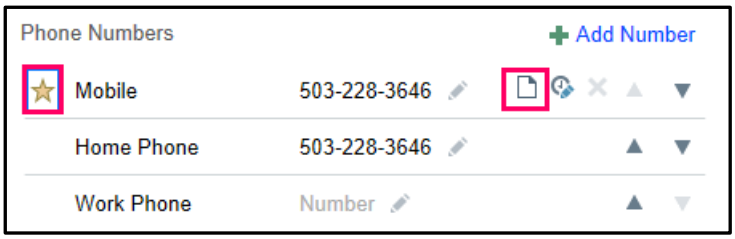
Registration – Demographics



Note: If you are missing information and cannot find out where that field lives, clicking the hyperlink in the checklist will direct you exactly where it needs information filled in

1. This is the Navigator Pane that jumps to different sections easily
2. To adjust the spelling of the patient's name or add a preferred name, click here. If you add a name to the "Preferred name" field, Epic adds it as an "Alias". This allows it to become a searchable name
3. We do not ask the patient their pronouns. If they volunteer that information though, it is good to list it which you can do by clicking this button
4. We do not ask the patient's these questions, but if the patient informs you of certain information that would pertain to these 3 fields then complete them accordingly

5. Add the patient's address with Apt/Unit number on the second line, **DO NOT USE a # sign or any other non-alphabetic character** in the address (this will cause an error in our billing process). Add the zip code under *City (or Zip)* and it will autofill the city, state, zip, county and country. If multiple cities are associated with the Zip Code, it will prompt an additional window to choose the correct city
6. Epic requires a home phone number. If the patient has no home phone, add the cell phone into the "Home Phone" and "Mobile" field. Clicking the **Star icon** next to the phone number identifies that for the front and back office teams that this is the patient's main designated phone number to use. If you list a phone number that is not the patient's directly number (like a spouse or parent) then utilize the paper icon to identify whose it is. *Remember to start all free text notes within the Registration with WHA to help Legacy identify what info we entered.



The next question you will ask is if the patient would like to opt-in for Text appointment reminders. If they say yes, click Add Number and label it as Text Opt-In with the phone number to text in the number field. See the next chapter titled **Televox Text Reminder** for more details.

7. Enter the patient's email address to sign them up for MyHealth (WHA's patient portal). If they decline or do not have an email address, check the "No email" box and fill in the reasoning of why in the newly prompted field
8. For a good patient experience, we phonetically spell out the **patient's preferred name** in this field so that all WHA team members will know in the future how to say the patient's name properly

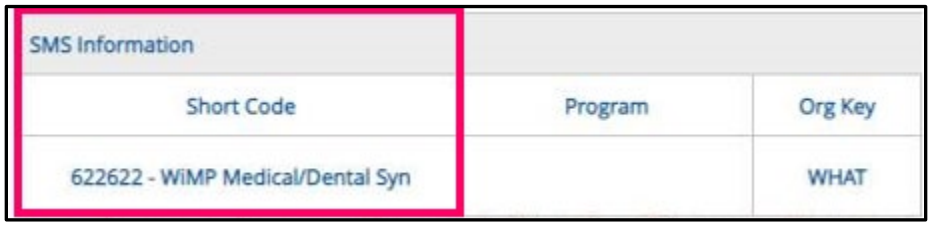
9. These fields are Census questions. Utilize the chapter prior titled **How to ask patient Registration questions** for more details on how to phrase these to the patient
10. The Permanent Comments field is used by all LEAP clinics to document HIPAA notes / voicemail notes (such as Permission to Communicate names, needed one time only verbal patient ok's, or if it is ok to leave a voicemail or not). See the chapter below titled **Permanent Comments**
11. This field will navigate you to the section of the Registration where the patient's Emergency Contact and other important third party contacts will be listed
12. All Guarantor and Insurance information can be opened by clicking this **Pat Guar and Cvg** button *(Patient Guarantor and Coverage)*

Televox Text Reminder

- Below is the timeline of Televox Text Reminder responses.
  When the patient replies to their text reminder:
  - within **2 hours** the response will display in Televox and Epic
  - after **2 hours** the response will only display in Televox
  - after **24 hours** the response will not display in Televox or Epic

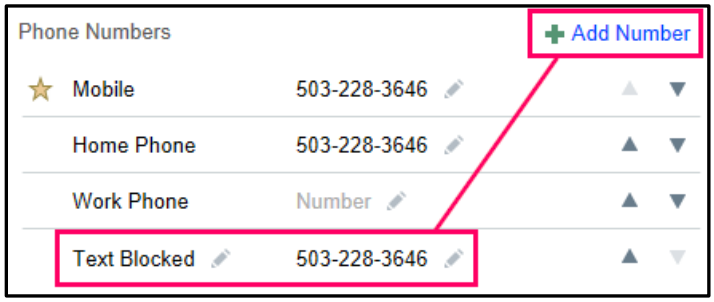
- Participating carriers include T-Mobile, Verizon Wireless, AT&T, Sprint, Boost, U.S. Cellular, MetroPCS, Virgin Mobile, and Cricket.

- Text carrier errors can occur for a couple reasons – if the patient does not have a participating carrier **or** their carrier requires a short code to allow messages.
  - If they do need the Televox's short code to be added to the user's account the patient would need to call their carrier and advise them to add the short code below.



- If the patient wants to **Opt Out** of text message reminders they should reply "**STOP**" to their next text reminder. Then you will need to do the following:
  - Change their "Text Opt-In" number type to "Text Blocked" (*See example)

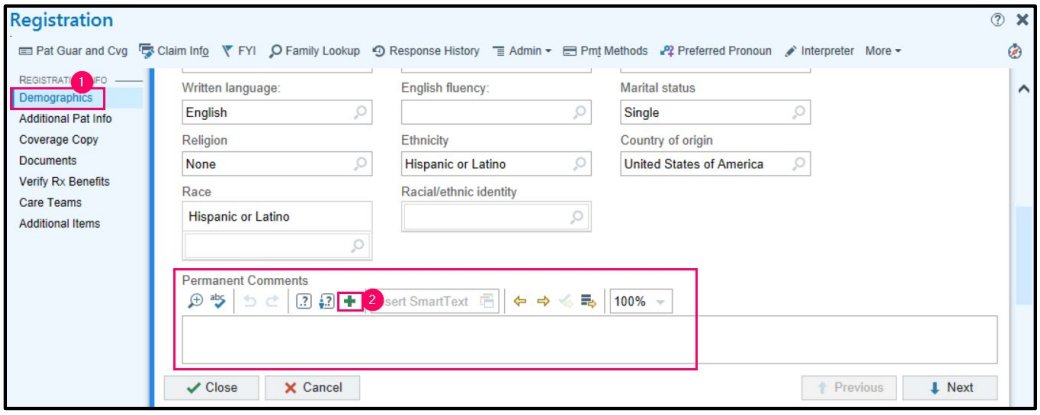
- If they are not receiving any text reminders and the patient requests to be Opted out of text reminders. Then you will need to do the following:
  - Change their "Text Opt-In" number type to "Text Blocked" (*See example)
  - Submit a Helpdesk ticket so Clinical Apps can manually Opt the patient out of text reminders in Televox.



Permanent Comments

The Permanent Comments box is used by all LEAP clinics to document HIPAA notes / voicemail notes (such as Permission to Communicate names, needed one time only verbal patient ok's, or if it is ok to leave a voicemail or not). This box lives under the Demographics section within the Registration.

Adding and Using Smartphrases within Permanent Comments for Standardization:



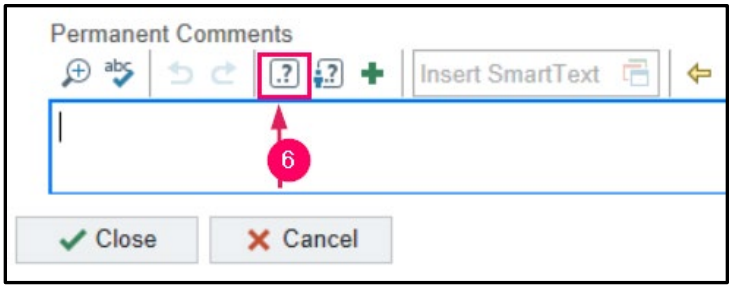


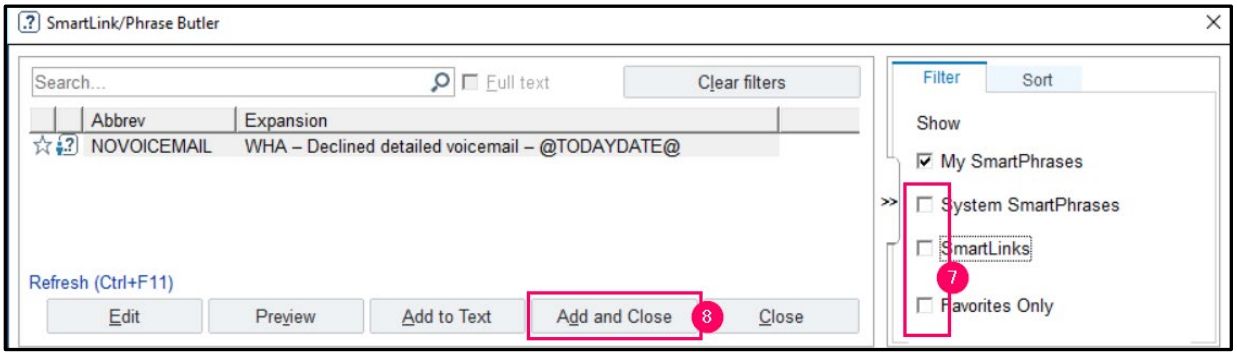
1. Locate the Permanent Comments box within the patient's Registration
2. Click on the Green + icon to add a Smartphrase as a custom favorite
3. For each creation, add one Smartphrase at a time. The 4 Smartphrases we have are:

   .wharoi .whanoroi .whavm .whanovm

4. Name your custom Smartphrase (no spaces or dashes)

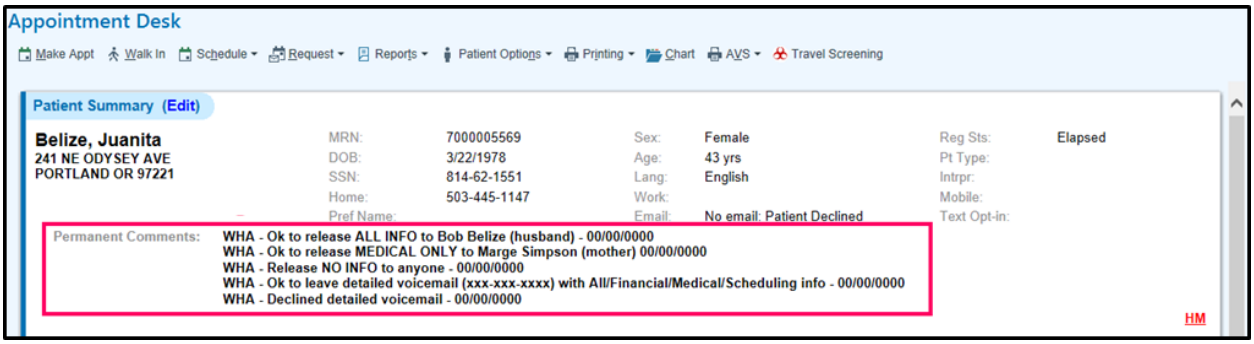




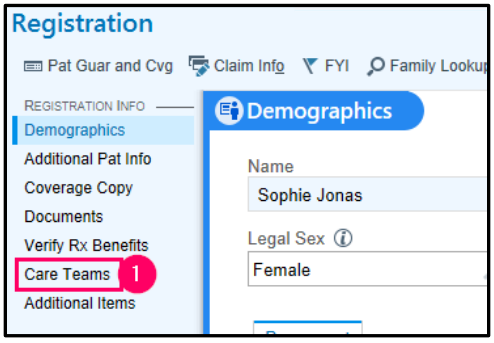


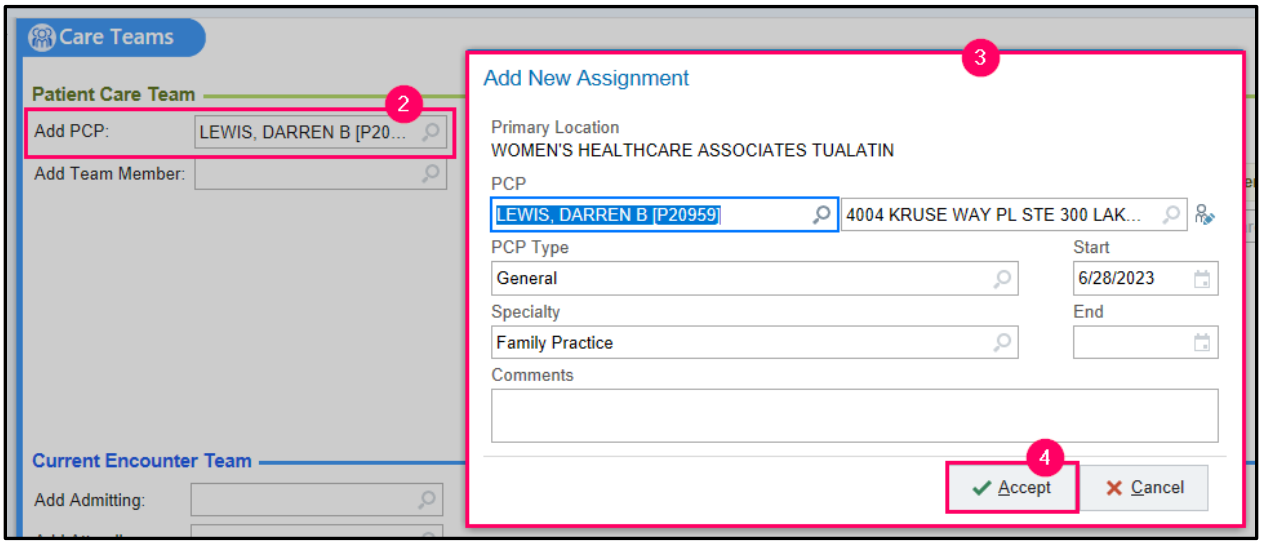
5. Click **Accept**
6. Click on this first **question mark** icon (utilize this icon when needing to pull in the Smartphrases you have added)
7. Uncheck these 3 boxes
8. Click **Add and Close**

View on the Appt Desk of possible Permission to Communicate options:



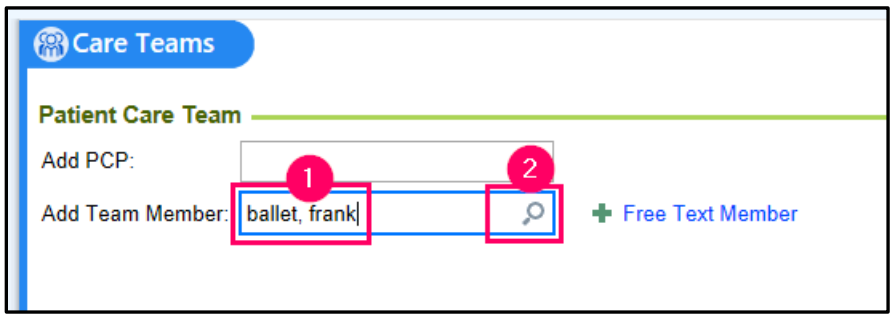
Registration – Adding a PCP



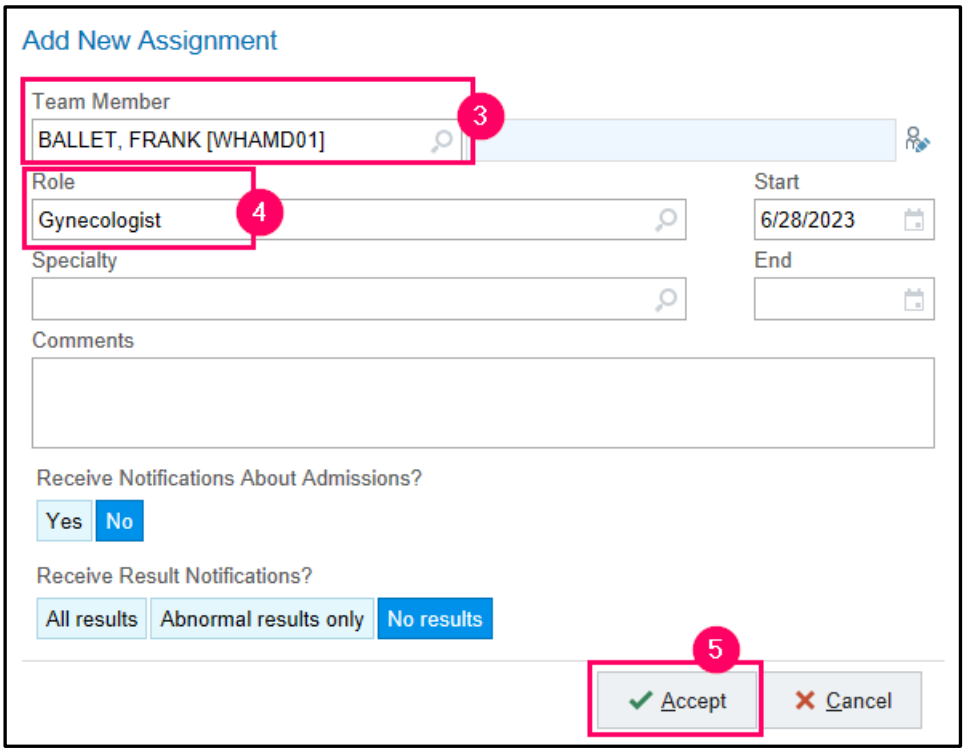


1. Click **Add** to add a Primary Care Provider and Responsible Provider
2. Pattern match the provider's name and click **Add**
3. Click on the correct provider
4. Click **Accept**. DO NOT change any field under Notifications

Registration – Adding WHA Care Team Members



1. Enter the name of the Care Team member that the patient would like to set as their provider
2. Click the **magnifying glass to search**



3. Click on the name of the Care Team Provider

4. Enter the **Role** of the Care Team Provider using the following guide:

a. **Obstetrician/Gynecologist** (MD, CNM, NP) – is when a patient has selected one provider at WHA for both OB and GYN care

b. **Obstetrician** (MD, CNM) – is when a patient has selected a provider to care for them during pregnancy

c. **Gynecologist** (MD, CNM, NP, PA) – is when a patient has selected a provider to care for their gynecology needs

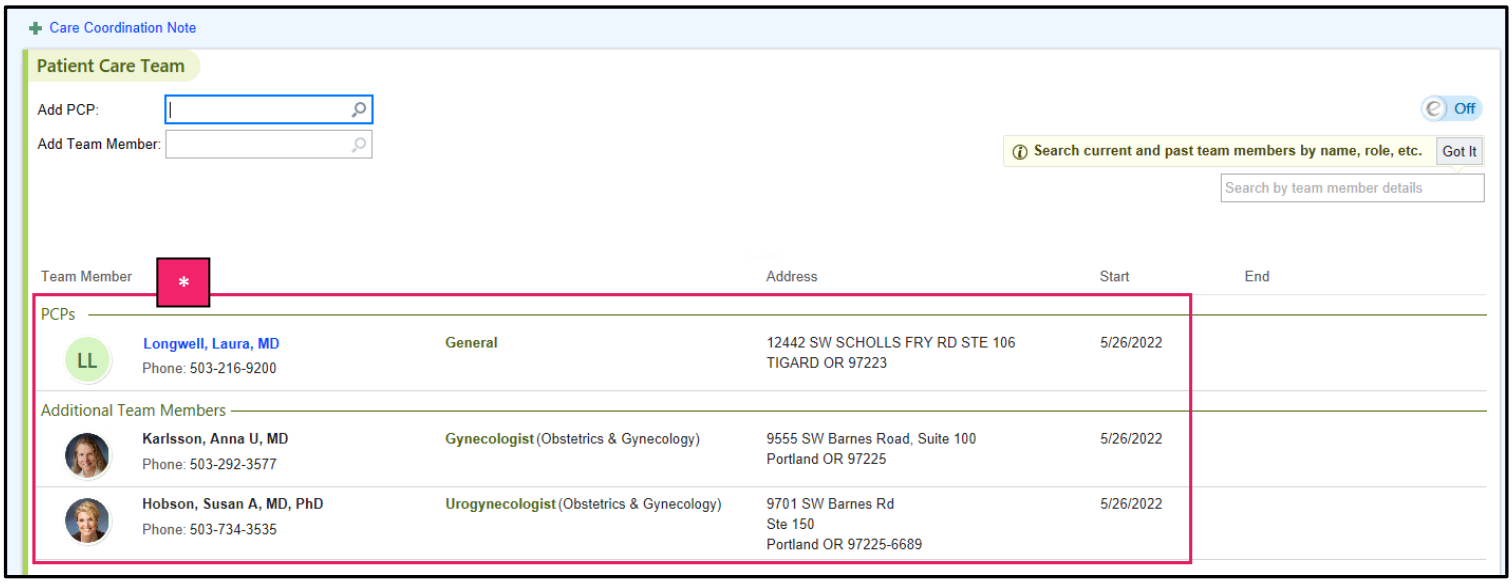
d. Maternal Fetal Specialist (MD) – is when a patient has selected or is being cared for by our MFM physician

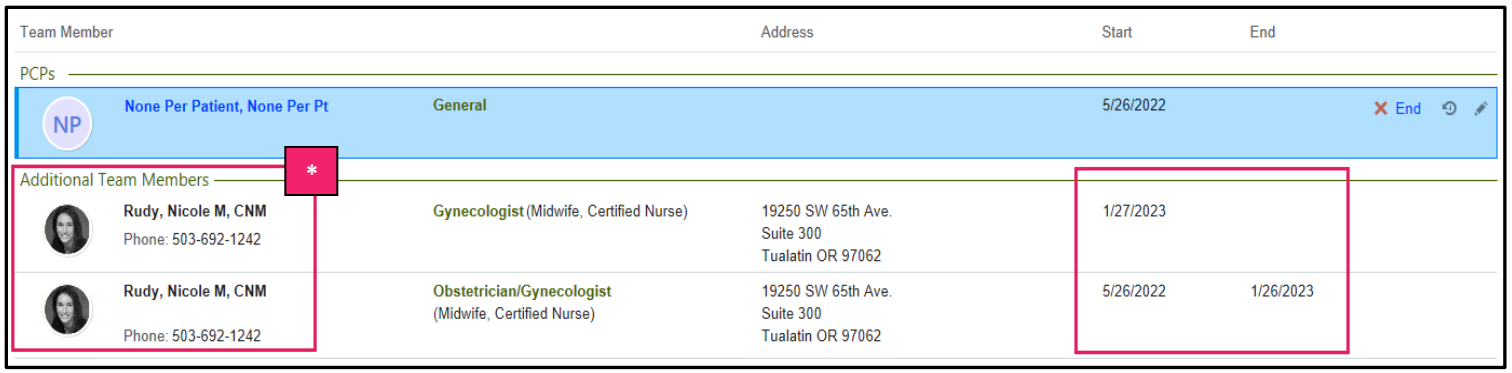
e. Referring Provider – external provider referring to WHA MFM's

f. Urogynecologist – when a patient is seeing a specialist for urogynecology needs

g. Psychiatric Mental Health NP – when a patient is seeing the Psychiatric Health Nurse Practitioner

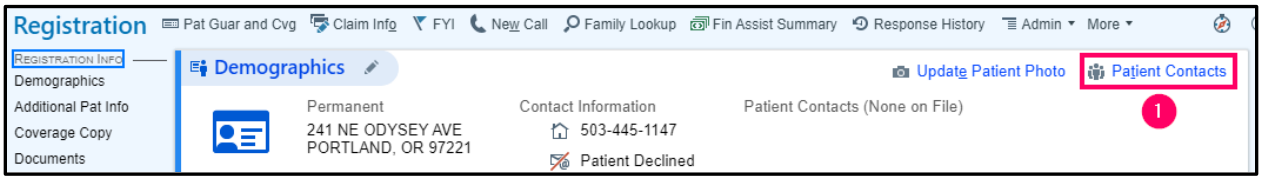
5. Click **Accept**

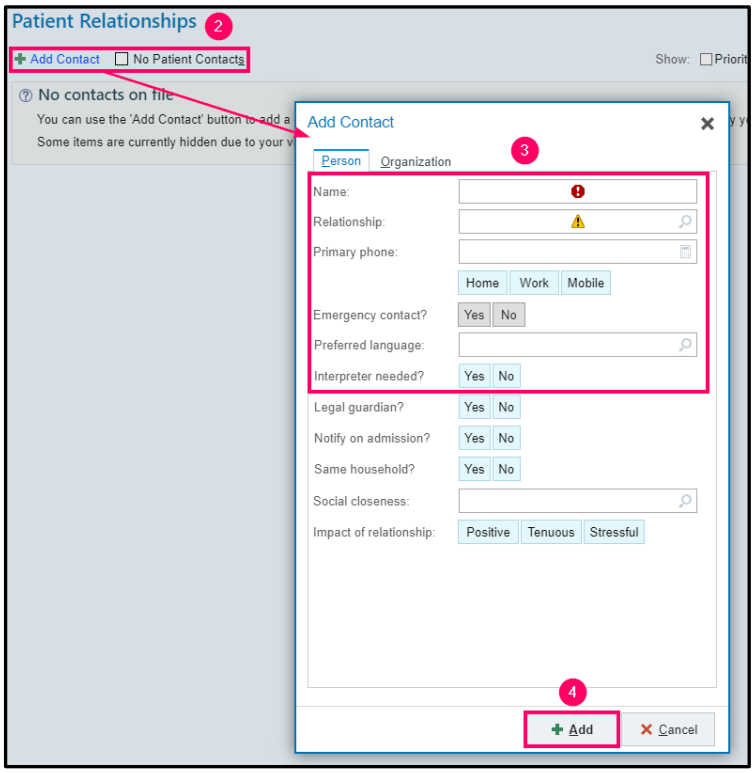




* These are examples of what a patient's Care Team preferences could look like

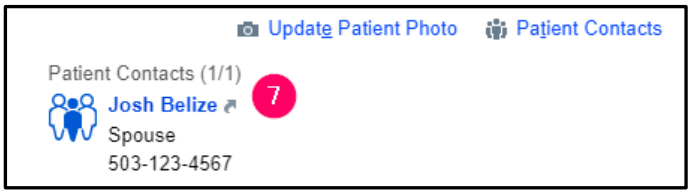
Registration – Adding an Emergency Contact





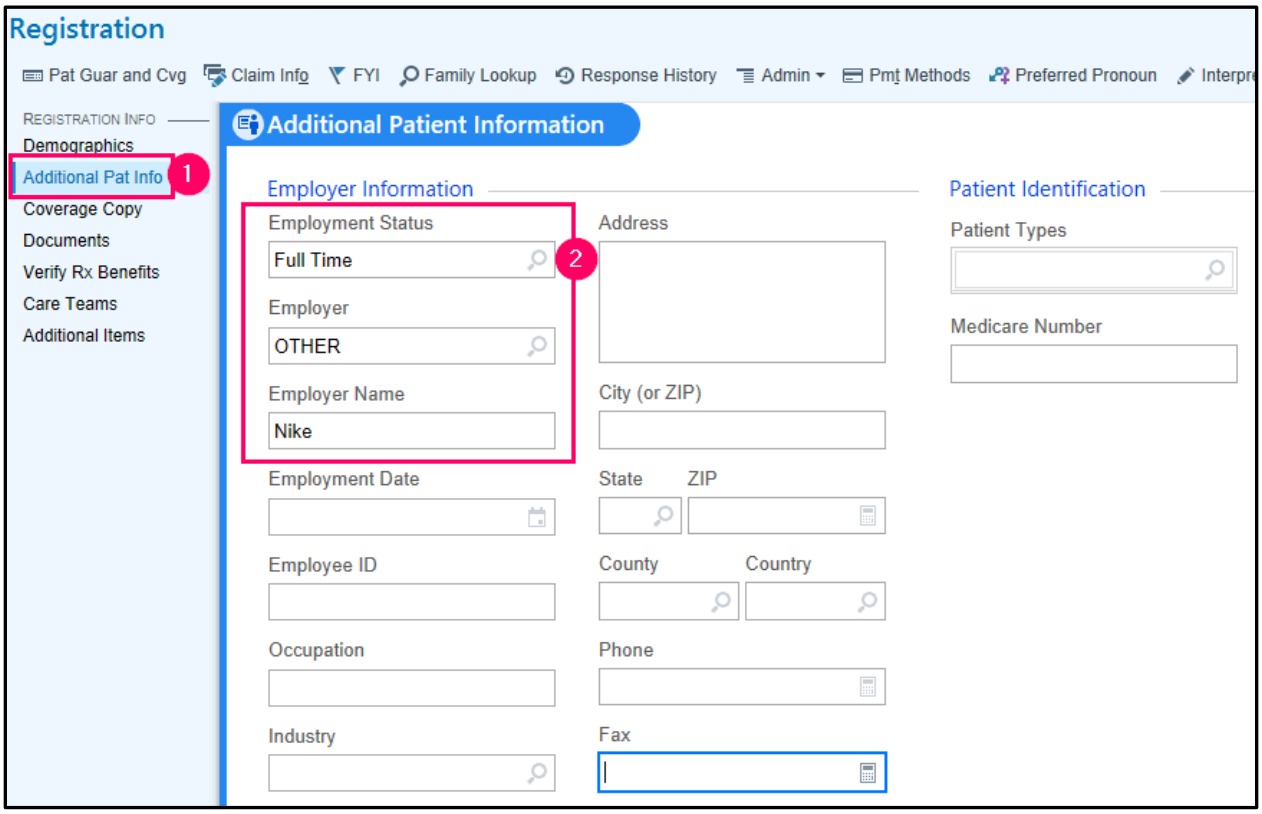






1. To add an Emergency Contact, click the **Launch Patient Contacts** hyperlink in the top right-hand corner of the Demographics tab
2. Click **Add Contact.** If the patient declines adding an Emergency Contact click **No Patient Contacts**
3. Enter the Emergency Contact's Name, Relationship, and Primary phone number, preferred language and if they need an interpreter
   - *As a LEAP/Connect Clinic, we are unable to choose the Emergency Contact option for WHA contacts. Our contacts will always fall under "Other Contacts"*
4. Click **Add**
5. Each contact needs to be labeled as a WHA listed contact. Click the **pencil icon** to open the edit contact window
6. Scroll to the bottom of the window, find the **Comment** section and enter specifically "WHA – Emergency Contact"
7. The contact(s) will appear under the Demographics section of Registration. However, we need to click into the contact itself to see the added Comment

Registration – Adding Employer Info



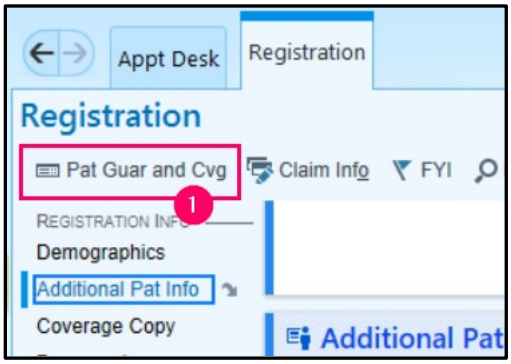
1. The patient's employment / school information will be found under the Additional Pat Info section of the Registration
2. Enter the patient's employment status and (if employed) the employer

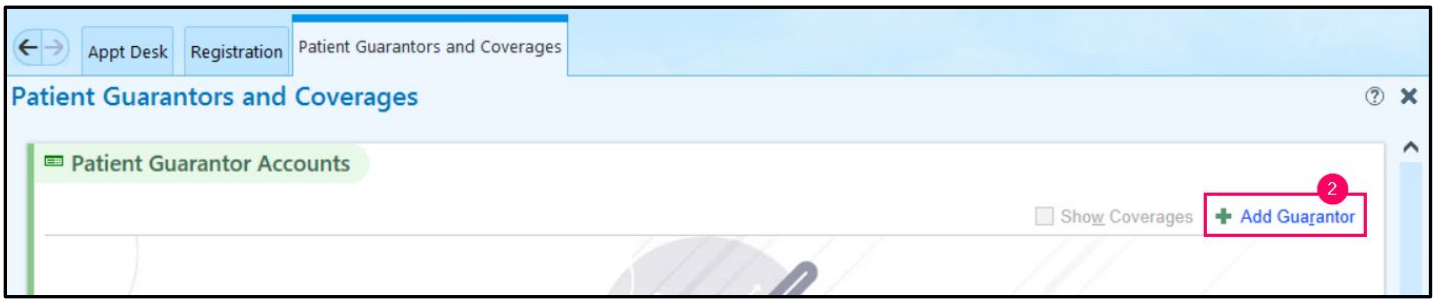
   *If the patient works for Legacy, type **Legacy under Employer**

   *If the patient does not work at Legacy, type **Other under Employer**

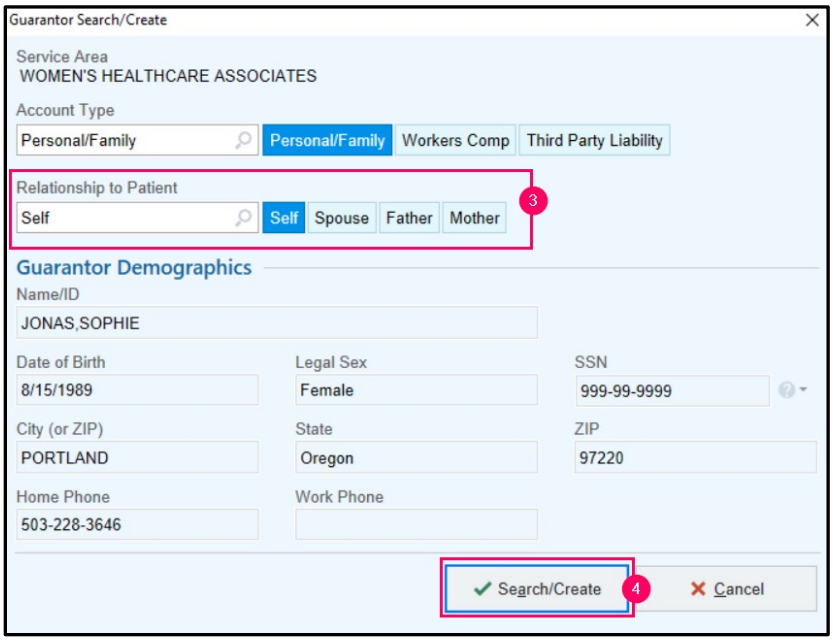
   Enter the Employer Name

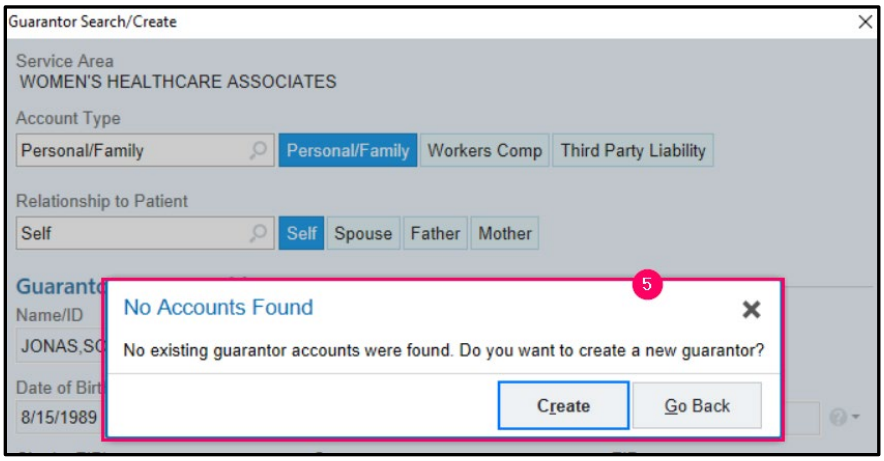
Registration – Adding a Guarantor/Insurance





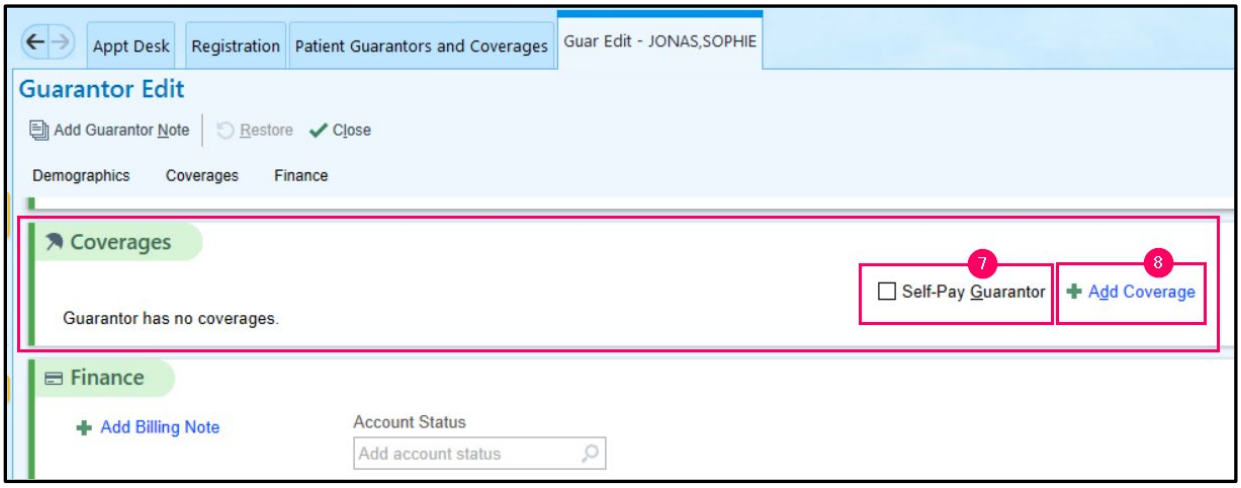
1. To navigate to the Guarantor section, click the **Pat Guar and Cvg** button in the top left hand corner of the Registration screen
2. To start adding a new Guarantor account, click **Add Guarantor**



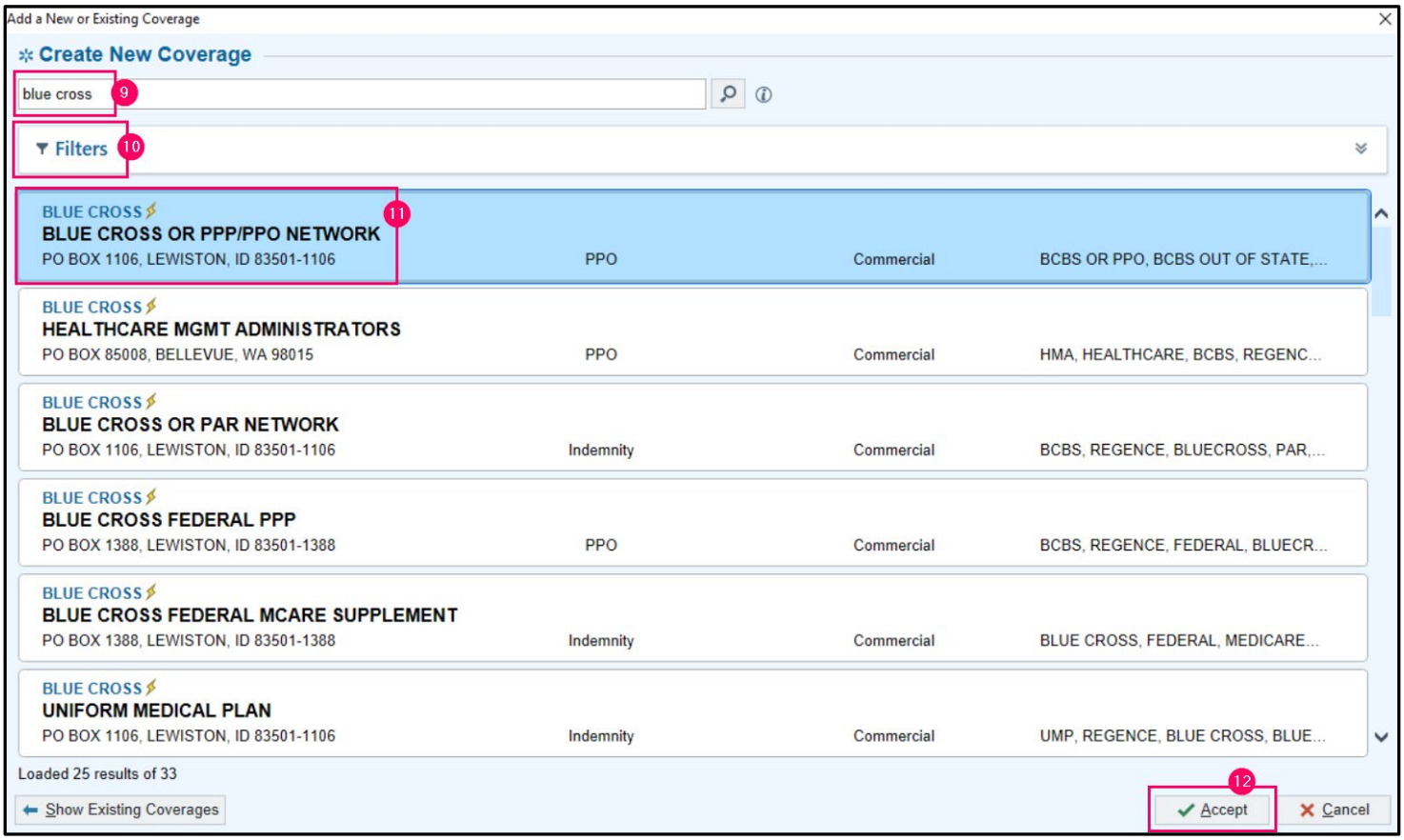


3. A Guarantor is the person who is financially responsible for all bills. **If the patient is under 18,** this will usually be their parent/guardian (patients under 18 can choose to be their own Guarantor though if they prefer). **If they are 18 or over,** the patient legally has to be their own guarantor unless they are a dependent adult
4. Once you have the correct person selected, click **Search / Create**
5. If another Guarantor account for the person you searched for is found, a window will prompt to have you verify and then select. If no accounts are found, click Create to add a new Guarantor account

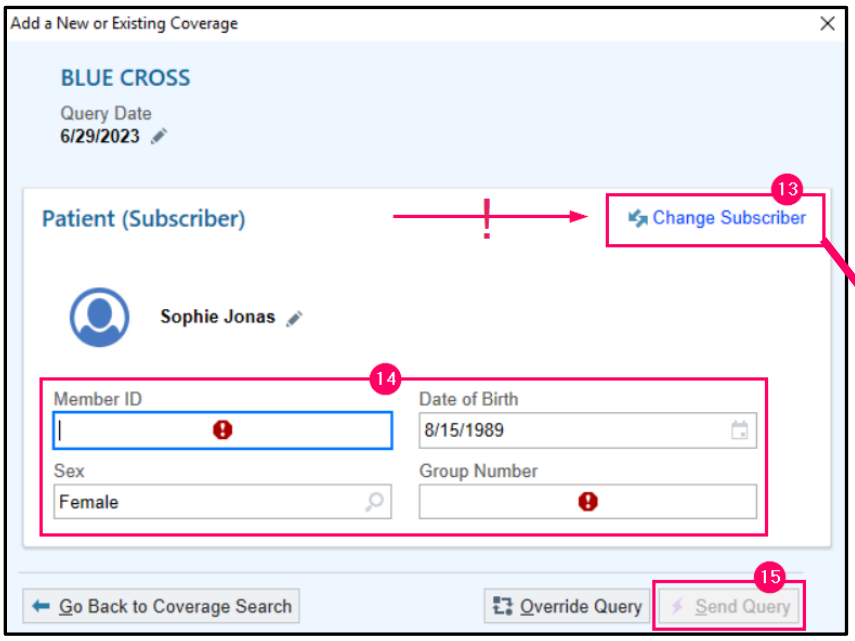


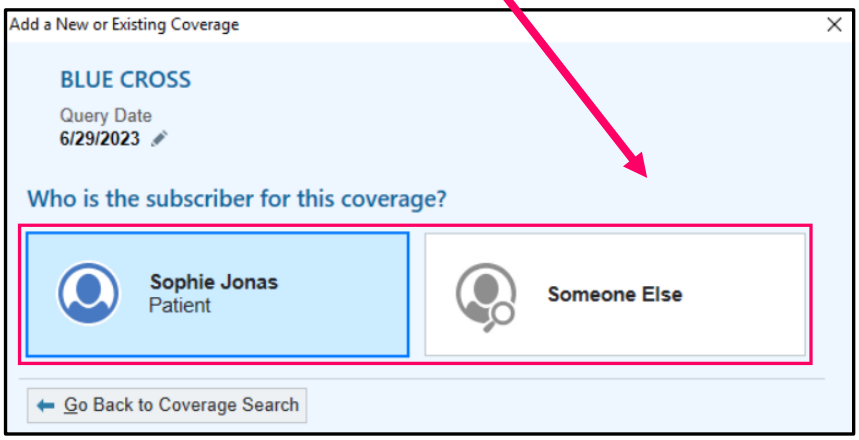


6. When the Guarantor Edit screen opens, a new tab will also appear at the top and the Guarantor verification will also be added to the Checklist
7. If the patient has never had any insurance listed in their Registration ever before, and the patient is truly Self-Pay click the Self-Pay Guarantor box. If this box is greyed out and you cannot click it but the patient is Self-Pay, send the Financial Coordinators a Staff Message letting them know
8. To add an insurance to the patient's Registration, click the **Add Coverage** hyperlink

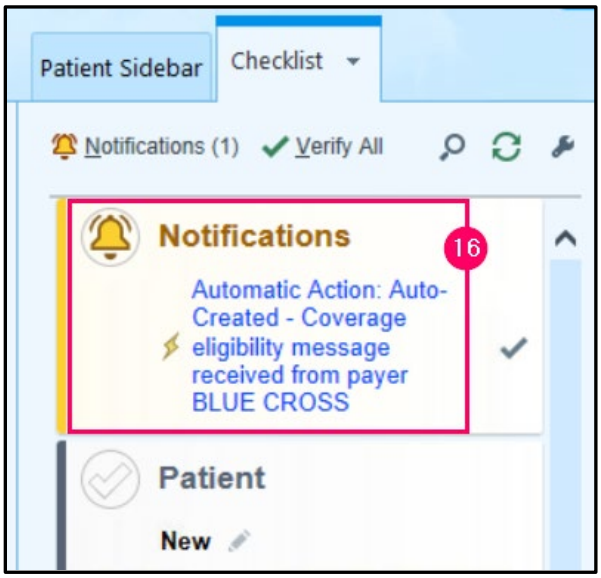


9. Type in the **patient's insurance payor's name**
10. If you are having difficulty finding the exact plan name, use the **Filters**
11. Once you find the exact plan, click on it
12. Click **Accept**



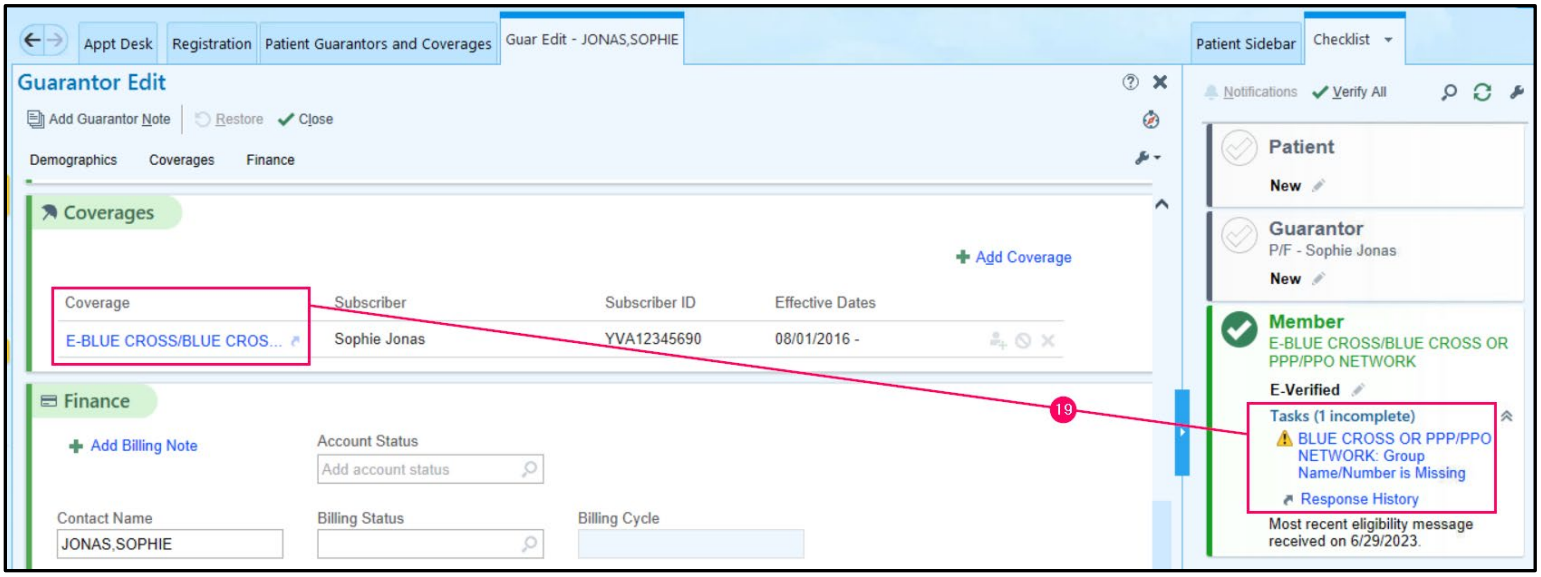


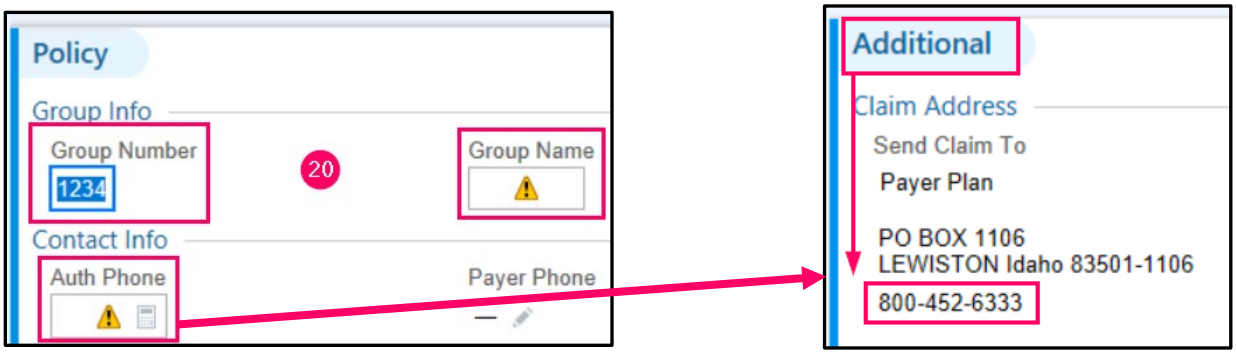
13. **IMPORTANT step**, ask the patient if the insurance is through their own employer, Medicare, or Medicaid **or** if their insurance is through a family member/spouse? If it is through someone else, click **Change Subscriber** and then **Someone Else** in the next window. If the patient is their own Subscriber skip this step.
14. If the patient is **their own** Subscriber, fill in the missing fields for Member ID and Group Number
15. Click **Send Query**



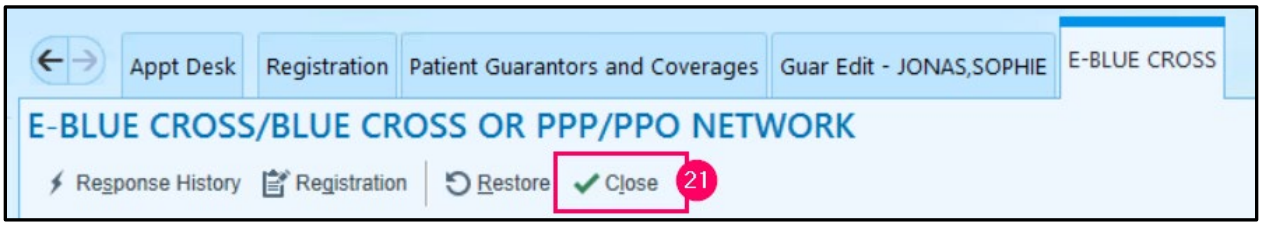


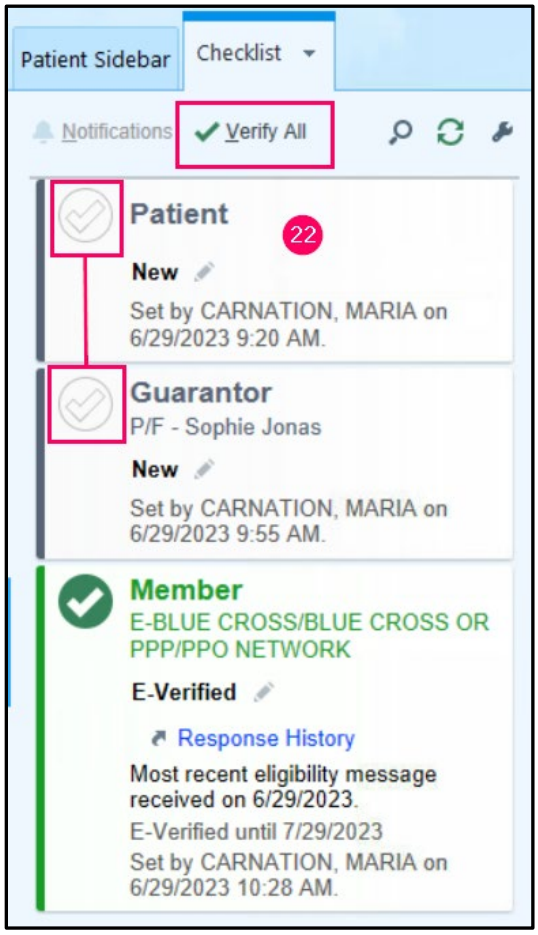
16. Once you send the insurance query, after a few seconds a **Notification** will appear in the checklist with a bell icon. Click the hyperlink associated with that bell
17. Verify that the insurance comes back as **E-Verified or Eligible**. If it comes back as **E-Rejected**, **Needs Review**, **Error**, **Contact Payor**, **Replacement Plan Indicated**, or **Data Mismatch** (which we should NOT be adding to the patient's Registration) call the **Financial Coordinator** team to help correct the error





19. Under the Coverages section, click the **insurance hyperlink**. Or you can click the hyperlink in the checklist to bring you to right to the missing fields
20. Fill in any missing fields. To find the Authorization Phone Number, scroll to the bottom under the **Additional section**





21. Click **Close** to go back to the main Registration window
22. Once you have completed each section, click the check to mark it as verified (or simply click Verify All to do all in one click)

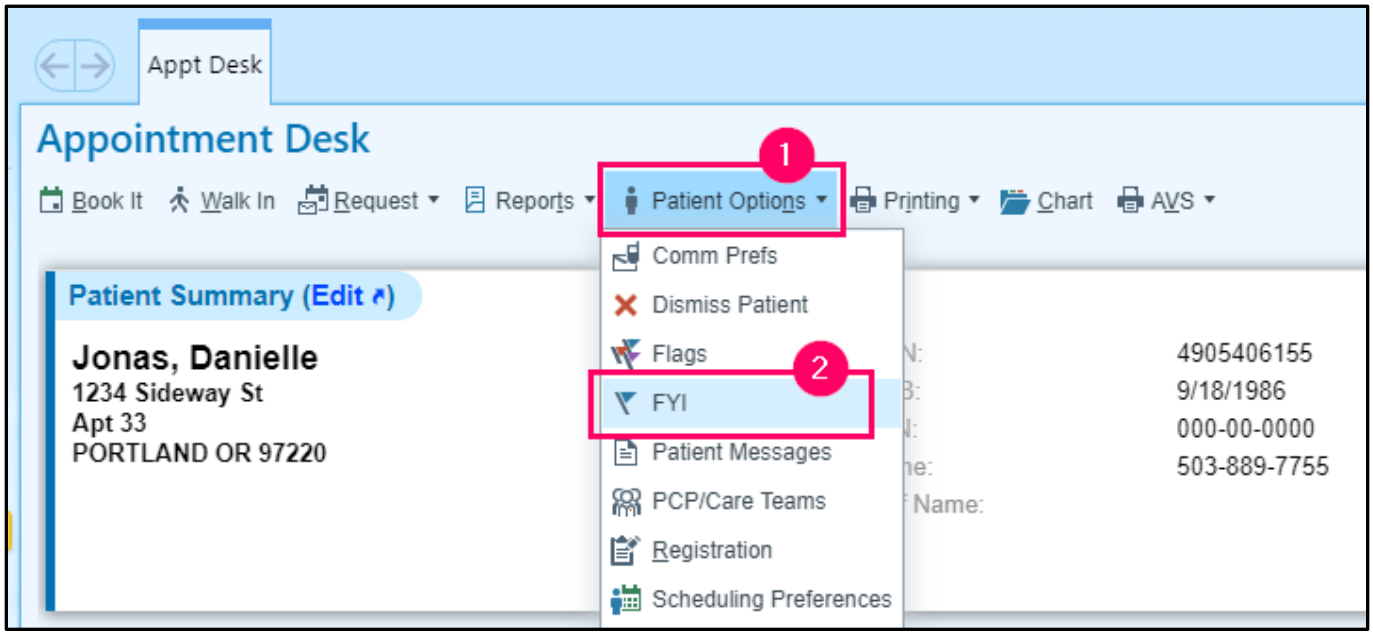
Registration – Editing Established Patient Information

When initially searching for the patient's registration, if the patient does not recognize the DOB/address/or name that you have on your screen, instead of editing it – notify your supervisor either verbally or through a Staff Message

Do not change unless you have the PSS's permission

If the patient says they recently had a Marital status change or has recently moved, BUT recognizes their old information, then it is ok to proceed with editing the patient's information without a PSS's permission

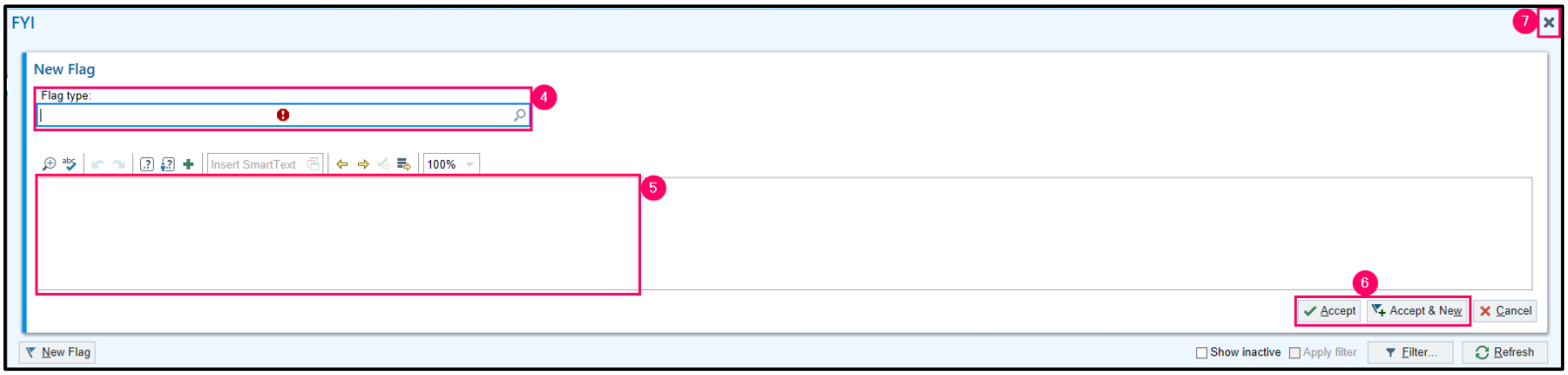
Patient FYI's on Appt Desk

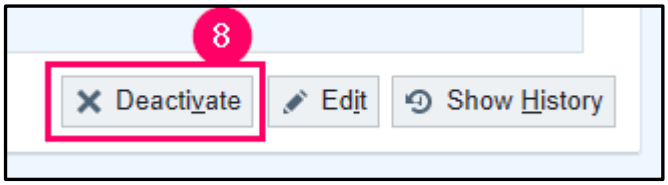


FYI's are used to share information regarding the needs of the patient regarding their account or scheduling. FYI's are seen by all Legacy/Connect clinic. FYI's display on the Patient Sidebar. *Example: "Patient is hard of hearing"*

1. Click on **Patient Options** in the activity tool bar
2. Click on **Patient FYI** in the drop-down menu

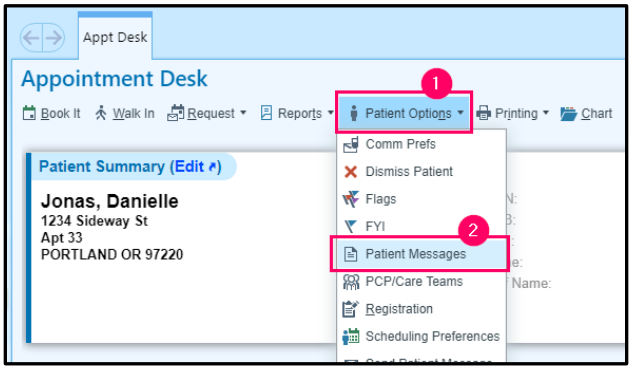


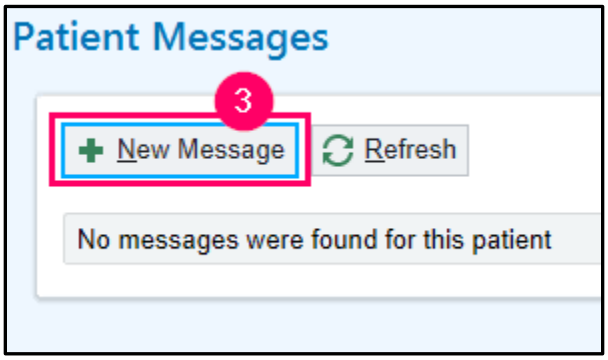


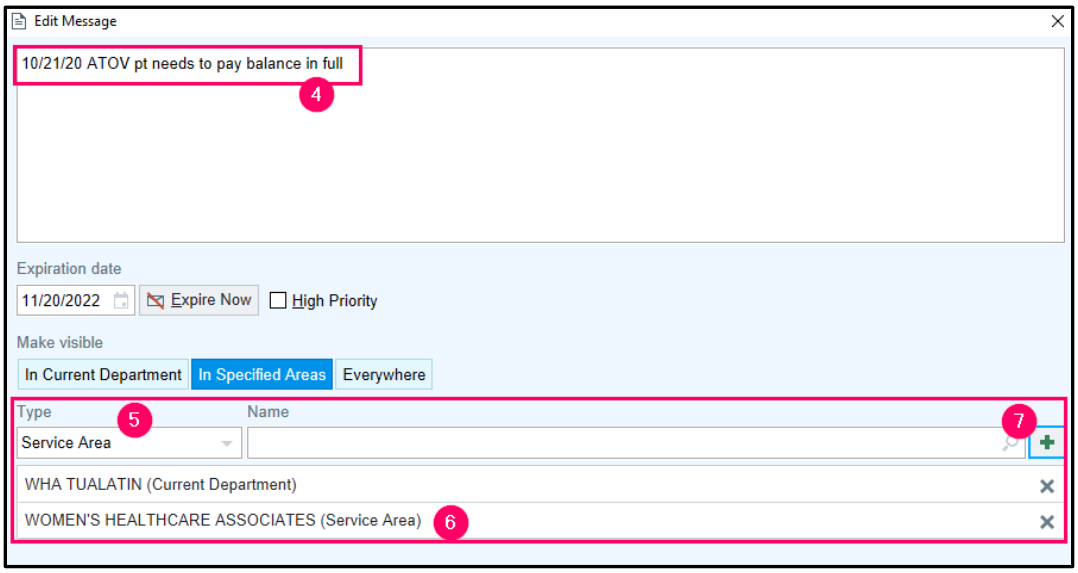


3. Click on the **New Flag** button
4. Enter specific **FYI**
   **You can use the magnifying glass to see the options*
5. Start with today's date and then enter details for the FYI
   It is Legacy Best Practice to start with the date and to keep in mind that All of Legacy / LEAP clinics use these notes and to keep them as neutral as possible Example do **NOT** put any notes like "Patient needs WHA Procedure Room" because it is not only for WHA and not all clinics have a Procedure Room
6. Click **Accept** if you are finished click **Accept & New** if you are done with the FYI and need to create another one
7. Click here to exit the FYI screen
8. If an FYI is no longer applicable, click on the FYI and then at the bottom of the screen click **Deactivate**

Patient Messages on Appt Desk



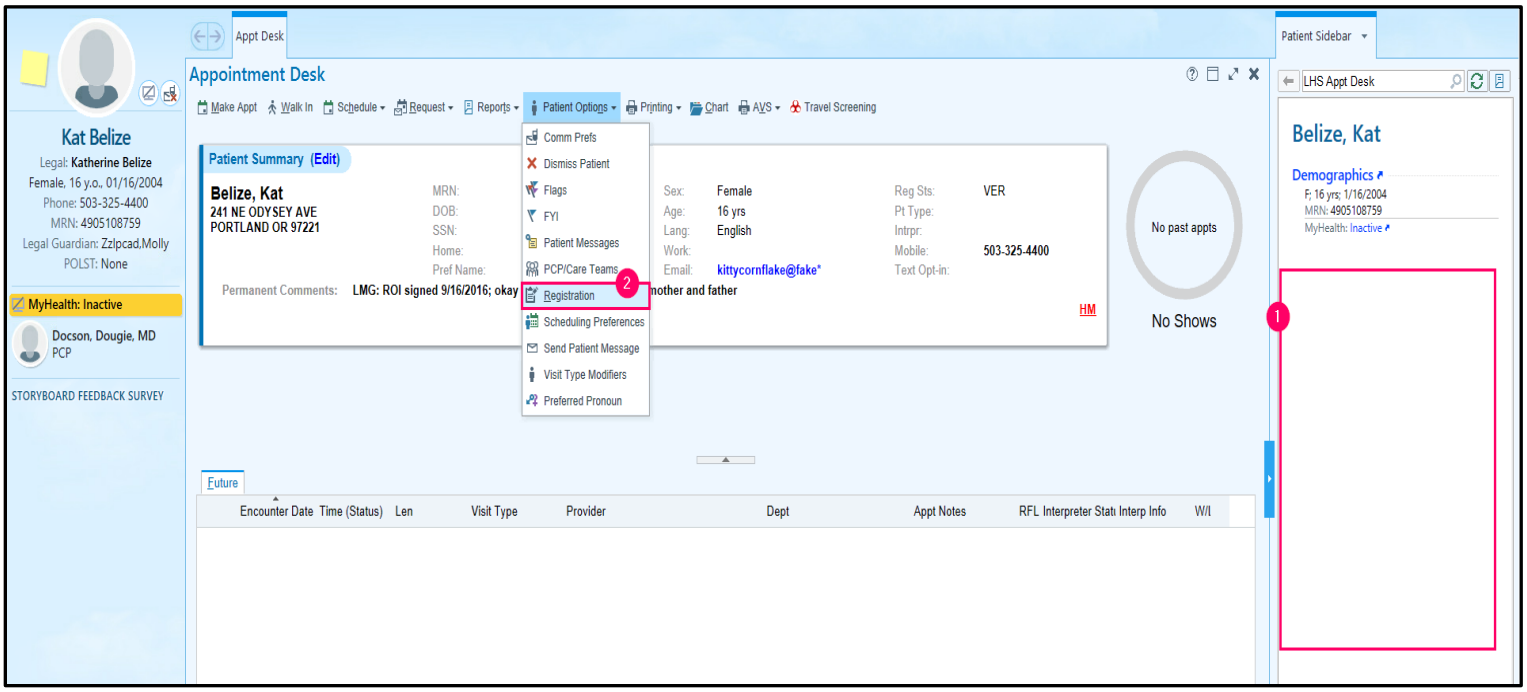


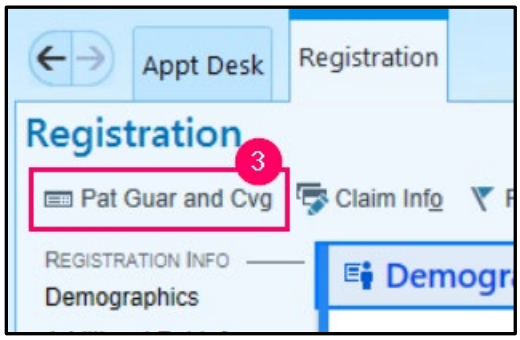


Patient Messages are used to share information about the needs of the patient regarding their account or scheduling

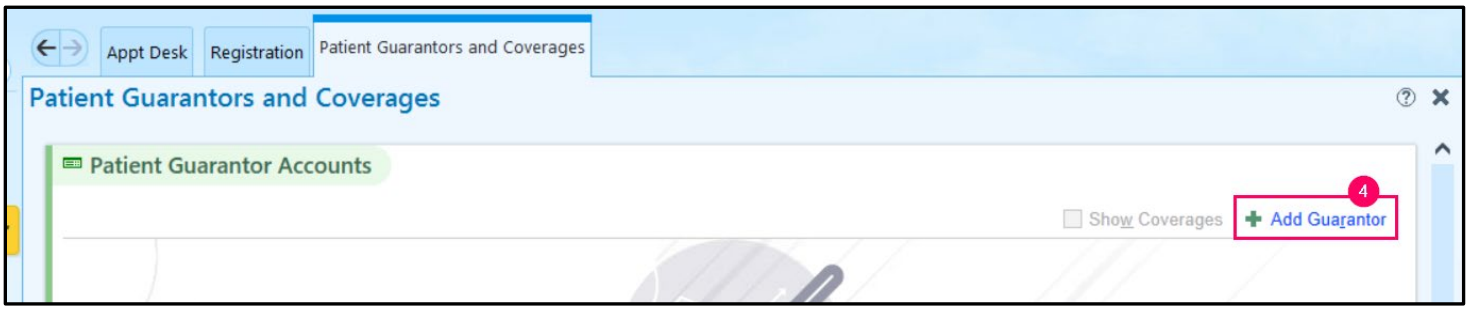
1. Click on **Patient Options** above in the Activity Tool Bar
2. Click on **Patient Messages** in the drop-down menu
3. Click on **New Message**
4. Start with today's date and then type in your message
5. Under Type choose **Service Area**
6. Under Name choose **Women's Healthcare Associates (Service Area)**
7. Click the **+ button** to add WHA Service Area

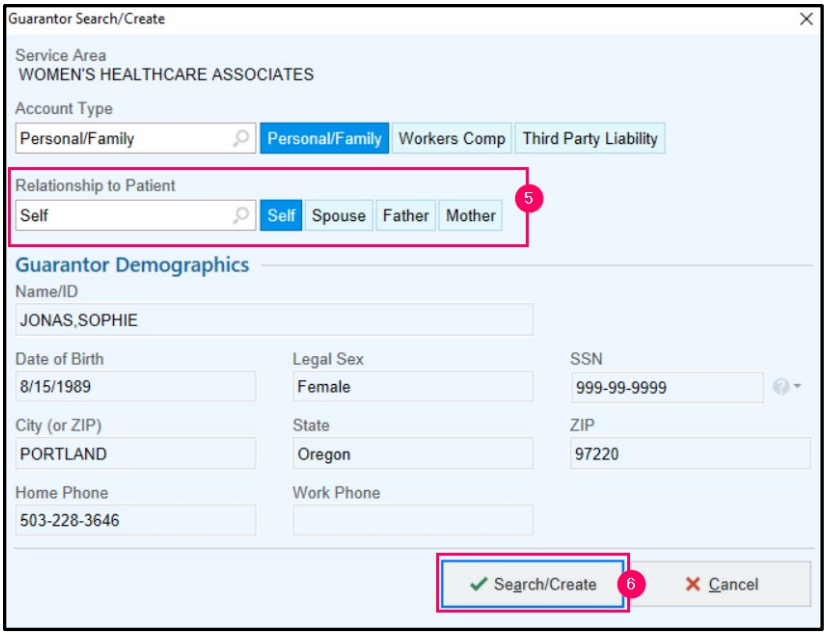
Registration – New to WHA but already in Epic

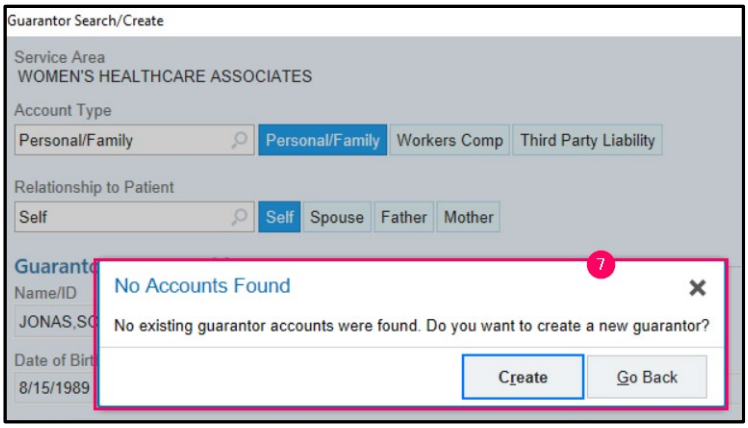




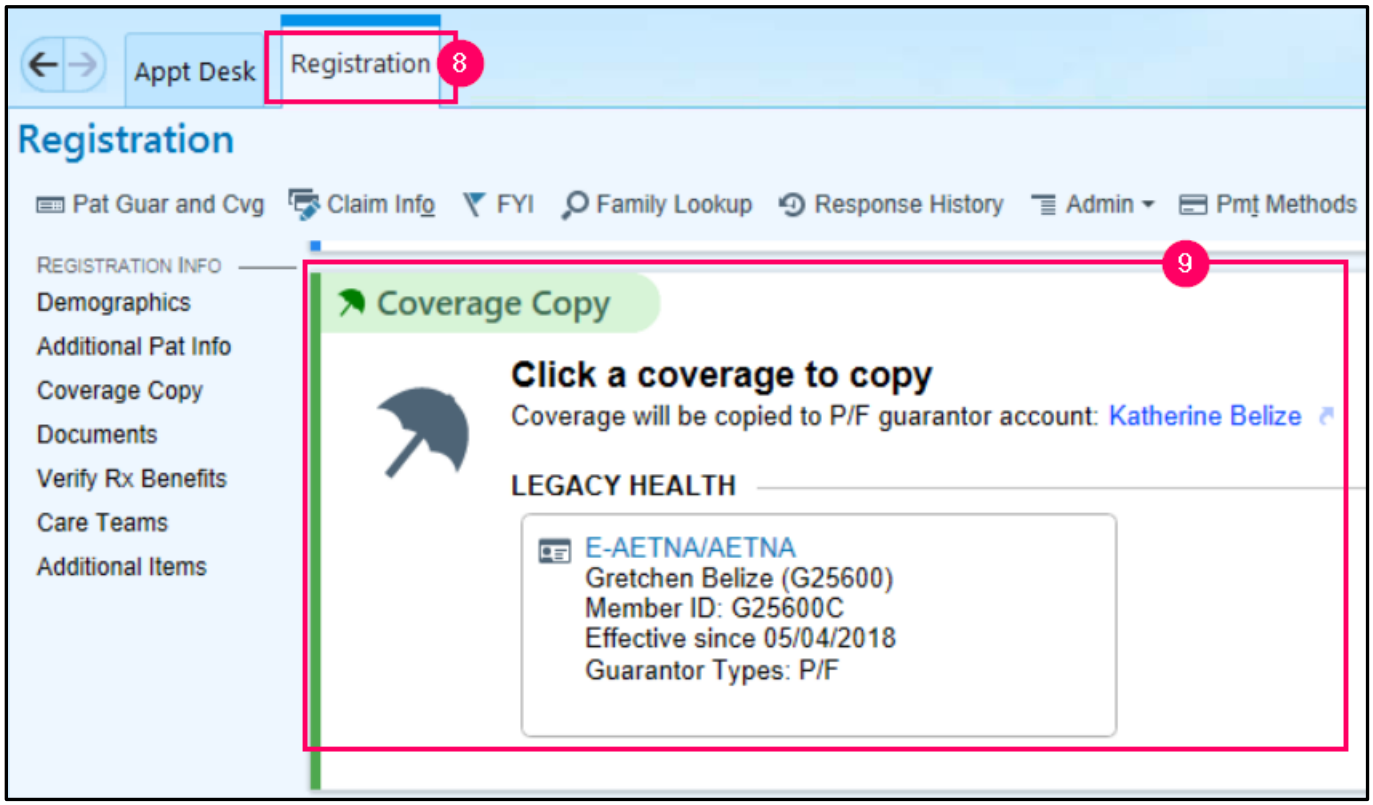
1. You can identify if the patient is not a WHA patient if there is no Guarantor account or insurance displayed in the Patient Sidebar
2. Click on **Patient Options** then **Registration** to open the Registration
3. Click the **Pat Guar and Cvg button** in the top left-hand corner of the Registration





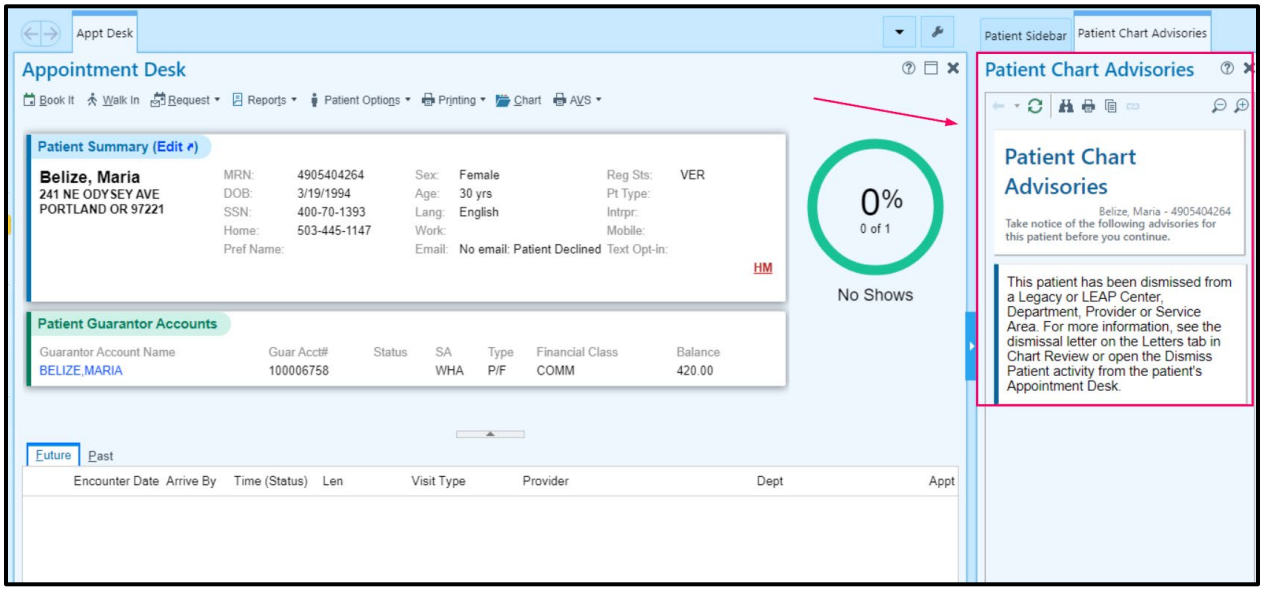


4. Click the **Add Guarantor** hyperlink
5. A Guarantor is the person who is financially responsible for all bills. **If the patient is under 18,** this will usually be their parent/guardian (patients under 18 can choose to be their own Guarantor though if they prefer). **If they are 18 or over,** the patient legally has to be their own guarantor unless they are a dependent adult
6. Once you have the correct person selected, click **Search / Create**
7. If another Guarantor account for the person you searched for is found, a window will prompt to have you verify and then select. If no accounts are found, click Create to add a new Guarantor account



8. To add an insurance traditionally, go to the section of this manual titled **Registration – Adding a Guarantor / Insurance steps 7 -21**. But because WHA shares a system with Legacy, occasionally some patients will import coverages from other Legacy Guarantors. This is called Coverage Copy. To find this section, click the Registration tab at the top
9. Scroll or click the **Coverage Copy** section within the Navigator pane. If a coverage has been found through the Legacy system, it will be listed under this section. Before adding any insurances listed, verify with the patient that this insurance plan is still active and current. If so, click the insurance hyperlink box to add

Patient Chart Advisories



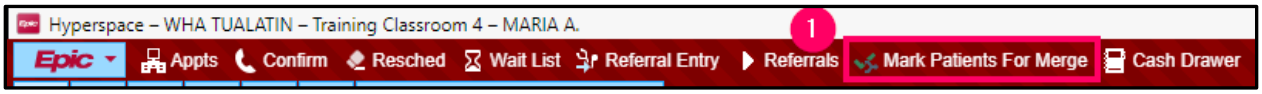
Patient Chart Advisories alert users caring for the patient to notify them that there is a chart correction in progress or that a label has been attached (such as Term of Care or KND)

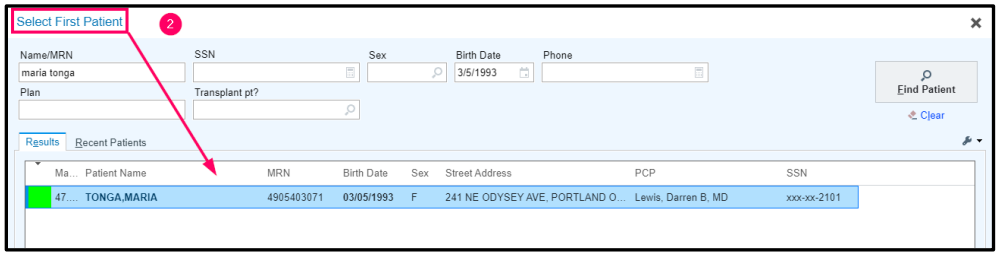
- These notifications will always be in the Patient's sidebar
- If you feel that a notification has been added incorrectly, submit a Helpdesk ticket to WHA's IT team

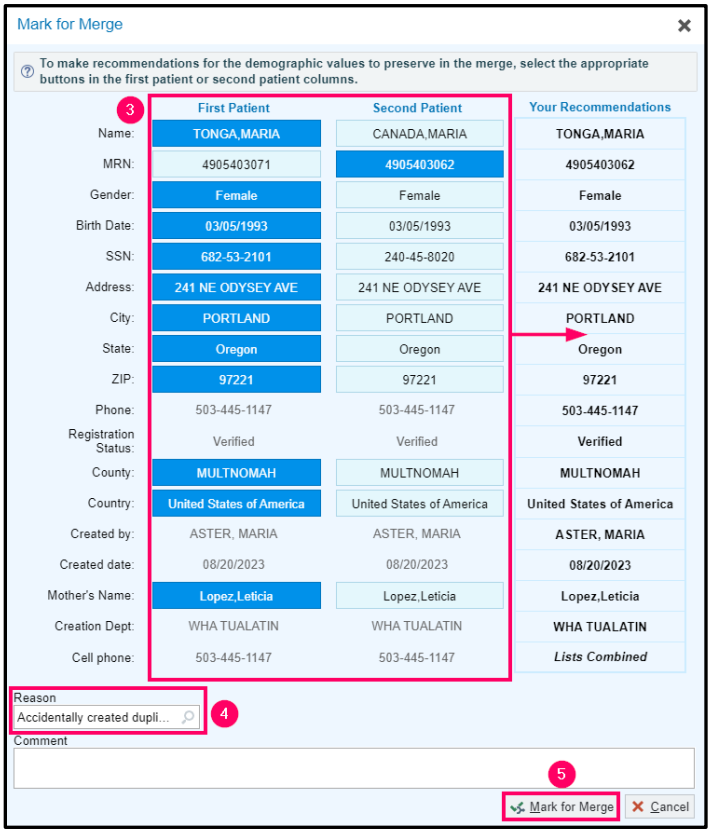
What steps to take:

- Familiarize yourself with each subchapter below and its contents
- Verify you are using proper patient look-up processes by using 3 patient identifiers to prevent future mistakes
- If you have further questions, contact your Supervisor or the Training Specialists team

Mark Patients for Merge



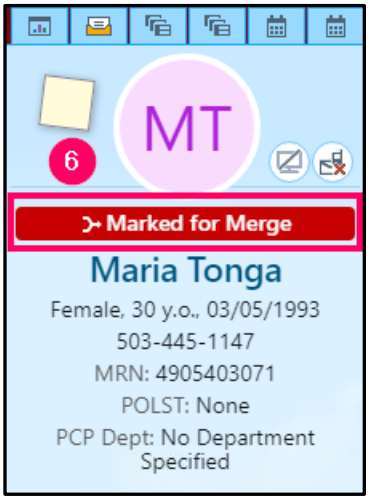


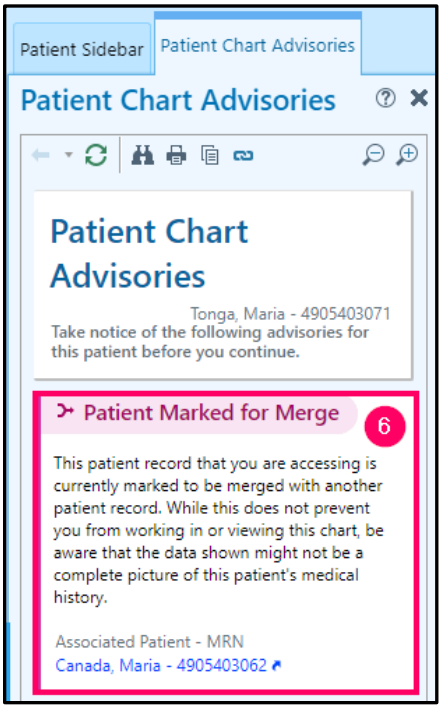


1. If you come across a duplicate patient (*or accidentally created a duplicate*) you will want to use the **Mark for Merge** activity located on the Hyperspace Toolbar

2. After selecting Mark For Merge, you will look up the **First** duplicate patient. Find the patient and click **Accept** to open a **Select Second Patient** window. Follow the same steps looking up the second duplicate patient

3. Select the most accurate basic demographic options from the First and/or Second patient columns as recommendations for the merge *(see more steps below)*





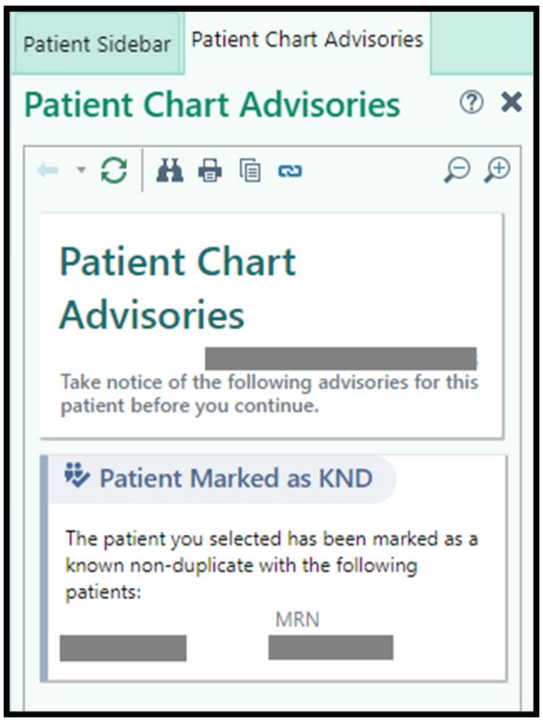
(*Previous page continued*)

4. Add a **Reason** as to why you are marking the patients to be merged. If you choose *Other*, make sure to include a free-text **Comment** below

5. Click **Mark for Merge**

6. When either patient's Registration, Chart, etc. are open there will be a **Patient Chart Advisory** within the **Patient Sidebar** or **Storyboard indicating the patient has been Marked to Merge**. Selecting the patient name hyperlink will jump you directly to the other patient's Chart (*and vice versa*)

Known Non-Duplicate (KND)



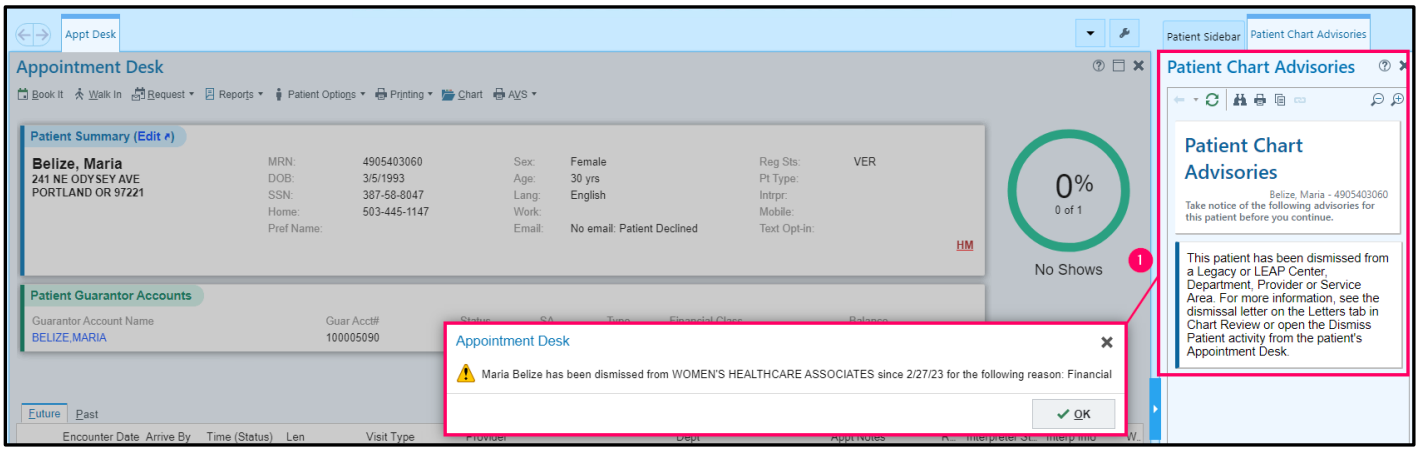
Known Non-Duplicates (also referred to as "KNDs") are patient records that look similar but are confirmed to be separate patients

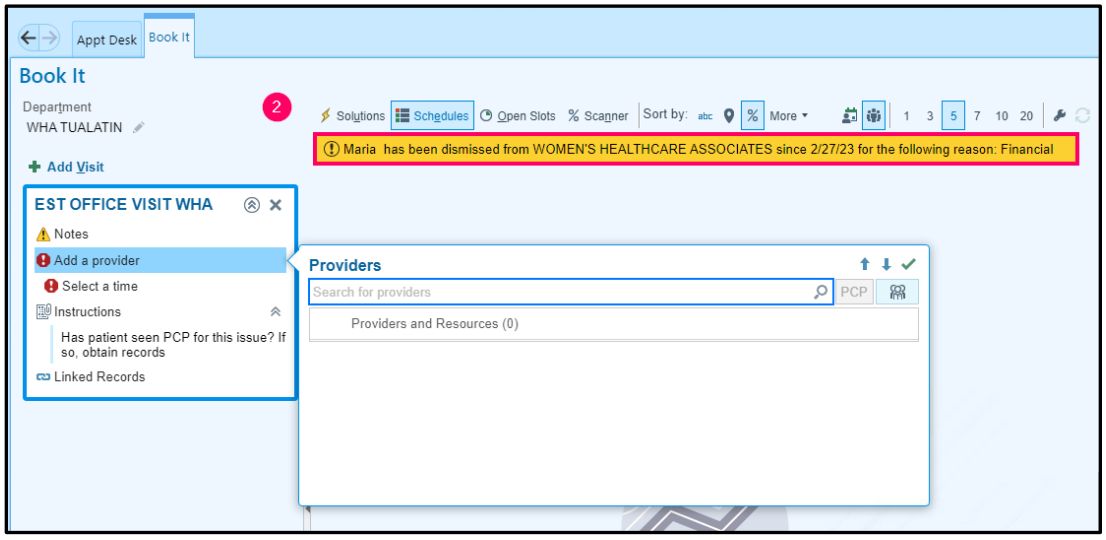
- Are actually separate patients with similar demographics and properties
- Example: twin sisters since they have the same birthdate and most likely the same last name, address, home phone number, etc.
- Pairs that are marked as KNDs do not appear on Potential Duplicate workqueues
- Can't be merged together unless the KND status is removed
- The system will display an error if a user attempts to merge KNDs

What steps to take:

- Note that this patient is **confirmed a different patient** than the one listed in the advisory
- If Registering patients that are twins, once off the completed phone call submit a Helpdesk ticket to WHA's IT team letting them know both MRNs so that they can add this KND Advisory to their Charts – *this is to help prevent any HIPAA errors in the future*

Term of Care

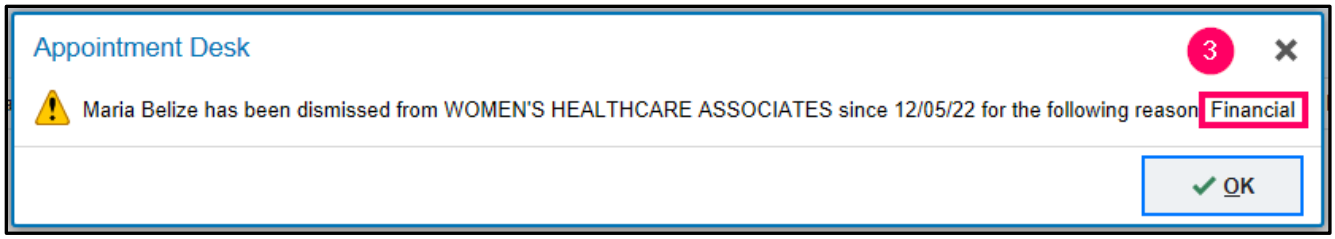




1. When a patient is marked as Term of Care from WHA, Epic will display a **Patient Chart Advisory** within the Patient Sidebar. A pop up will also appear indicating why and when the patient was dismissed

2. The Book It button will still be available but the patient will not be able to be scheduled at this time

    - If you proceed to click Book It: After entering a Visit Type, a message will appear not allowing for scheduling due to the patient's current status with WHA





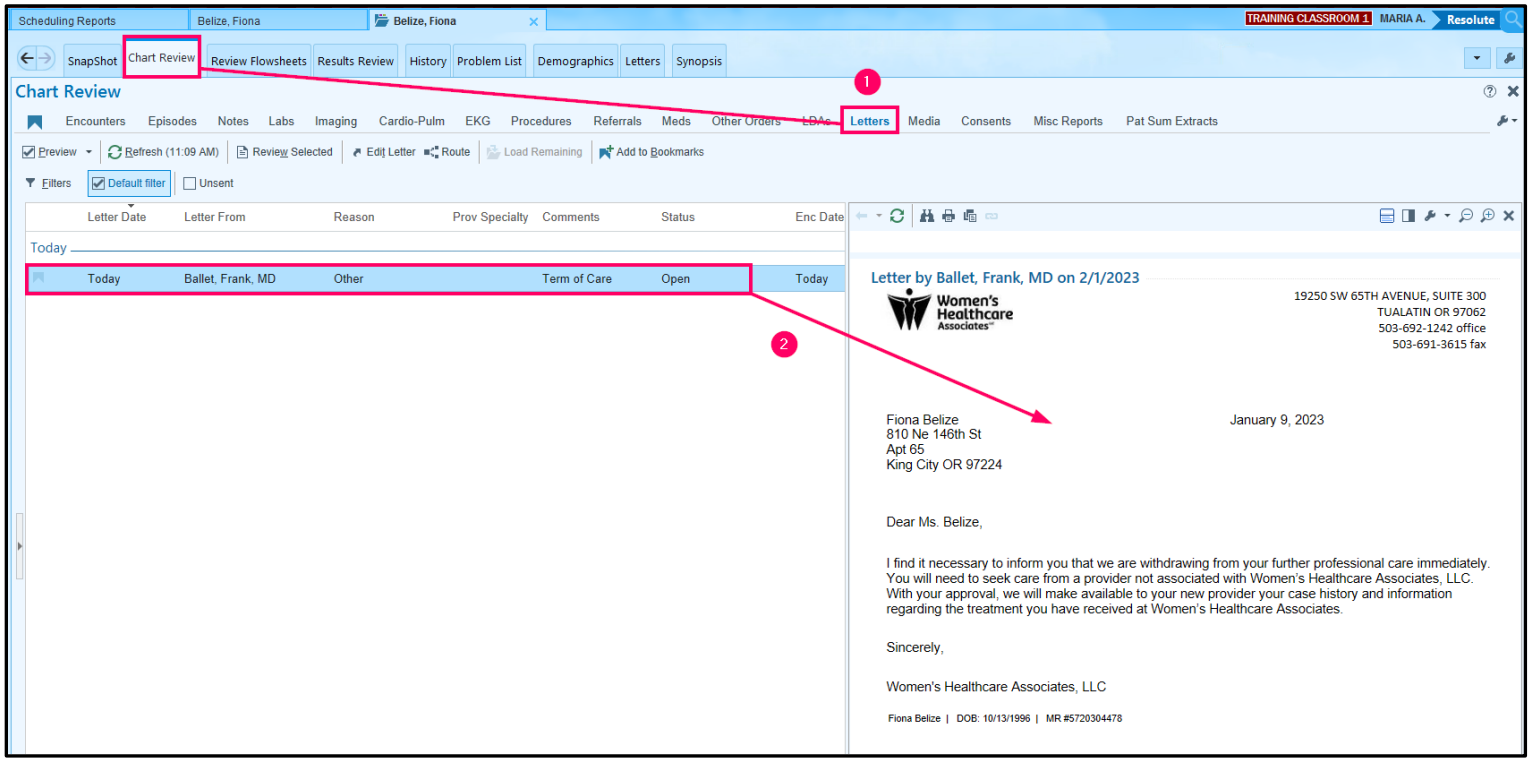


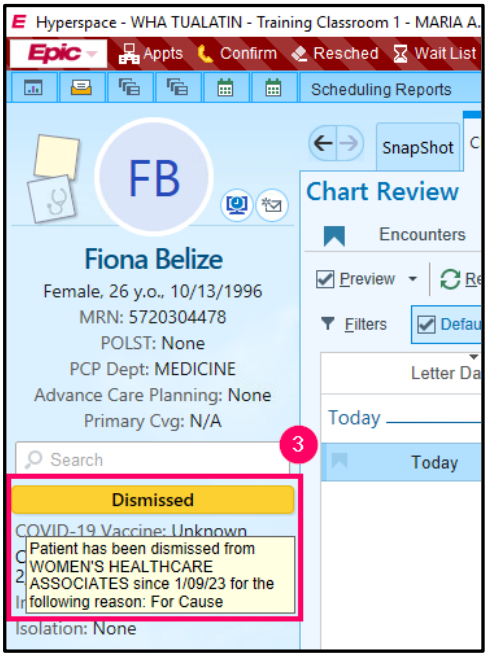
3. Determine what the patient was dismissed for

- **Financial**: Best practice is to do a Warm Transfer to the **FC Backline #1871**. Let the FC know the patient's info then they will give you next steps for the patient
- **For Cause**: These reasons are not financial related and should not be sent to the Financial Coordinators. Ask Supervisor for more details/help
- **No Shows**: These are patients who have been dismissed for too many No Shows in the clinic. After 5 years, this dismissal will automatically expire and the patients can be seen again. Ask Supervisor for more details/help

If the Patient Services staff member feels comfortable searching, you can find the patient's dismissal letter within the Chart to give the patient more information (*see next steps*)

Term of Care Letters



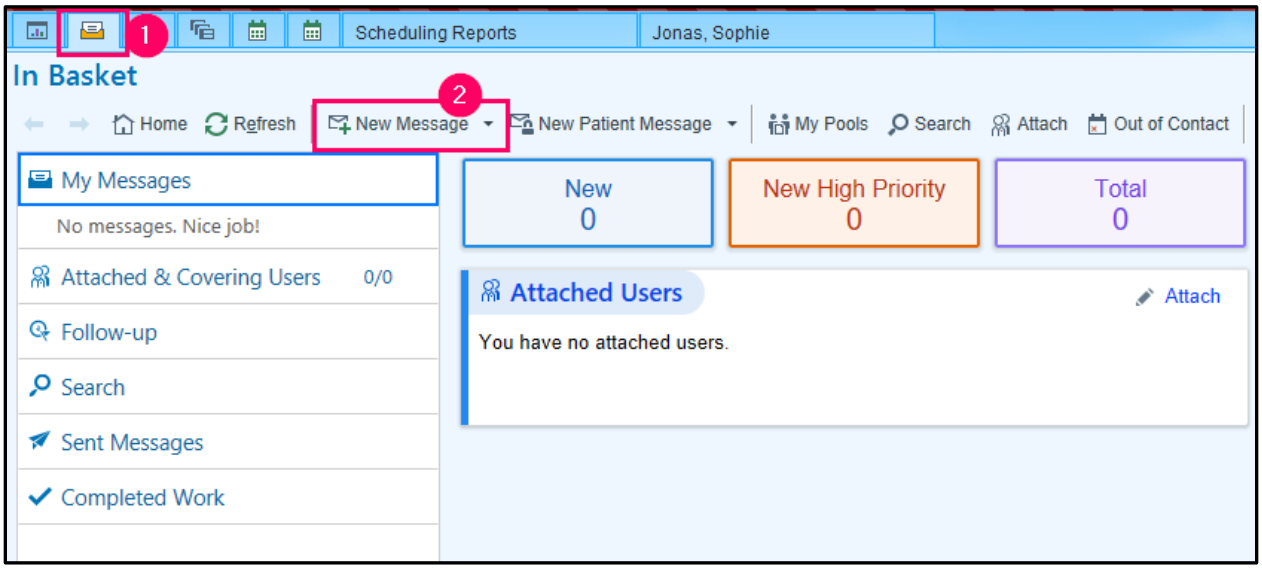


1. To find any term of care letters, go to the patient's chart. Select **Chart Review** then the **Letters** tab

2. Find the appropriate letter and select to expand for more information. Give the patient any pertinent info regarding their dismissal. If needed, contact your Supervisor or Practice Manager for further assistance

3. **Note:** When within the Chart, a **Dismissed** alert will also appear on Story Board. *Hover to Discover* for more information

Staff Messages

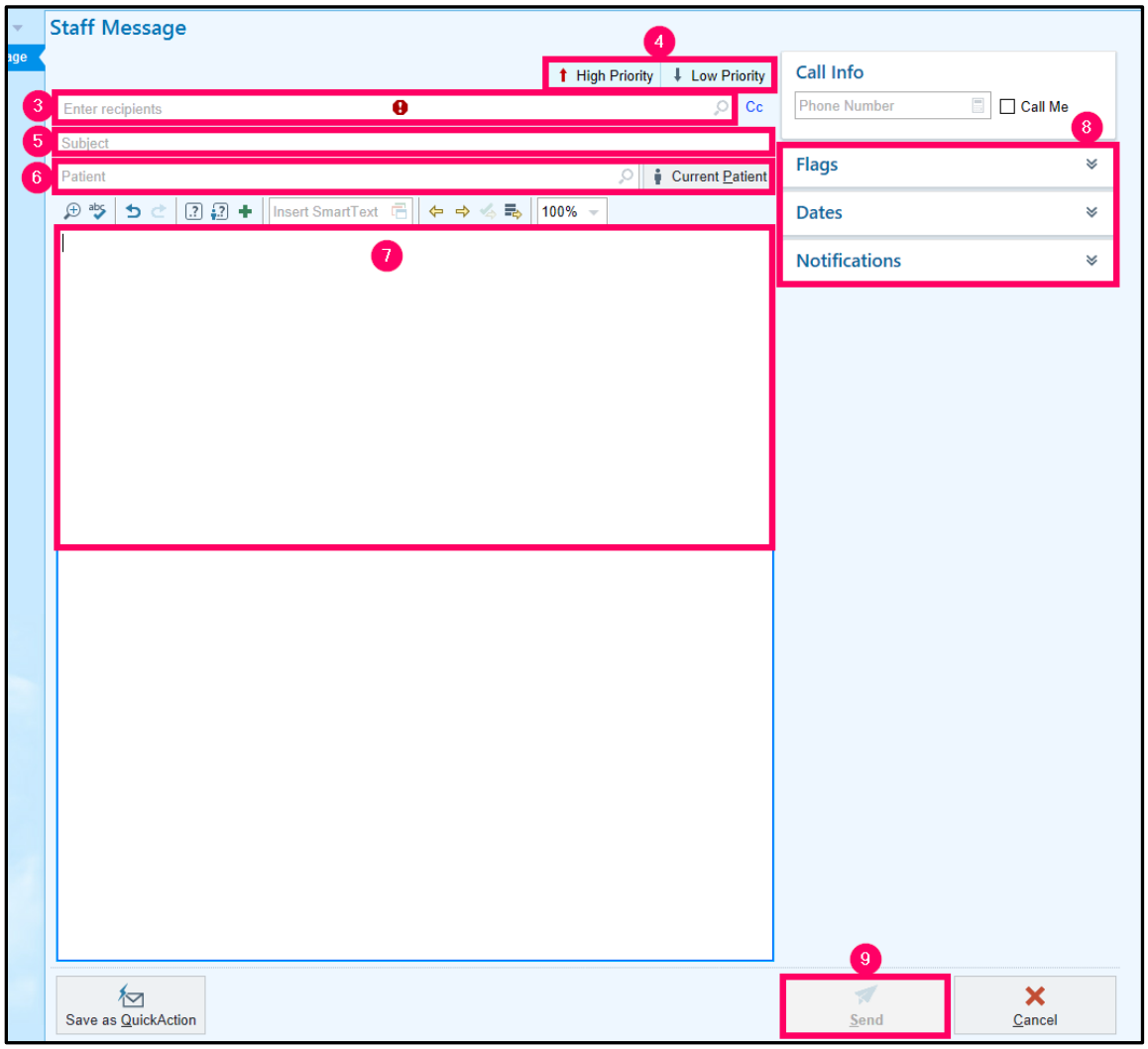


Staff Messages are formatted like an e-mail. It is not a permanent part of the patient's chart, but is sent when items need to be communicated and don't need to be saved permanently (though it can later be changed to be permanent, if needed, by converting the message to a Telephone Encounter)

1. Click on the **In Basket** icon

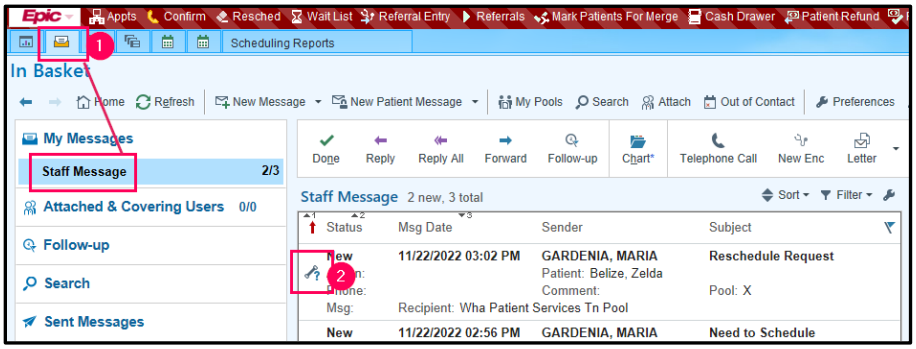
2. Click **New Message**

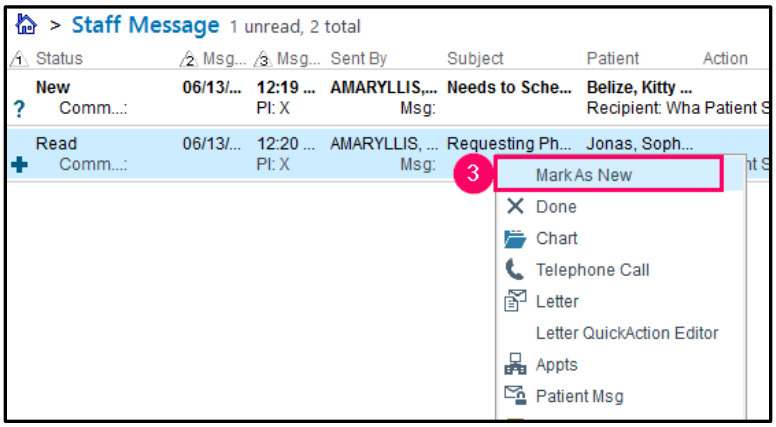
Note: "Patient Msg" is to send a message directly to a patient via MyHealth



3. Type in the name of the pool or individual you are routing the message to [P WHA …] Ex: P WHA TN OB Coordinator
4. You can change the **priority** of the message if needed
5. Enter a **Subject** for the message
6. Put in the patient of whom the message is regarding
   *You can use "=" if the patient's Appointment Desk is the last one you were in
   *You can type in the patient's name
   *You can use "Patient Lookup" to find the patient
7. Type in your message
8. If needed: You can attach a flag, edits dates (Example: Change the delay date to have the message be sent the next week) or set a notification relating to message status
9. Click **Send**

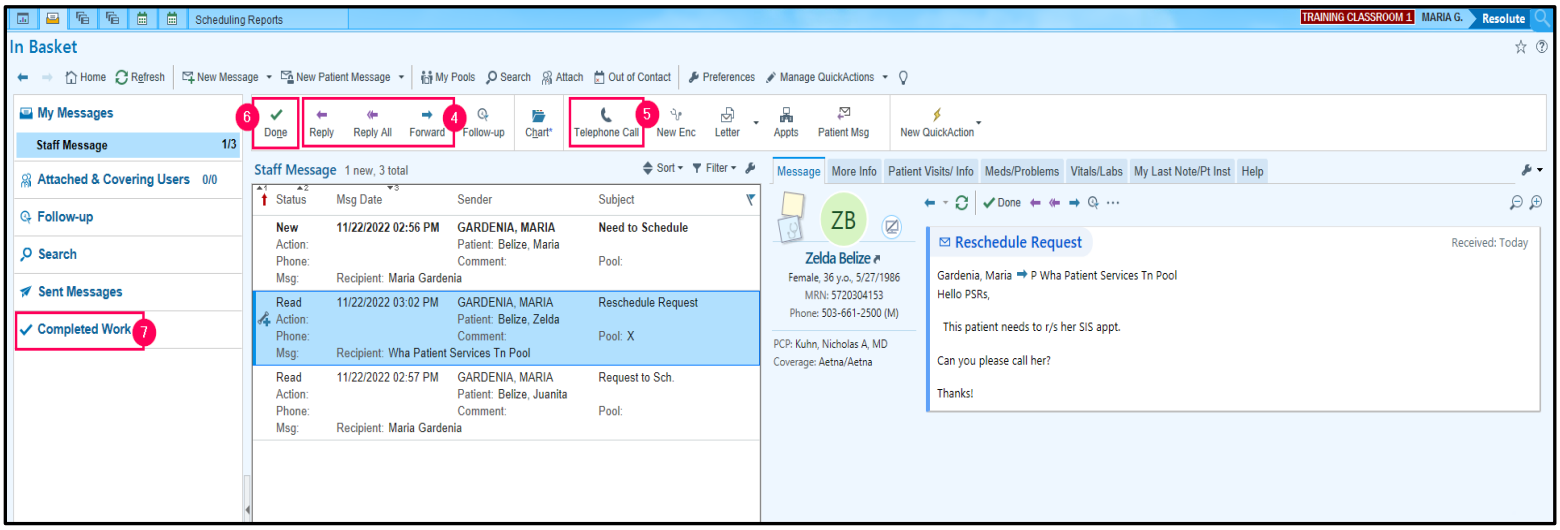
Managing Staff Messages





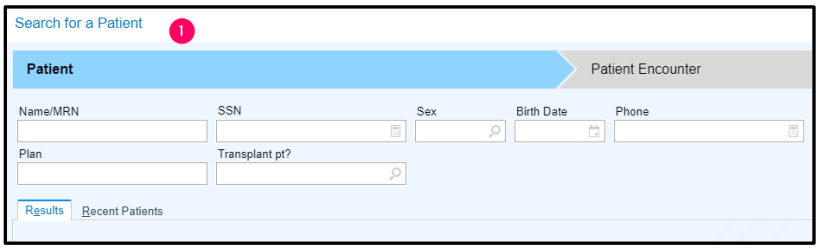
1. Click on the In Basket workspace tab in the top left of your screen then the **Staff Message** selection under My Messages
2. To take responsibility for a staff message that is in your pool, click on the baton.

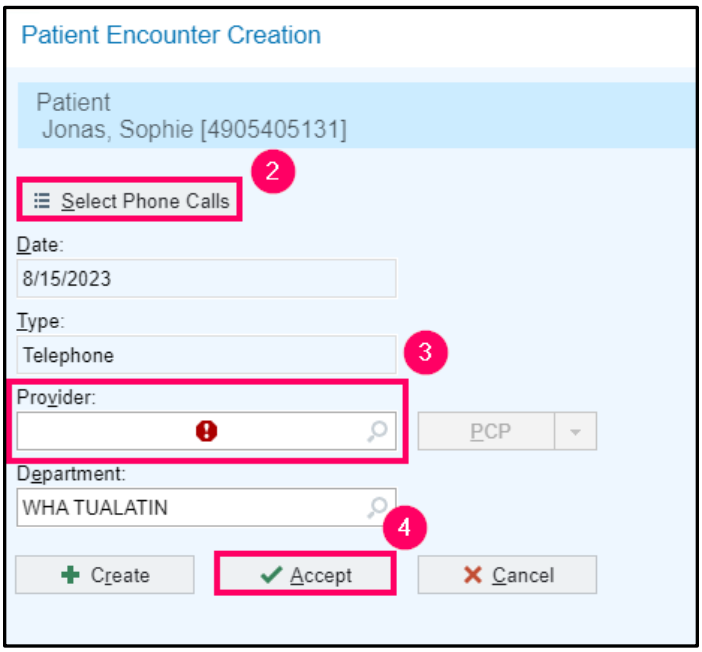
   This will turn to a for your view and a – for anyone else looking at the pool. To put the message back in the pool if you're not able to work on it, click on the
3. If you do not plan on keeping and working on a staff message after clicking on it, make sure you right click on the message and select **Mark as new**



4. Here you can Reply, Reply All or Forward the message
5. You can convert the Staff Message into a Telephone Encounter if needed
6. When you are finished with all you need to do with the Staff Message, you can "Done" it to remove it from your In Basket
7. If you need to see any Staff messages that you have clicked "Done" on, click the **Completed Work** selection the bottom left of the screen. This will house all your completed messages/In Basket work

Telephone Encounter and Medication Refill Request

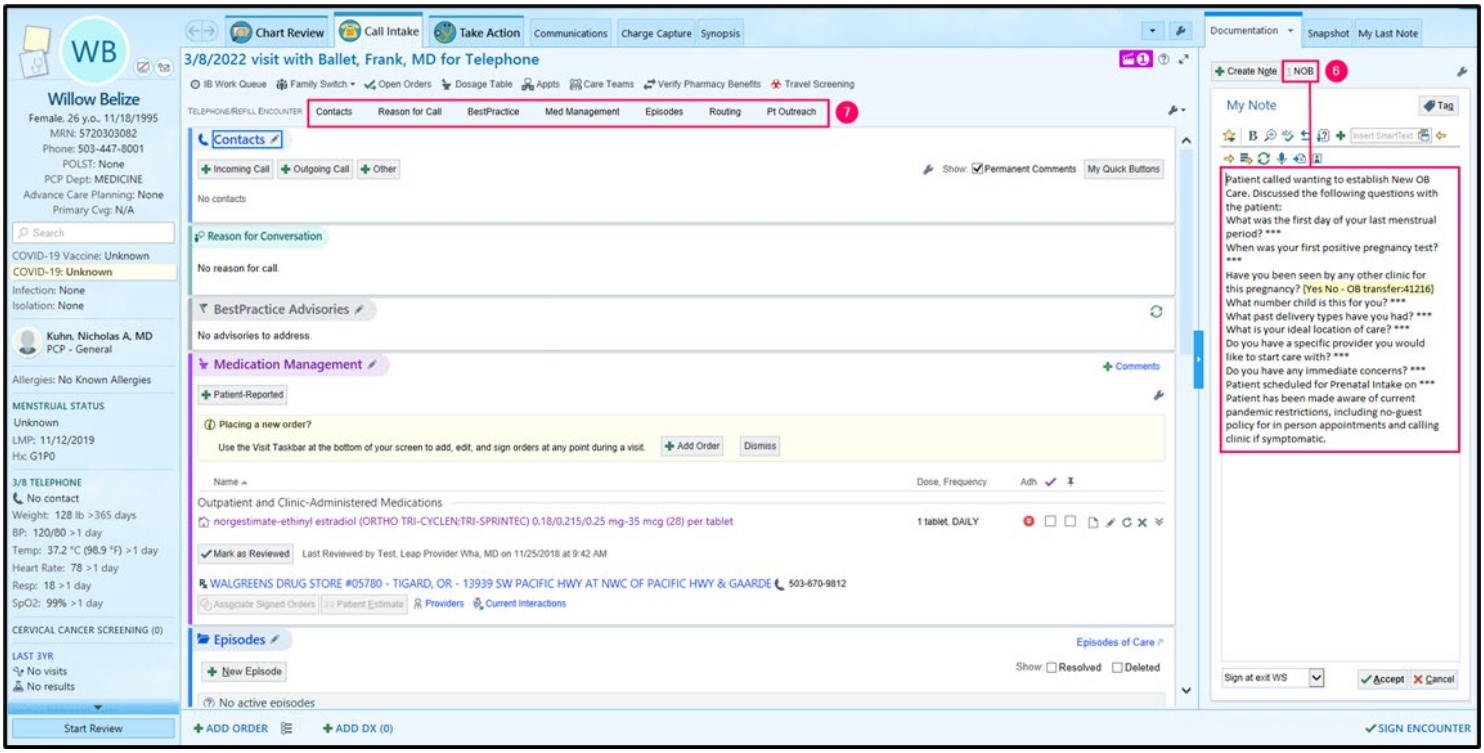






Click on the Telephone Encounter button or Refill Medication button as needed. They should be pre-loaded on your tool bar or under the Epic button under *Patient Care*

1. Look up the patient. If there are already Open Encounters in the Chart, they will appear first. Make sure to add to any relating Open Encounters as applicable vs creating a new encounter
2. Click Select Phone Calls
3. If Add the patient's **WHA provider**. If unknown, add *Unknown Provider*
4. Click **Accept** to open the Encounter
5. If you cannot see the Sidebar on the right-hand side of the screen, click on the **little blue arrow** to open up



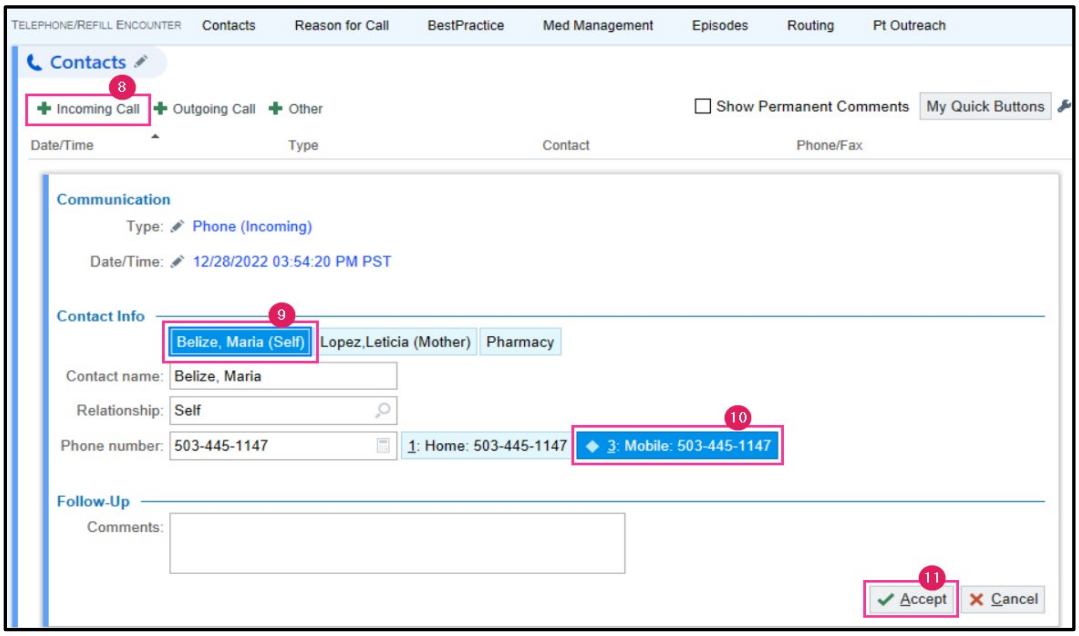
6. **Document the reason** for the patient's call. Use a Smart Phrase if applicable Smart Phrases are also called "Dot Phrases" because they start with a dot (period) – See our section on *Smart Phrases* on how to add new ones

7. Use this toolbar to navigate through the Telephone Encounter and Refill Medication request. You will need to complete these other three sections:

   ***Contacts**

   ***Reason for Conversation**

   ***Routing**



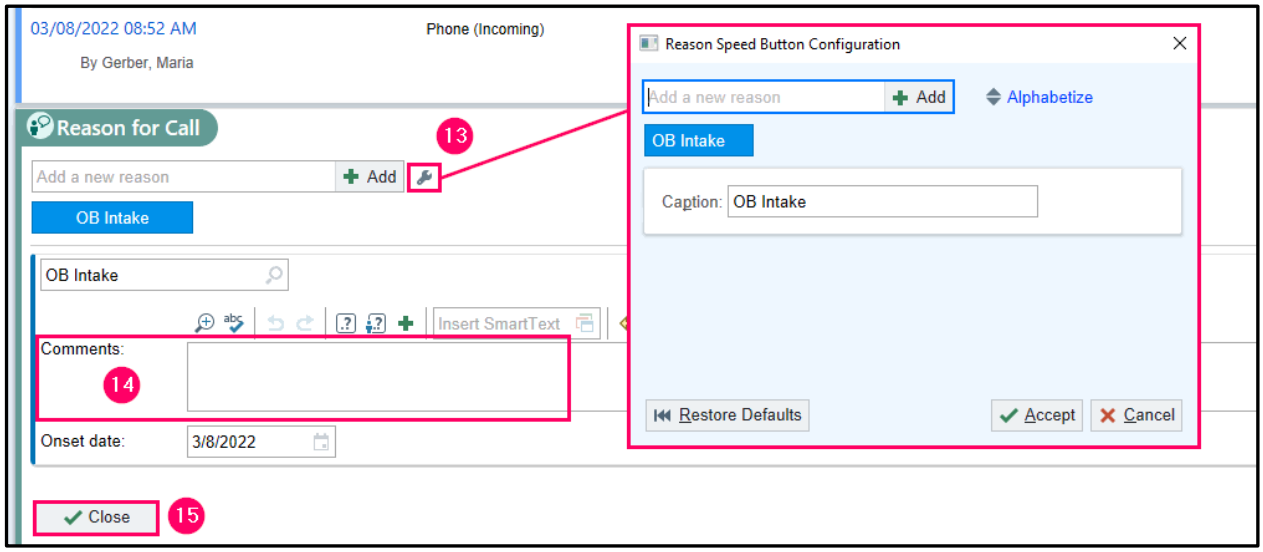
8. Once you click on **Contacts** in the toolbar, click on what type of call it is *Example: Incoming*

9. Indicate who called. *Example: click on the "Self" button if it was the patient who called*

10. Indicate the phone number the recipient should call the patient back at by clicking on the appropriate phone number button

11. Click **Accept** once finished





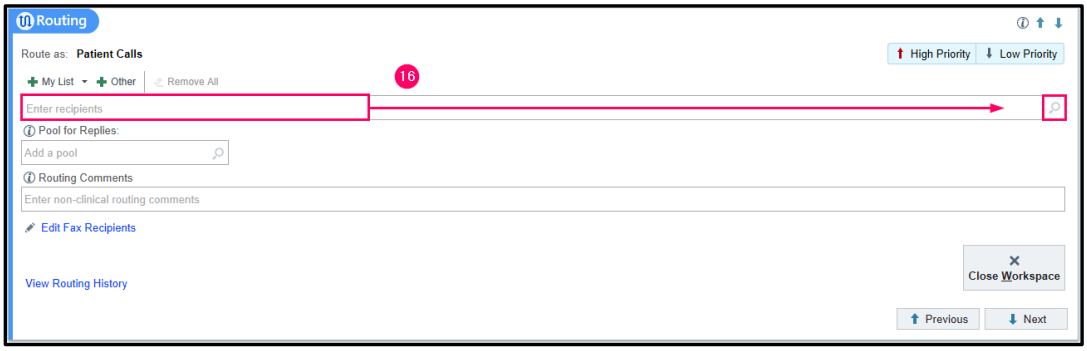
12. Click on **Reason for Conversation** in the toolbar and indicate the reason for the Telephone Encounter (Note: OB Intake should always be chosen for NOBs)

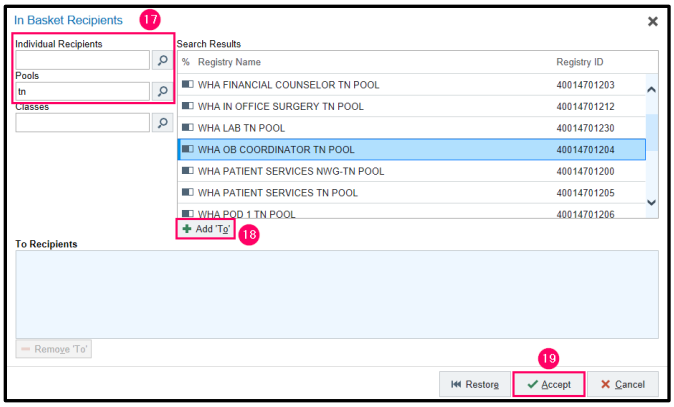
13. You can search or you can select the Preference List to search for more options within the database

14. After adding a Reason, it will appear within the encounter. To add Reason Speed Buttons, select the **wrench icon** and search the database. You can add as many Speed Buttons as needed and change the captions from this window. Click **Accept** once done

15. **Add** a short brief comment – which will be the **title of your Telephone Encounter** in the chart

16. Select **Close** once the Reason and Comment has been set







16. Click on **Routing** in the toolbar and then click on the **magnifying glass icon**

17. Search and select the pool or individual that you are needing to send your message to

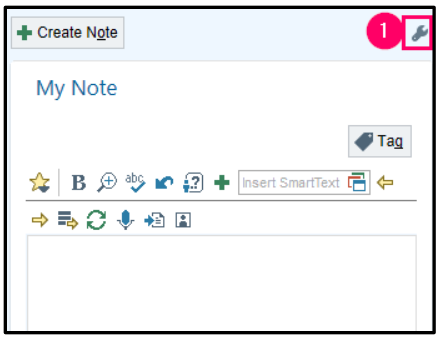
18. Click **Add To**

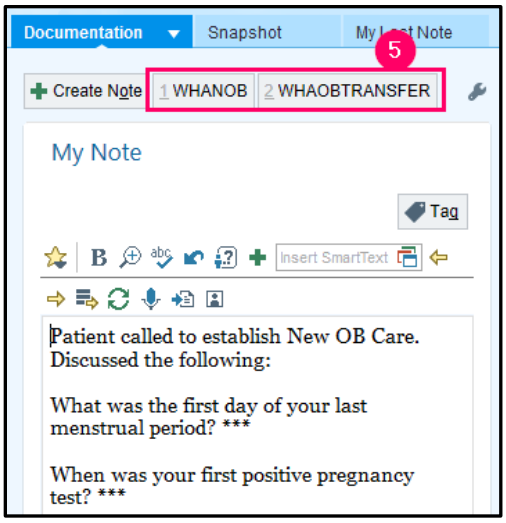
19. Click **Accept**

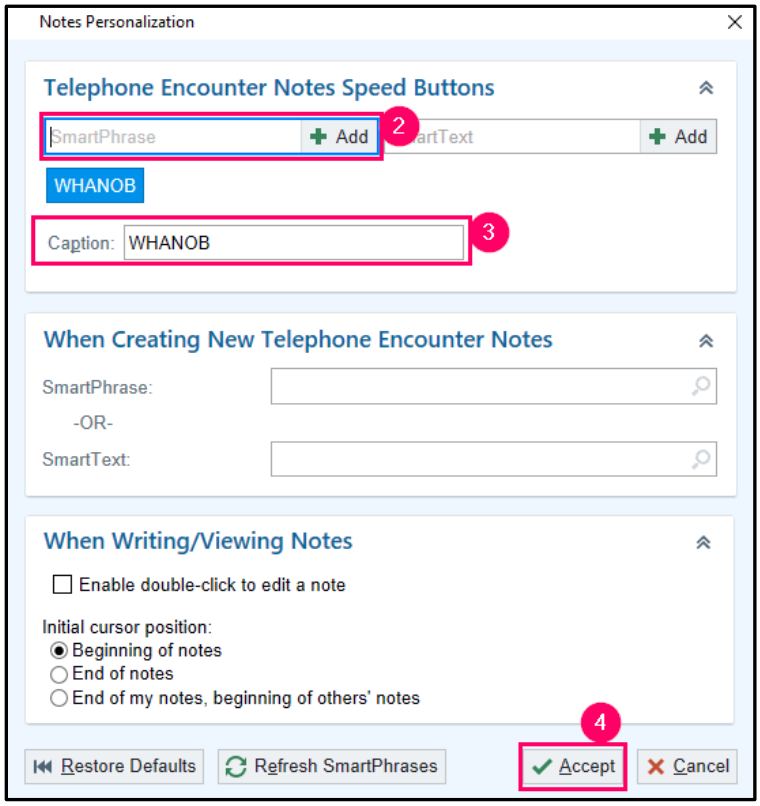
20. If you want any replies to this encounter to be routed to a specific Pool vs. yourself directly, you can enter that Pool under the **Pool for replies** section (*similar to Step 15*)

21. Click **Send and Close Workspace** to send the Telephone Encounter or Refill Medication request to your recipient(s)

Creating Smart Phrase Speed Buttons







1. Click on the **wrench icon** on the Documentation pane

2. Type in the Smart Phrase – Do Not include the period at the beginning. *Example:* WHANOB

3. Title your speed button (this is just for you, so you can name it whatever works best for you). *Example: NOB or New OB*. To add another Speed Button, click **Add** again

4. Click **Accept**

5. Click on your new speed button to populate your Smart Phrase and begin documenting

   *Use F2 on your keyboard to move through wildcards ***. Delete any wildcards *** that are not needed. If you do not complete or delete all wildcards ***, the encounter cannot be fully closed

OB / GYN Scheduling

Ultrasound Guide

WHA Ultrasound Scheduling Guide effective 11/2025		
OB US Order Name	**Singleton Time**	**Twins Time**
Dating/Viability - *(≤ 13.6 wks) - Do NOT schedule from 7-8am*	50	50
LIMITED or DETAILED First tri. anatomy w/ NT - (12-13.6 wks) - *Do NOT schedule from 7-8am*	50	60 (*if known Twins)
Sequential screening - (12 - 13.6wks) - *Do NOT schedule from 7-8am*	50	60
Dating/viability - (≥ 14.0 weeks) - *Do NOT schedule from 7-8am*	60	120 (*if known Twins)
TVCL (Transvaginal Cervical Length) only (16-24 wks)	50	50
Anatomy (≥ 14 wks) Routine *or* Detailed	60	120
-Including TVCL (≥16 wks- 24 wks)	70	130
F/up anatomy - limited (suboptimal views)	50	60
- Including TVCL (≥16 wks - 24 wks)	60	70
F/up anatomy and/or growth - *no prior* WHA *Anatomy* study	60	120
F/up growth / Fup anatomy + growth	50	60
- Including BPP	50	60
F/up placental location - Including transvaginal (≥32 wks) (Low lying placenta, placenta previa, vasa previa, placenta accreta)	60	70
Mono/di twin early anatomy (16 wks)/TTTS surveillance		120
Mono/di twin f/u anatomy/growth (20 wks)/TTTS surveillance		60
Mono/di twin TTTS surveillance - routine		50
Mono/di twin surveillance - routine w/ growth and/or BPP		60
BPP only	50	60
AFI only	50	50
Fetal position	50	50

GYN US Order Name	Time	Special Instructions
Pelvic US	60	Schedule follow up Provider appt 3+ days out
US guided GYN Procedure (IUD insertion/removal)	60	MD to join no sooner than 15-30 mins into exam, for 30 min
US guided GYN Procedure (endometrial biopsy)	60	
SIS w/complete pelvic US done in last 6 months	70	

Helpful Contacts:	What they can help with:
US Scheduling Line	Help Schedule
NWP US Scheduling Coordinator CQ Line	Approval for Spots

Call NWP Scheduling before scheduling on a NWP US room

Call NWP PS Team if you have an Ultrasound Order that is not listed - Example: Fetal Echocardiogram

Important Note :

If a patient is having their first US with WHA and they are ≥ 14 weeks they need a 60 min US unless more time is notated in guide

Color Key :

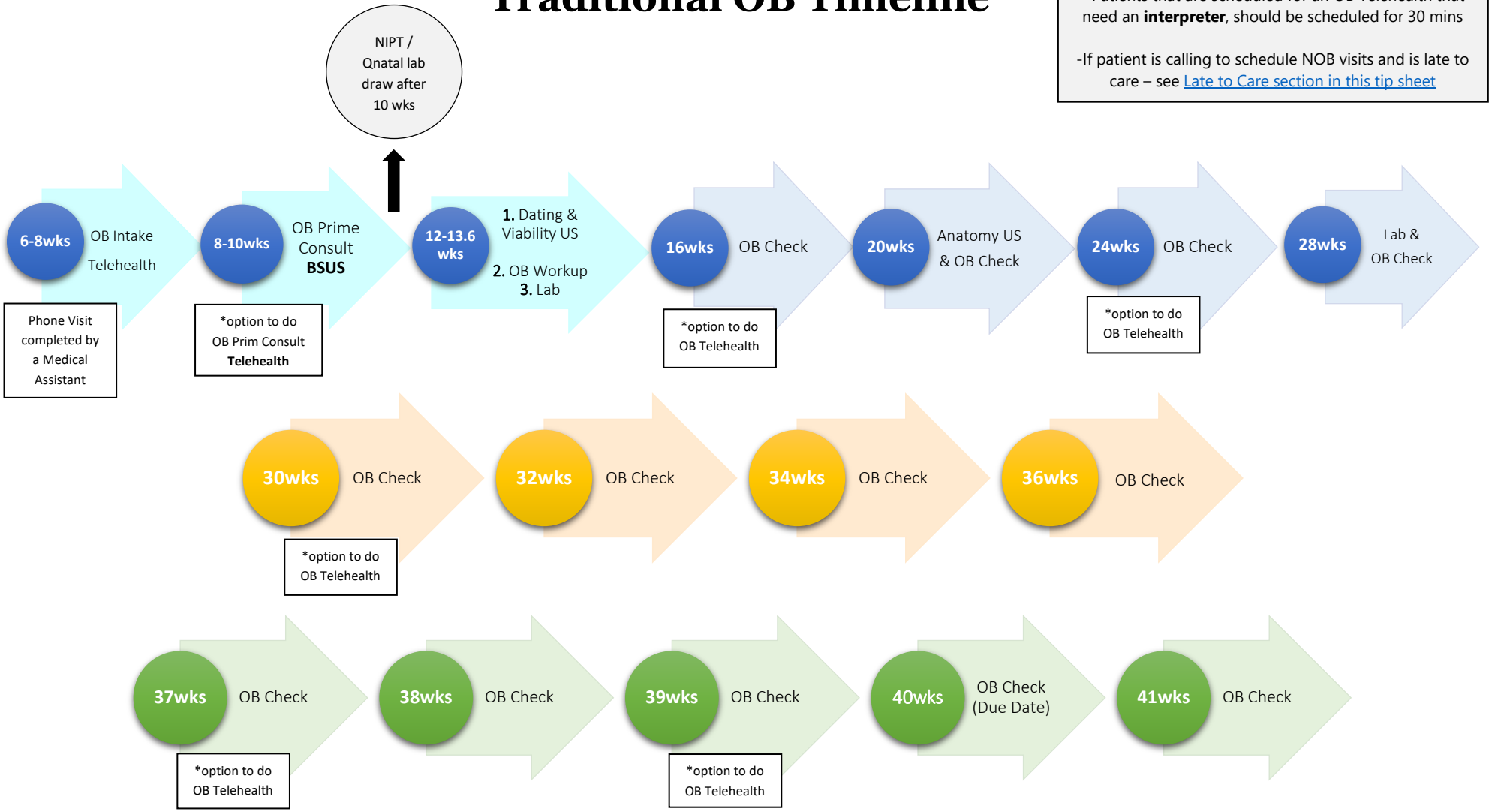
OB Complications (Cramping/Bleeding) and exams indicated by this color require a same day follow up appt with a Provider. These exams can also be scheduled together as one exam

Traditional OB Timeline

Note:
- Need to leave a 30 min gap after **each OB US** before the provider visit

- Patients that are scheduled for an OB Telehealth that need an **interpreter**, should be scheduled for 30 mins

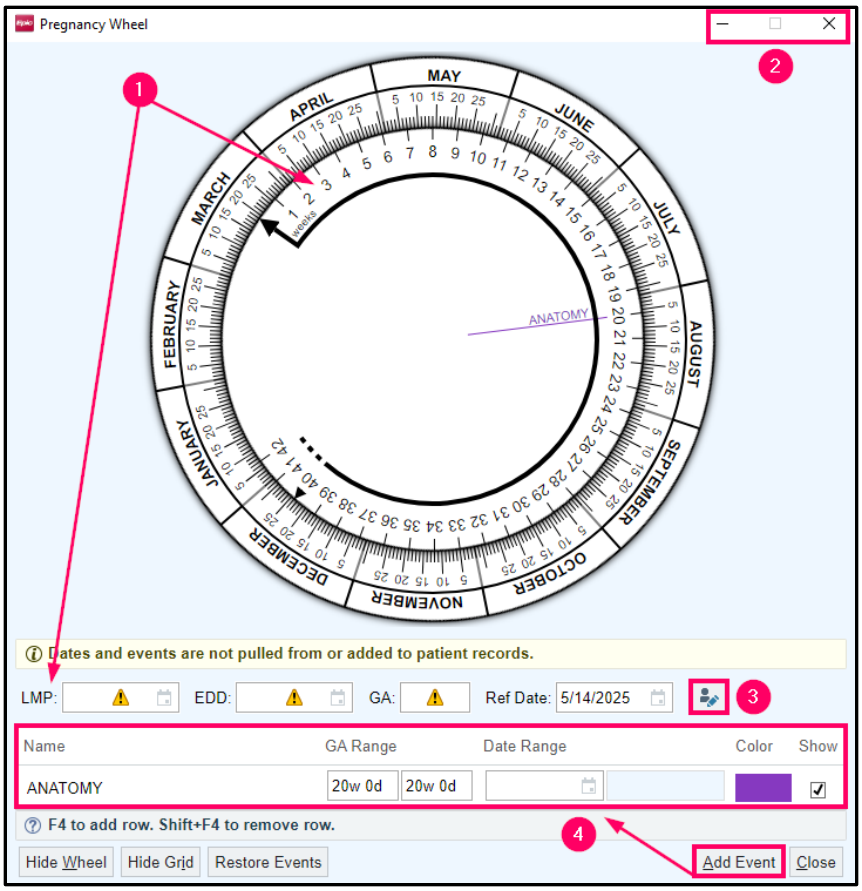
-If patient is calling to schedule NOB visits and is late to care – see Late to Care section in this tip sheet



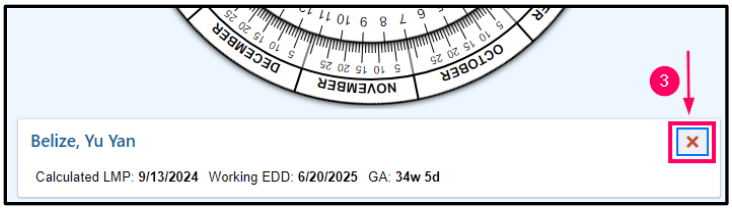
Traditional OB Timeline

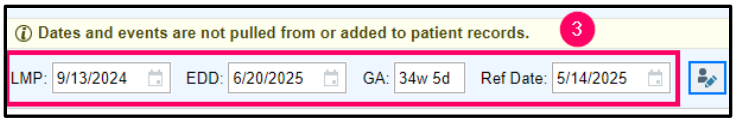


Pregnancy Wheel









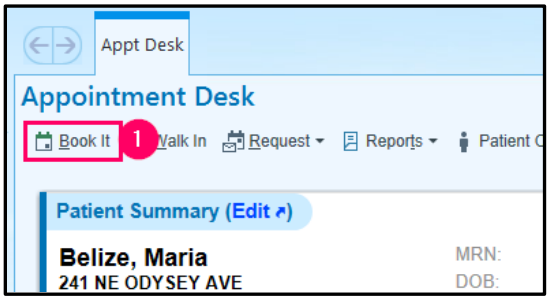
1. Click and spin the wheel weeks to update the LMP/EDD/GA dates or add this info manually under the yields (*Example: Adding the LMP will calculate the EDD & GA*) – See step 3 for pulling in current pregnancy info for a specific patient

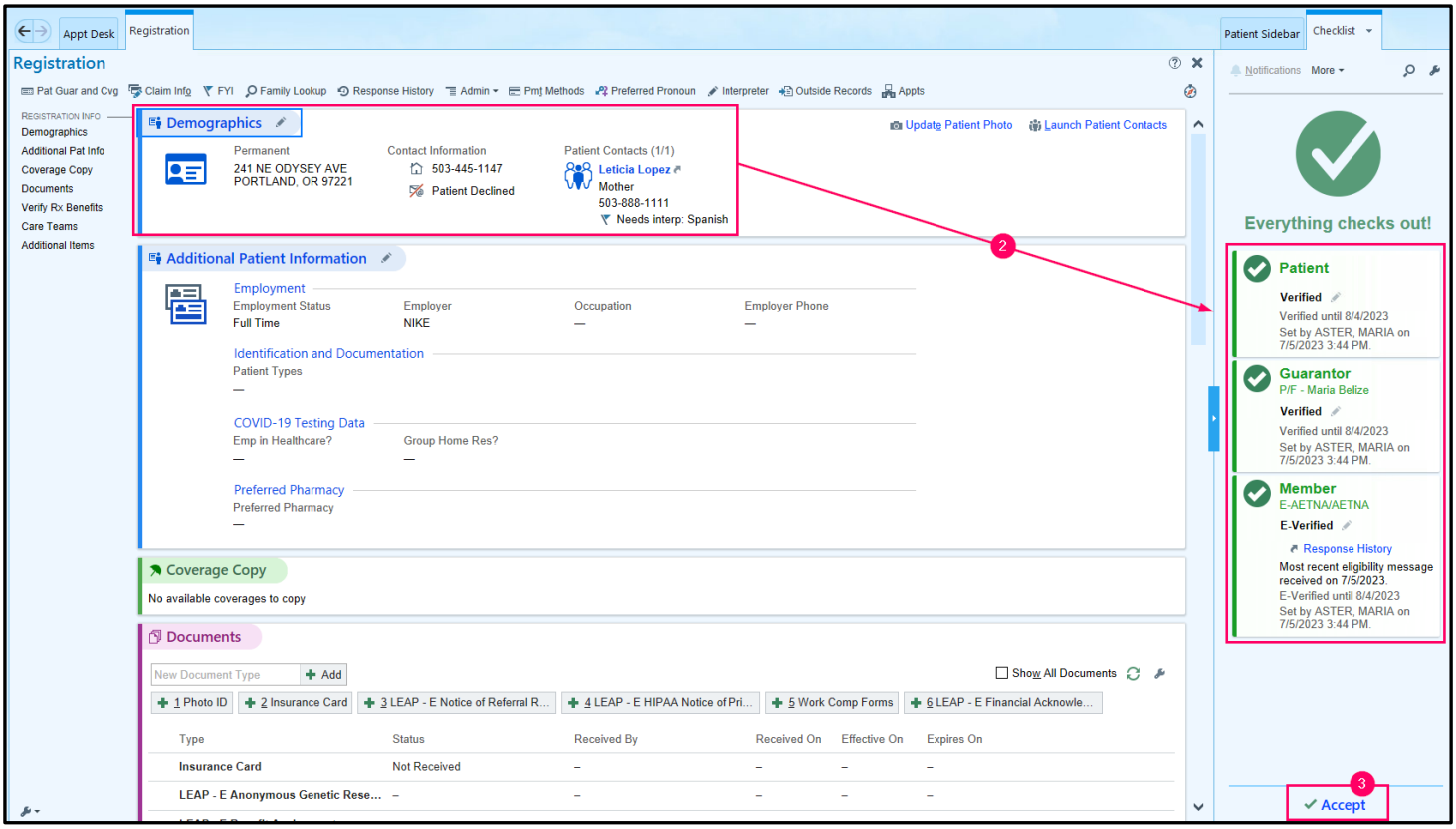
2. The wheel opens as a **separate Epic window**. It can be minimized during scheduling or any other work in the background. Select the window from your desktop task bar to pull it back up when needed. Click the X to close

3. If the patient has current pregnancy info under their chart, it can be pulled in.
   - Click the **select patient** button to look up and pull in the patient. Once the info is pulled in, make sure to click the **red X** to use all of the wheel features
   - If using the Pregnancy Wheel from the **Chart view**, this info will auto pull in
   - Use the **Ref Date** after scheduling to calculate GA at time of the visit when applicable

4. **Events** can be added along with the GA Range (*It is recommended to pull these from the OB Timeline on page 72*) to help with scheduling.
   - Choose a **color** and click the **Show box** for the event appear on the pregnancy wheel

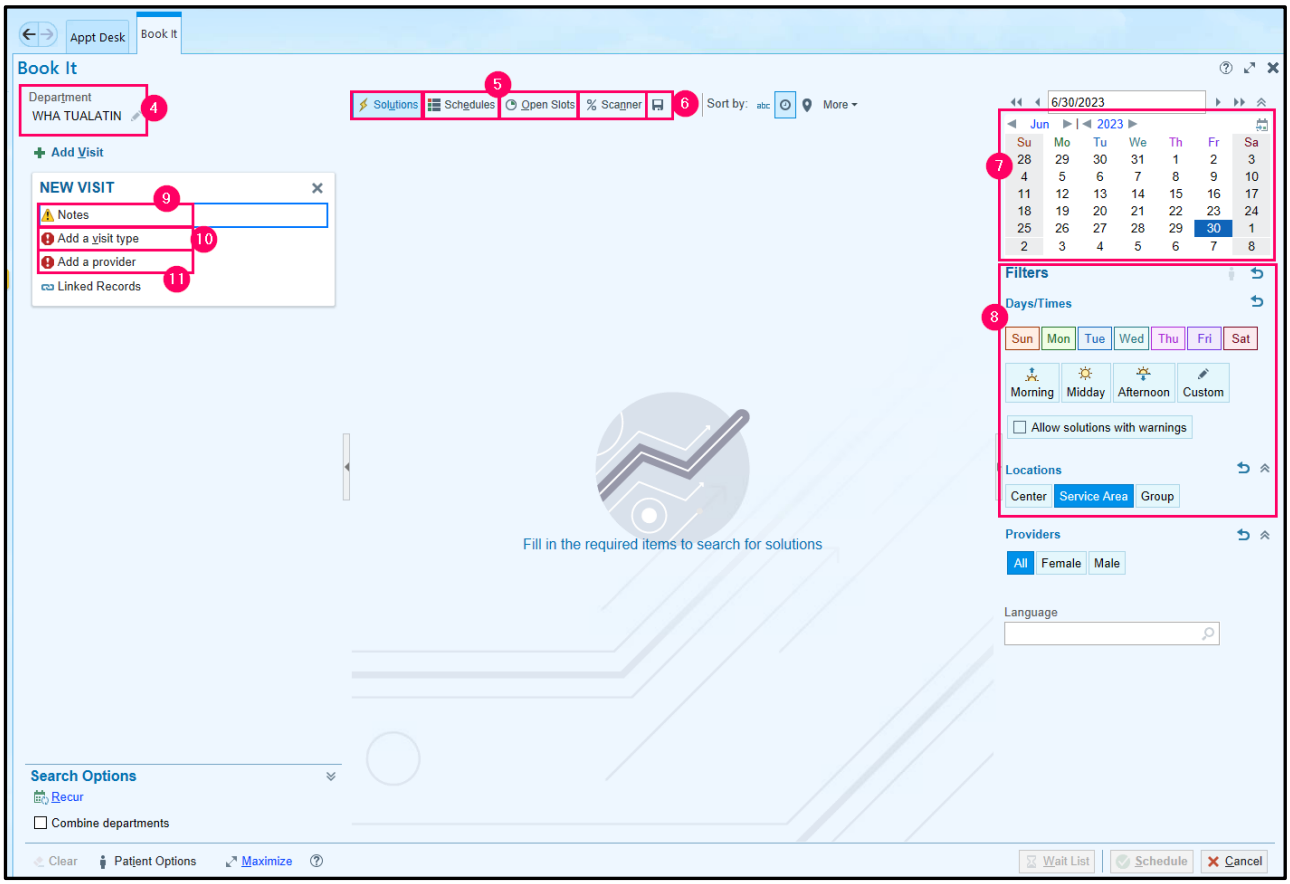
Scheduling – Basics





1. Click on **Book It**
2. Verify patient's address, billing address (guarantor account) and insurance then

   Verify the Checklist by clicking any  - Fill out/Complete any missing information before moving onto Scheduling
3. Click **Accept**



4. If the patient you are scheduling needs to be booked in a different clinic then change the Department before entering any other scheduling details

5. There are different views within Book It

   - **Solutions:** Will use an auto-search function to find the next available appts
   - **Schedules:** Will pull in the clinicians/resources schedule so that you can manual search for appts
   - **Open Slots:** Will list each available slot for the clinician/resource you entered
   - **% Scanner:** Will give a day by day overview of how full percentage wise

6. To save one of the views, click the **floppy disc** icon so that it pulls open first each time you click Book It

7. Select the **date** that you want to start your search on

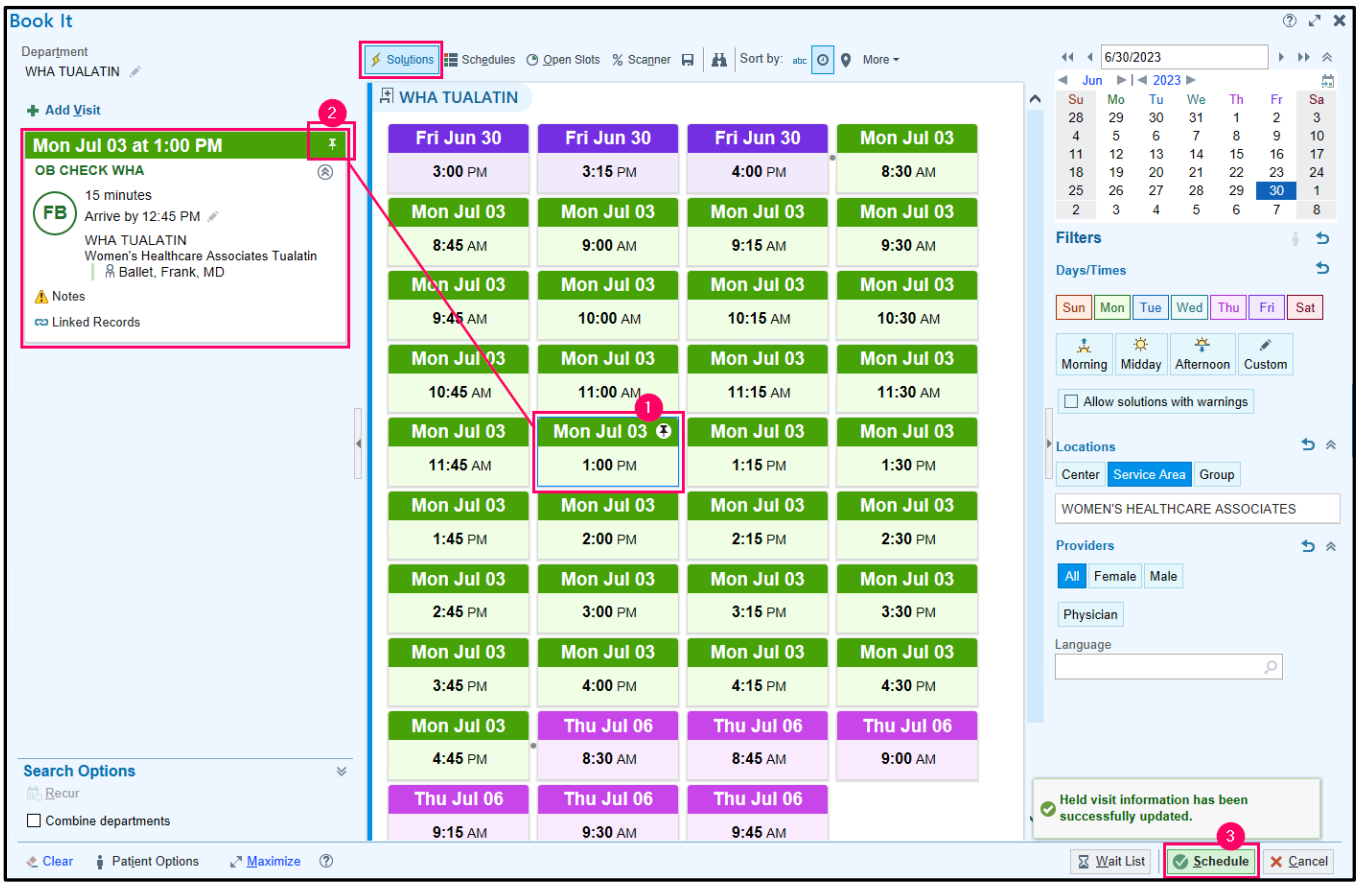
8. Utilize helpful search **Filters** as needed (*Days of the week, etc.*)

9. Enter the **Appointment Notes** to indicate what the patient is coming in for

10. Enter the **Visit Type** that the patient is being seen for

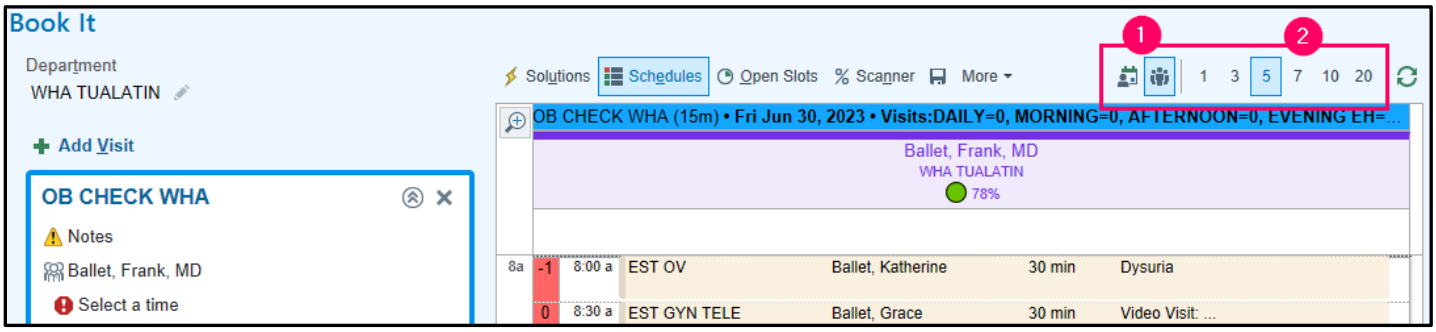
11. Enter the **Provider(s)/Resource(s)** name that you are going to schedule the patient with

Solutions View for Scheduling (*Auto-Search*)



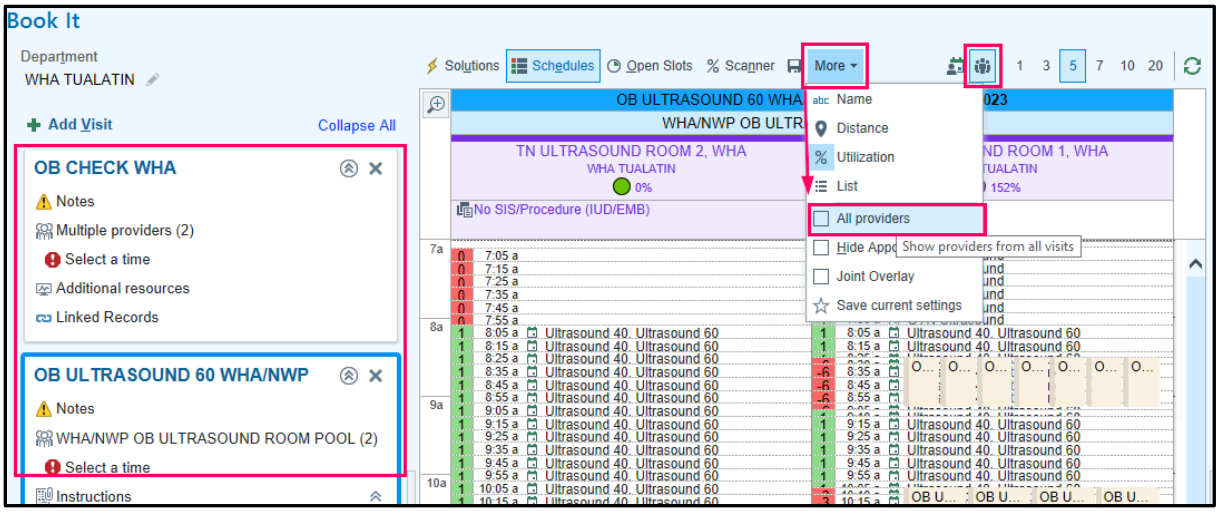
1. Within the Solutions view, to choose an appt click on it once and it will be "held" with a push pin
2. The Visit Card will change the color of that day and fill in the information of the appt that you selected. To unselect an appt you "held", click the **push-pin** within the Visit Card
3. Click **Schedule**

Schedules View for Scheduling (*Manual Search*)

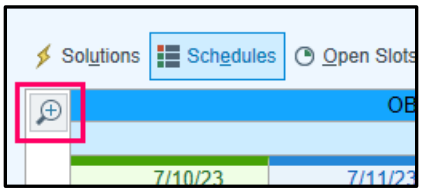


1. Within the Schedules view, there are 2 different layout options.
   The left icon is a single provider/resource option, which allows you to view one provider/resource for multiple days.

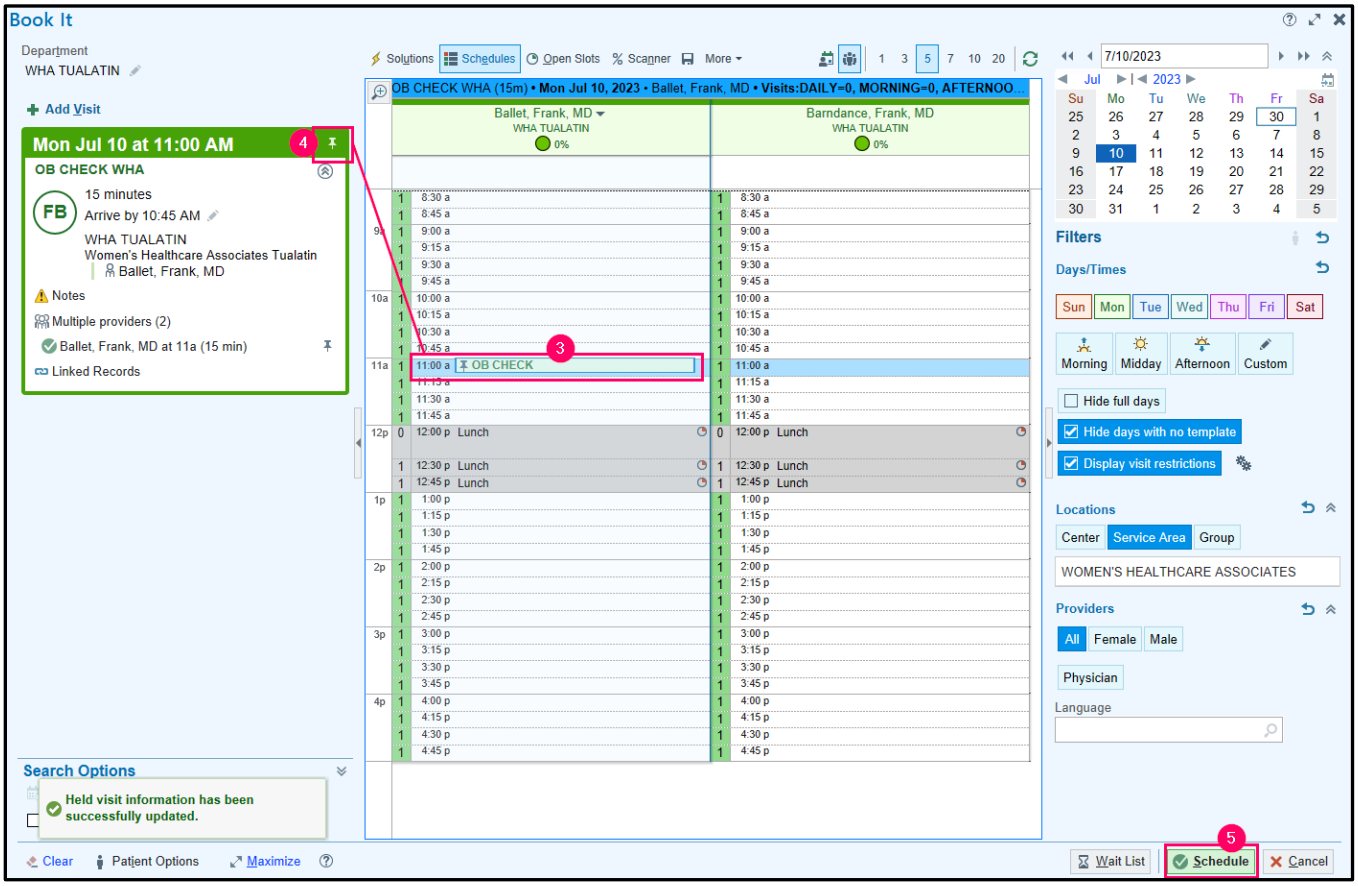
   The right icon is multiple providers/resources for a single day.
2. The numbers to the right of the 2 Schedules icons, correlate to the specific layout that is selected. The left icon = numbers decide how many days to view. The right icon = how many providers/clinicians to view



If you are in the multiple provider Schedules layout and are using more than one Visit Card (scheduling more than one visit type) and you do **NOT** see all of the provider(s)/resource(s) listed, click the **More** drop down menu and check the **All Providers box**

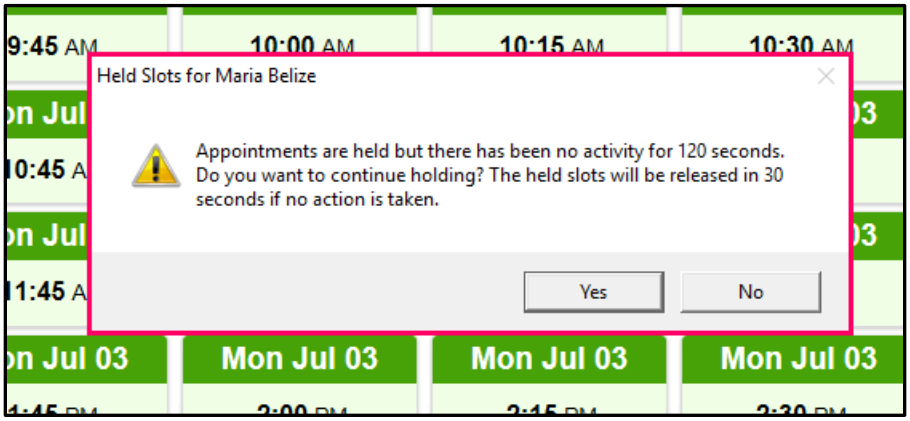


Note: To expand the Schedules view up or down / left or right, click on the magnifying glass

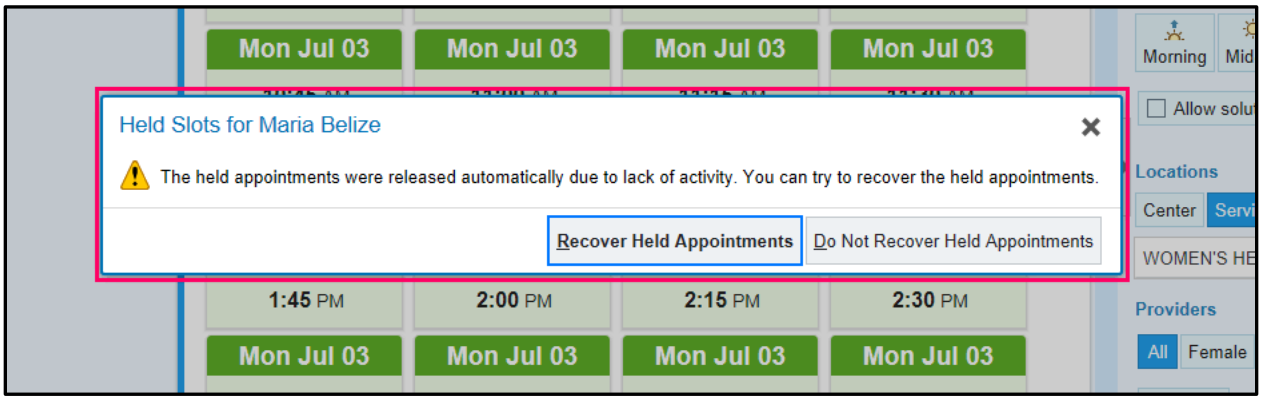


3. Within the Schedules view, to choose an appt double click on the "start" time of the visit and it will be "held" with a push pin
4. The Visit Card will change the color of that day and fill in the information of the appt that you selected. To unselect an appt you "held", click the **push-pin** within the Visit Card
5. Click **Schedule**

Warnings for Appt Release during Scheduling



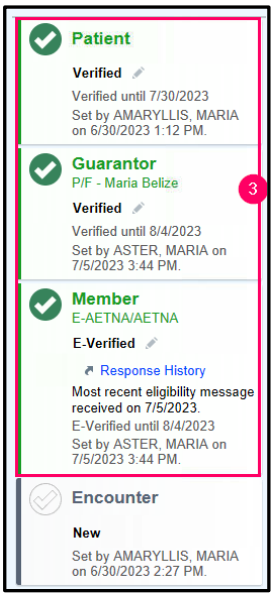
If you have selected/held an appt and do not click the Schedule button, after 120 seconds you will get this warning

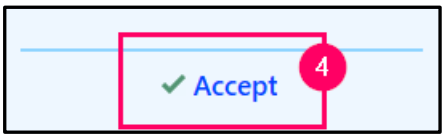


After you see that initial warning, you will get a warning that lets you know that your held appt(s) were released. You can click **Recover Held Appointments** if the visit(s) are still available

Appt Review / Completing the Scheduling Process







1. Review any patient instructions
2. Click **Schedule**
3. Verify any indicated information
   - Review any ⚠ and ❗ signs. The "Encounter" verification can wait until the patient comes in as most of it regards paperwork they will need to fill out

4. Click **Accept** to get back to the patient's Appointment Desk

Chaperone Info & Scripting for Schedulers

Situation / Background: It is a requirement for WHA to offer a chaperone for any sensitive/physical exam. For those visit types, more directions will pull into the Patient Instructions section of the Appointment Review window (see screenshot below for example). Adding this information to the Appt Review screen after scheduling, will automatically send to the Patient's Myhealth so they can click on the imbedded link and learn more from the Oregon Medical Board informative brochure. Schedulers could be the first point of contact regarding informing the patient of this chaperone option and will need to use the scripting below to help guide them.

Scripting: *"A trained medical chaperone will be offered for your comfort at the time of the visit, if your provider does a sensitive exam. If you have any additional questions about a sensitive exam or the use of a medical chaperone, please ask your provider."*

If patients have any further questions, inform the patient you will be sending a message to a clinical team member to call them back. Then start and send a Telephone Encounter to the Pod of the clinician the patient's is seeing.



FAQ:

Question: Where does the link that is embedded in the Patient Instructions go?

Answer: What to expect during a physical exam?

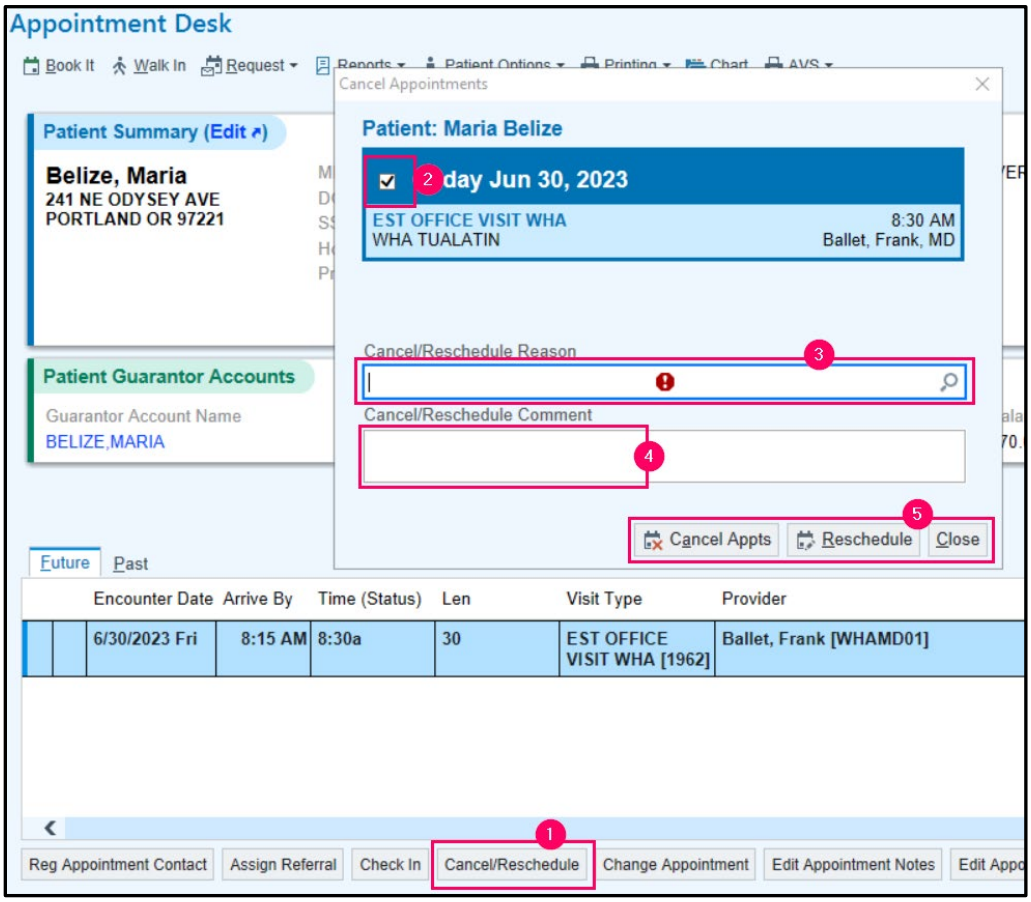
Question: What visit types will I see these new Patient Instructions prompted for?

OB CHECK W/ NST WHA	OPEN SCHED OFFICE VISIT WHA
POST OP WHA	URODYNAMICS WHA
MFM CONSULT NWP	NP COMMON CONCERNS WHA
MOD RISK CONSULT NWP	EST COMMON CONCERNS WHA
MOD RISK FOLLOW UP NWP	OPEN SCHED COMMON CONCERNS WHA
UROGYNECOLOGY WHA	ER FOLLOW UP WHA
OB CHECK NWP	EST WELLNESS VISIT WHA
MYHEALTH OFFICE VISIT WHA	EST OFFICE VISIT WHA
MYHEALTH WELLNESS WHA	NP WELLNESS VISIT WHA
OB WORKUP W/ BEDSIDE US WHA	NP OFFICE VISIT WHA
MOD RISK FOLLOW UP W/ NST NWP	REPEAT PAP WHA
OB TRIAGE W/ NST WHA	OB TRIAGE WHA
PRE OP WHA	GYN TRIAGE WHA
OB PRIM W/ BEDSIDE US WHA	INITIAL PRENATAL APPT WHA
PESSARY CHECK WHA	OB CHECK WHA
REEST WELLNESS VISIT WHA	OB WORKUP WHA
REEST OFFICE VISIT WHA	POSTPARTUM WHA

Question: What if I can't answer a question the patient has about being offered a chaperone during their appointment?

Answer: Inform the patient you will be sending a message to a clinical team member to call them back. Then start and send a Telephone Encounter to the Pod of the clinician the patient's is seeing.

Cancel vs. Reschedule

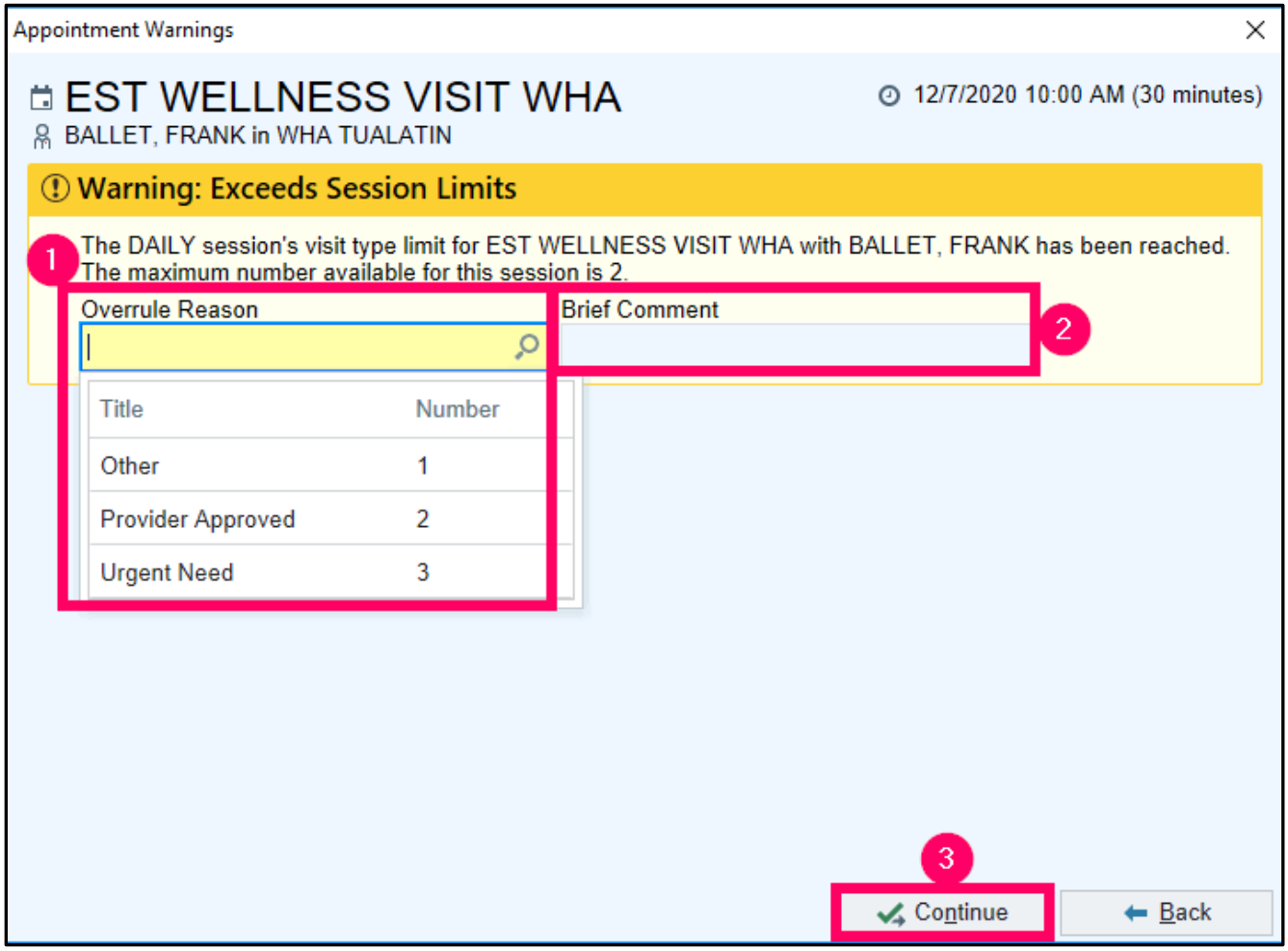


1. Click the Cancel / Reschedule button after selecting which appts to edit
2. Check mark the box(es) next to the appointment(s) that need to be canceled or rescheduled. If you selected several/linked visits from the Appointment Desk, they will be check marked here.
3. Indicate the reason for Canceling / Rescheduling the appointment(s)
4. Always **add a comment** if you cancel a visit. It helps other departments with reporting
5. Click on **Cancel Appts** or **Reschedule**

- It is recommended to click on "Reschedule" unless you know for sure that the patient wants to cancel their appointment and not reschedule it. Clicking "Reschedule" will allow you to search for new appointment(s) while keeping the currently scheduled appointment(s) on hold in case the patient changes their mind. As soon as you reschedule to the new appointment(s) the old appointment(s) will be canceled and released for someone else to schedule on

  Note: If you cancel, the appt(s) will be moved to the "Past" tab on Appt Desk

Appointment Warning Overrule Reasons

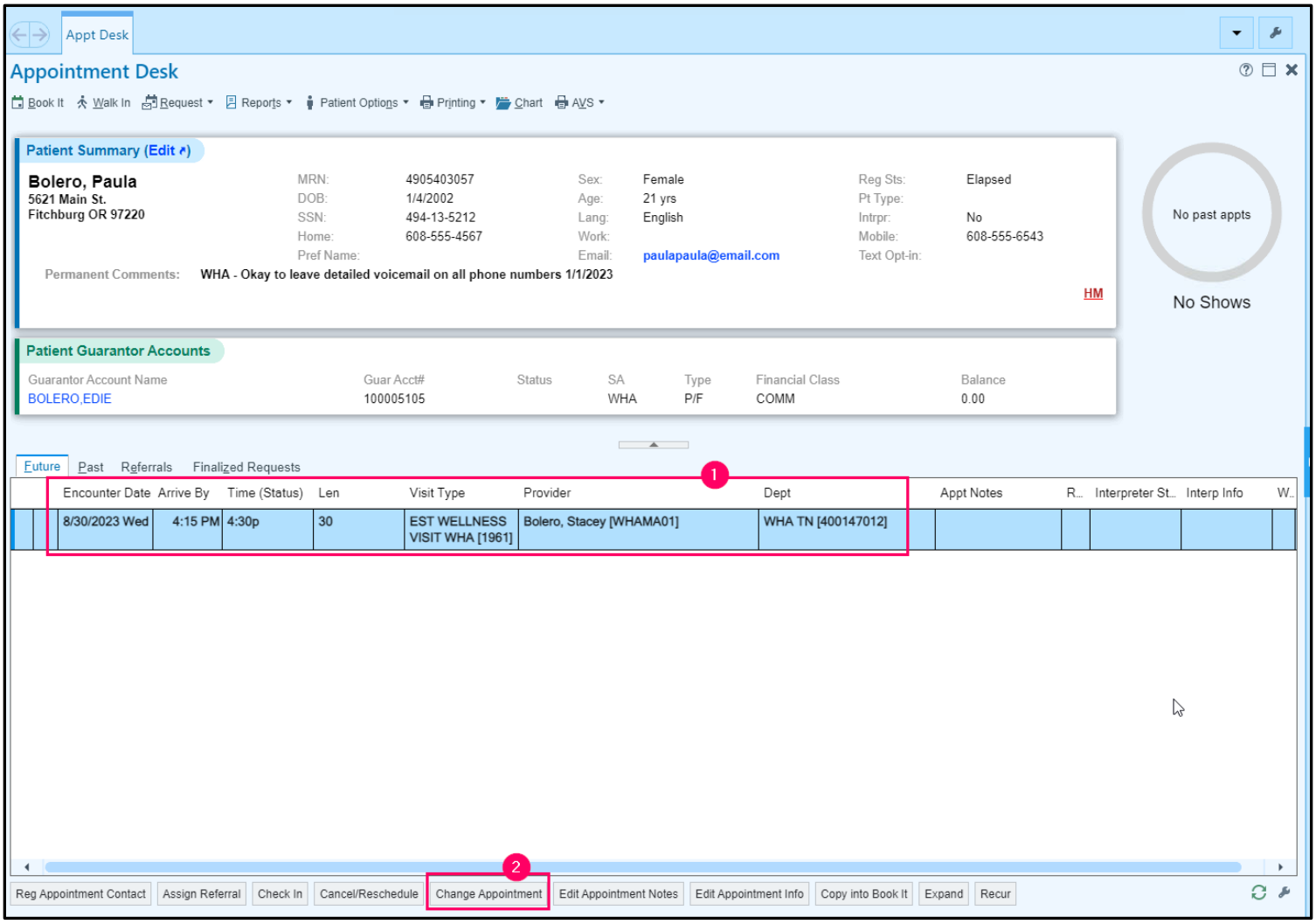


Some clinicians will have visit limitations built into their template. In this example, Dr. Ballet is allocated to only see 2 wellness exams on this day. By scheduling another wellness exam on that day it will bring up this Appointment Warning. You have to have a reason of why you are overruling this template limit. *Note: If a session limit is reached, you will not see options for that day if using Auto Search.*

IMPORTANT: Only overrule if you have permission from the Provider, MA/RN, PSR or Supervisor.

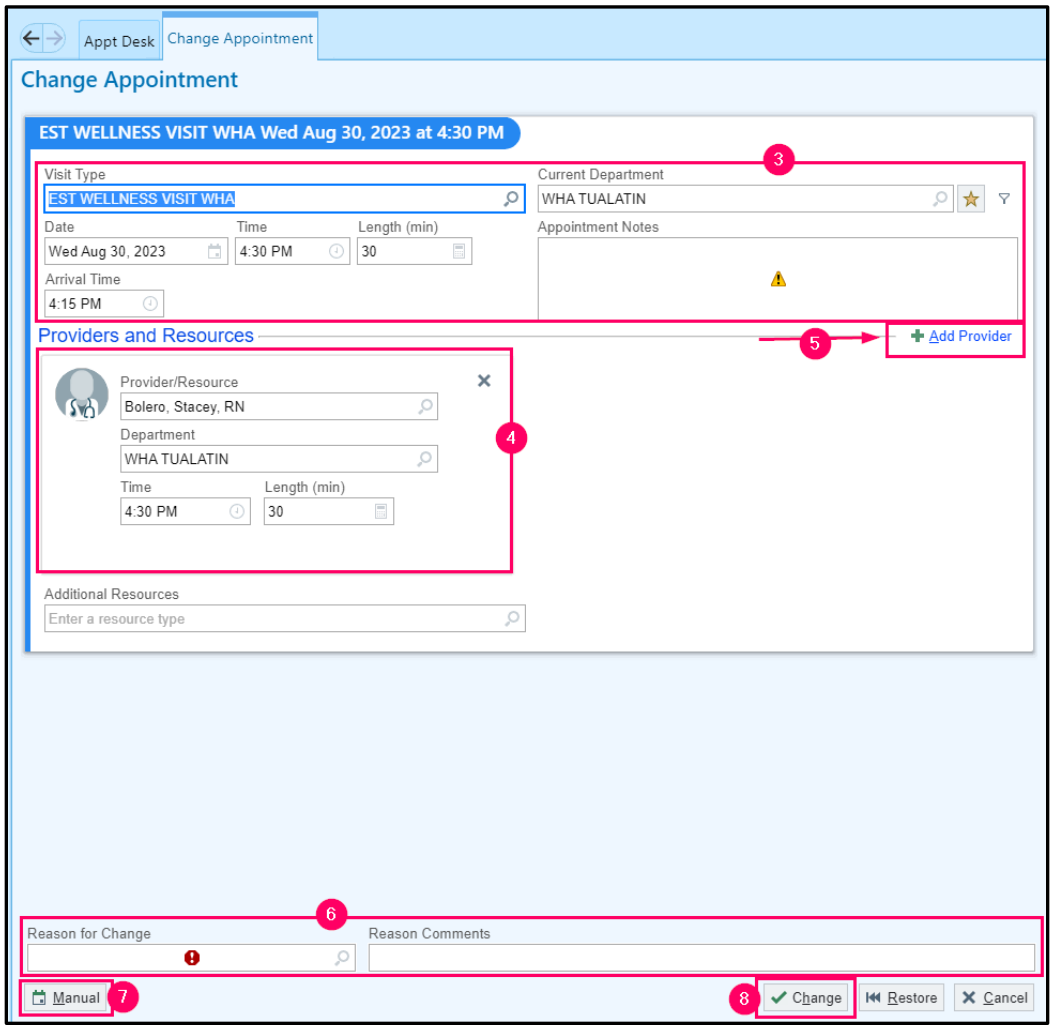
1. Click the magnifying glass, which will have this drop-down menu to select 1 of 3 reasons
2. Each Overrule Reason requires a comment to explain why you are bypassing this template limit
3. Click **Continue**

Using the Change Appointment function on the Appt Desk



The Change Appointment function within Epic is very help for quick edits or reschedules, as long as you are certain that the time is available on the Provider/Resource templates

1. Click on the visit within the Appt Desk that you would like to edit/change
2. Click **Change Appointment** on the bottom toolbar



3. This section is to edit what visit type, date, Dept. or Appt Notes you are needing to change

4. This section is to edit which specific provider/resource/dept to change based on what is currently scheduled

5. You can add additional resources or providers if needed by clicking on Add Provider (like an NST or Procedure Room)

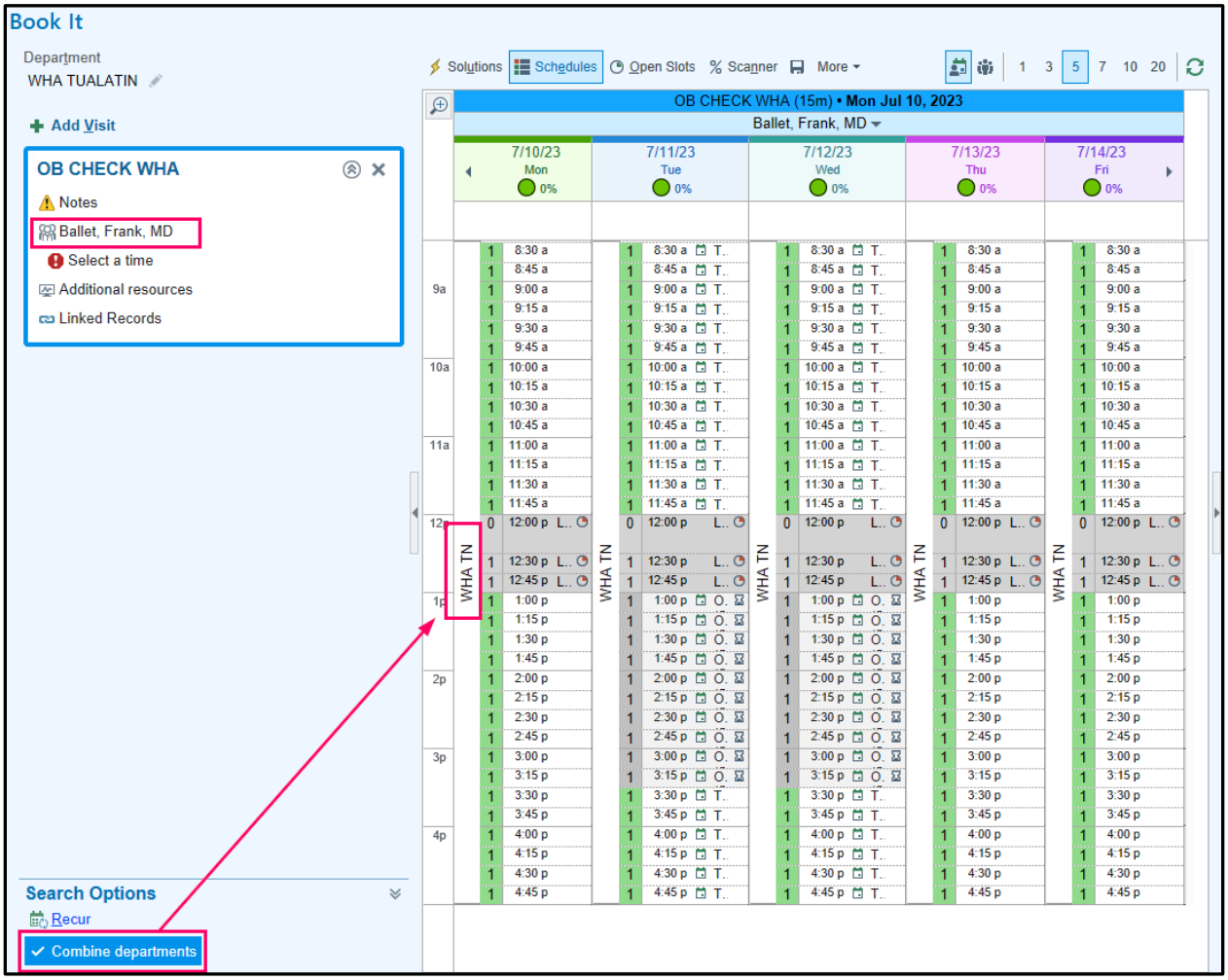
6. Each time you use the Change Appt function you are rescheduling a visit, so you will be required to put a Reason for change and a comment associated with why

7. If you aren't certain that there is time available where you would like to move the appt to, you can click Manual to move to a Manual schedule view

8. Click **Change** to finalize any edits made

Searching Across Multiple WHA Depts

When Providers work at multiple sites it can be difficult to search for openings on the correct template. Utilize the Combine Departments box in the bottom left hand corner to show the provider's template for all Departments (if they have more than one).



Scheduling - Appointment Notes

Appointment Type	Appointment Notes
Behavioral Visits	Consult = Initial Consult Return Visit = Follow Up
Circumcision	*No notes needed – indicated in visit type*
ER Follow-up	Reason for ER visit, hospital, and date seen (gestation age if pregnant and known)
Est Office Visit	Reason for visit
Est Wellness Visit	*(No notes needed, leave blank)*
Est Wellness with Problem Visit	List the Problem/Concern pt is having
GYN Triage	Reason for visit
GYN Ultrasounds	*(No notes needed, leave blank)*
Injection	Indicate what type of Injection
IUD Insert/Removal	Indicate type of IUD
Lab	*(No notes needed, leave blank)*
Mammogram	*Discussed COVID Vaccine*
Nexplanon	Insert, removal, or replacement
NP Office Visit	Reason for visit & if being referred/where
NP Wellness Visit	*(No notes needed, leave blank)*
NST	Gestational Age at time of the appt
Nurse Visit	Reason for visit, Gestational Age if applicable

OB Check	Gestational Age at time of appointment
OB Triage	Reason for visit, Gestation Age at time of appointment if applicable
OB Ultrasound	Exam Requested (*copy+paste from US order*), Gestational Age at time of appointment **or** LMP if EDD is unknown **OB US 12-13.6wk** = LMP (*Staff attaching the order will add the Exam Requested to the US appt notes*)
OB Workup	LMP date
Post-operative	Date of surgery and post-op week
Postpartum	Date of delivery
Procedure	*No notes needed – indicated in visit type*
Sonohysterogram US	*No notes needed – indicated in the visit type*
Wellness Visit	*(No notes needed, leave blank)*

Telehealth Visits	Appointment Notes
OB Telehealth Visit	*See Telehealth Visit Guide
GYN Telehealth Visit	*See Telehealth Visit Guide
New Patient GYN Telehealth	*See Telehealth Visit Guide
PP Telehealth Visit	*See Telehealth Visit Guide
BH Telehealth Visit	*See Telehealth Visit Guide

Psych Med Telehealth Visit	*See Telehealth Visit Guide
OB Intake Telehealth Visit	*See Telehealth Visit Guide
OB Prim Consult Telehealth Visit	*See Telehealth Visit Guide
Wellness Telehealth Visit	*See Telehealth Visit Guide

Scheduling – Depo Injections

The Depo Injection is usually scheduled every 12 weeks. To figure out when the patient should be scheduled next, find the last completed Depo injection visit and cross compare it to the Depo Scheduling Calendar.

Question: What if a patient needs to schedule their next Depo but it would be outside the window given by the Depo Scheduling Calendar?

Answer:

The patient can still schedule that injection visit with our office – they do not need to have a provider appointment.

The Clinical team will assess the patient and their situation and do a pregnancy test if needed.

Question:

What if the patient hasn't been seen specifically by a WHA provider within a year?

Answer: They can still be scheduled for their Depo injection but will need a provider visit scheduled (if not already scheduled). It can be an Office Visit if the patient has any other concerns or they can schedule their yearly Wellness Visit. The provider visit does not need to be on the same day as when the patient wants their next Depo injection scheduled.

Scheduling – Est Wellness with Problem visit type

Background: Some of our WHA Providers have started seeing patients for Est Wellness visits with a GYN problem or concern.

Quick link to this visit's Scheduling Requirements section

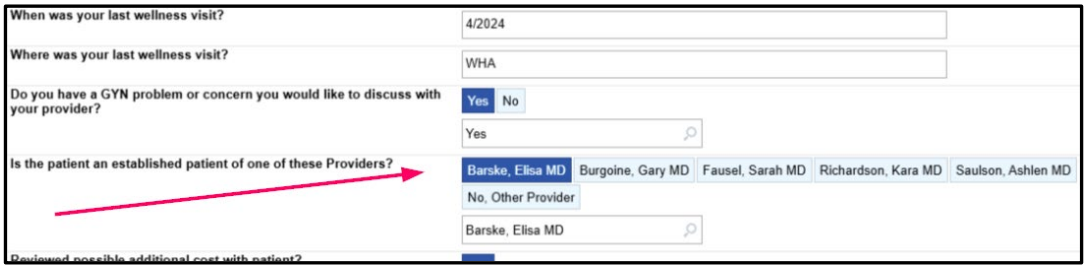
What to know about this visit type:

1. New GYN Patients are **not** included in this workflow

2. The visit should be scheduled with the patient's preferred provider (ex: if the patient doesn't already see Dr. Barske, they shouldn't schedule with Dr. Barske)

3. When scheduling, **always** start with the visit type **Est Wellness Visit**

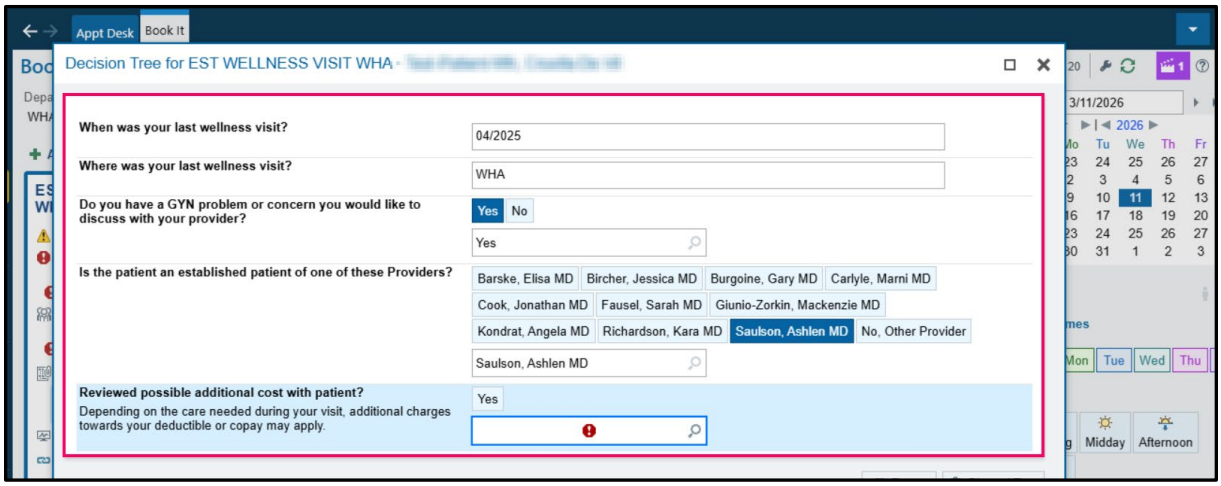
4. From there, a questionnaire will appear to help guide you if it would be appropriate to schedule the patient for the Est Wellness with Problem visit type with one of the Providers included in this workflow or



5. If the patient has a GYN problem/concern and wants to see a Provider that is not listed as an option to choose from, select "No, Other Provider" and continue to split the Est Wellness visit and Est Office Visit into **2 different appts on 2 different calendar dates**

Visit Questionnaire & Scripting Walkthrough:

Always start specifically with the Est Wellness visit type (do **not** start with the Est Wellness w/Problem visit type), a questionnaire and some helpful scripting will appear. **Scheduling Reminder:** you can click the "Combine Depts" box in the bottom left corner to see the schedule for a Provider at all their associated depts



Question 1 & 2: Answer based on the patient's last wellness visit

Question 3: Ask the patient the question listed – this is only for Problem/Concern issues **NOT** Procedures (IUD, Colpo, Leep etc. are not included).

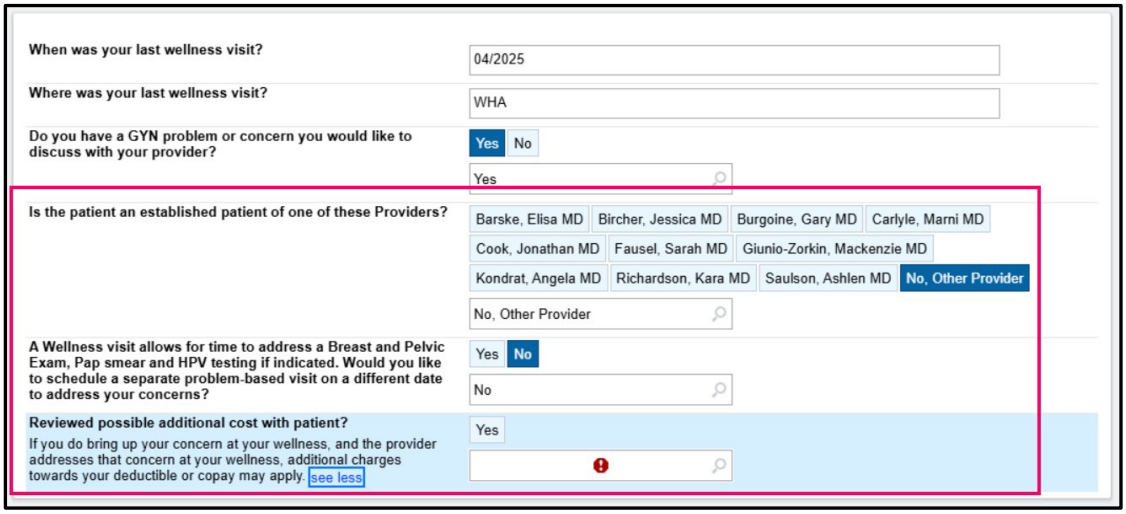
Note: if the patient chooses "no" to this answer the questionnaire will guide you to schedule only the Est Wellness Visit and will skip the next 2 questions

Question 4: Only the Providers that have opt-ed in for this visit type will have a button to choose from. If your patient wants to see anyone else choose "No, Other Provider".

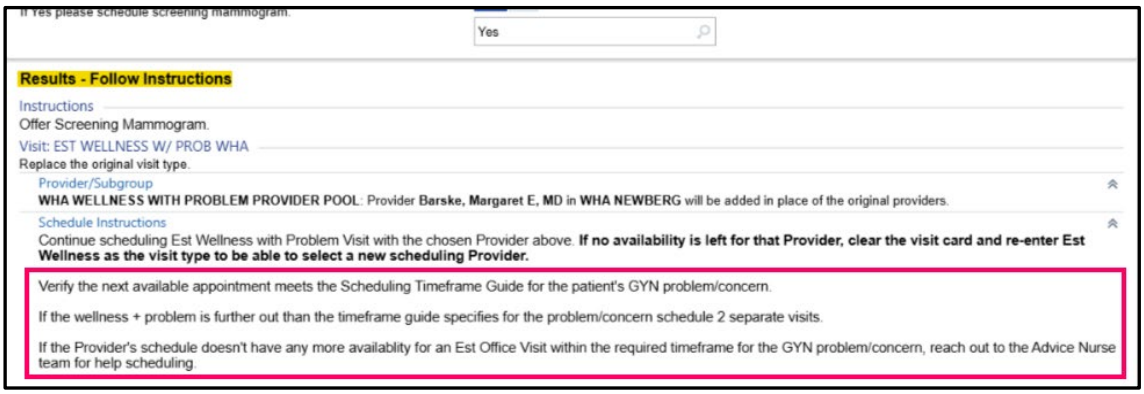
If a Provider button is chosen, read the scripting that is below the question to the

patient. If they have questions, you can have a Financial Coordinator assist them before or after scheduling.

Note: if the patient chooses “No, Other Provider” 1 or 2 more questions will appear addressing that the problem visit will need to be a separate date then the Wellness visit and it may also have additional charges if the concern is brought up on the day of the patient’s Wellness

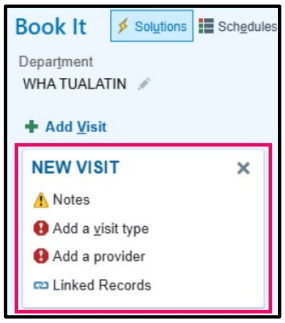


Follow Up Scheduling Instructions:



Below the questionnaire are helpful scheduling instructions and reminders.

These notes are for the scheduler to help determine if it is appropriate to continue scheduling an Est Wellness w/ Problem visit or if it is needed to clear the Visit Card *(see photo on next page)* and start from the beginning with an **Est Wellness** visit type and a different Provider selection.



Click the X within the Visit Card to clear it and start fresh

Requirements for the Est Wellness with Problem

1. When scheduling a qualified patient for an Est Wellness with Problem Visit, the appt still needs to:
   - Abide by the 'one year since the last Wellness visit' insurance rule (ask a Financial Coordinator for more information if needed)

   - And needs to be within the required timeframe for the GYN Problem/Concern that is listed within the Scheduling Timeframe Guide

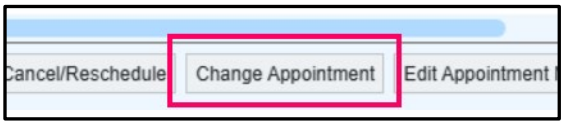
If you cannot schedule the Est Wellness with Problem visit within those 2 requirements above, separately schedule the Est Office visit and Est Wellness visit on a different calendar dates

Reminder: If you cannot schedule the Est Office Visit for the GYN Problem/Concern within the timeframe that the Scheduling Timeframe Guide requires, reach out to the Advice Nurse team for help

2. If you do schedule an Est Wellness with Problem visit, add the Problem/Concern into the Appt Notes

3. If you are rescheduling an Est Wellness with Problem visit, the questionnaire will appear for you to be able to make any edits or changes based on the patient's needs

4. The "Change Appt" function **will not** repopulate the questionnaire and will cause scheduling inaccuracies



Scheduling – Est Office Visits

What to know about this visit type:

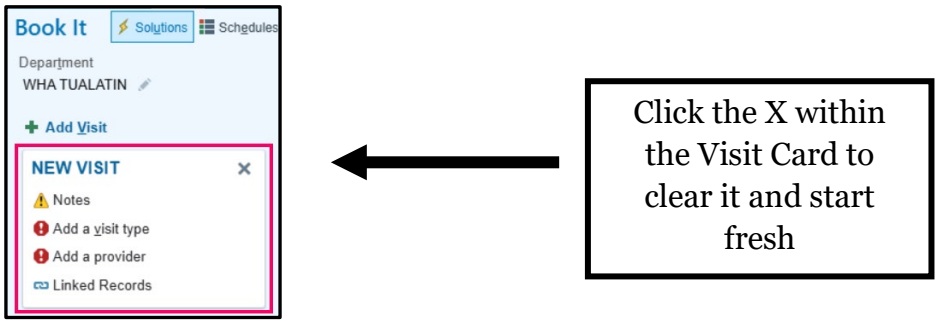
- After selecting this visit type, you will be given 2 options which lead you down 2 different paths (Follow Up **or** New Reason/Concern) within a questionnaire
- Depending on the answers chosen, the visit type name might change
- **Always initially choose Est Office Visit** and let the questionnaire assign the most accurate visit type

Option 1# Follow Up – (visit type = Return Visit)



- Choosing Follow up within the Est Office Visit questionnaire is **only for patients who have seen a WHA clinician within the past 6 months for the same specific reason or concern.**

- If the patient wants to see another clinician or there is no availability with the original clinician, the scheduler must clear the Visit Card and re-choose the Est Office Visit type and select the New Reason button within the questionnaire.



Option #2 New Reason/Concern –



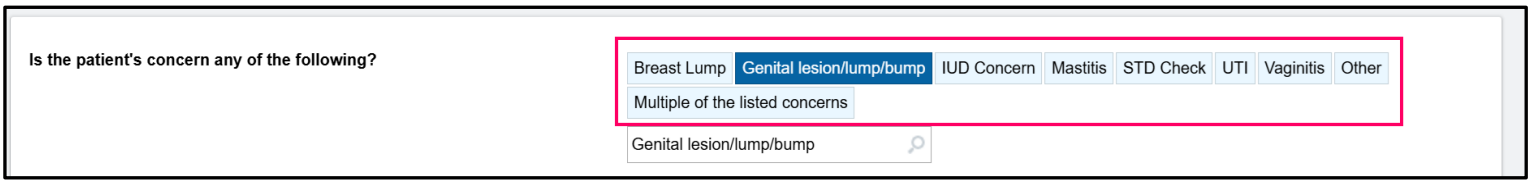
- This selection will guide you to scheduling either an Uncomplicated Est Office Visit or a regular Est Office Visit

- **Always initially choose Est Office Visit** and let the questionnaire assign the most accurate visit type

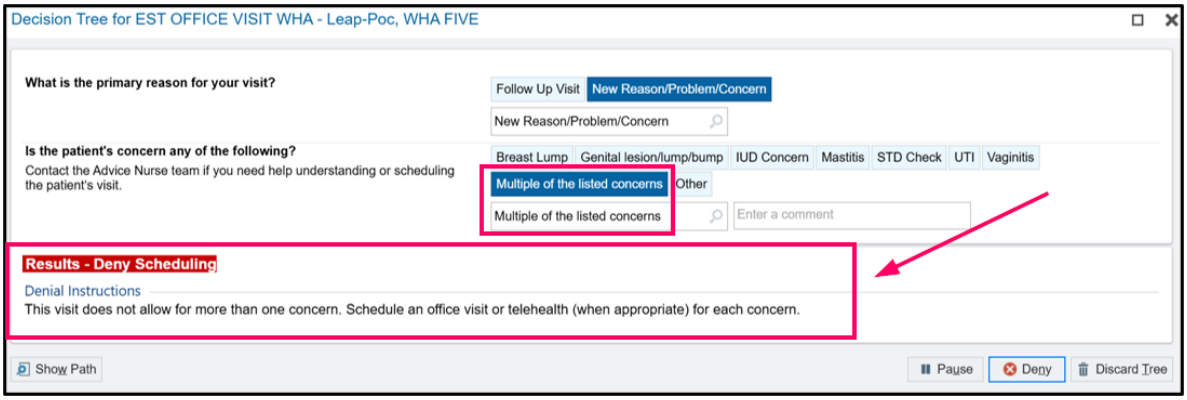
- **Do not** use the word 'Uncomplicated' to the patient, it will show just as an Est Office Visit on their Myhealth account

- Uncomplicated Est Office Visits are built so that the Clinician can get a better view of what the visit is regarding and adjust their visit length in the template background

- This visit type path has 9 options to choose from. If you are needing more clarification on which one to choose, call the Advice Nurse team



- If the **Multiple of the listed concerns** option is chosen because that is what accurately reflects the patient's needs, an Epic scheduling alert will appear in **red**. This visit type does not allow for more than one concern and instead the scheduler should separate each concern into two visits on different days.



Utilize this scripting when explaining the situation to the patient:

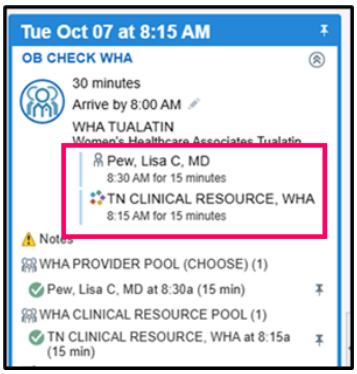
"Each concern will most likely take the full amount of time that you are scheduled for, which concern would you prefer to be seen first? You are welcome to bring up that second concern at this visit if there is time, but we cannot guarantee it will be addressed."

Scheduling – Advanced Visit Types

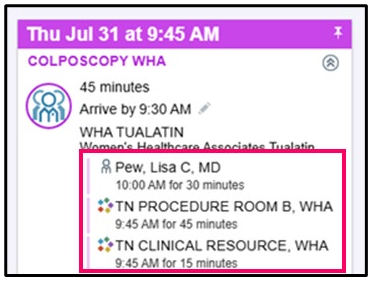
Advanced visits are built and used when more than one Clinician / Resource needs to be applied to the same visit slot. These are the main visits WHA schedules due to the Clinical Resource (Medical Assistant/LPN Rooming Template) being attached to Provider appts.
Visits like BH, Mammograms, Labs, and Ultrasounds don't need the Clinical Resource attached because those visits do not normally have an MA/LPN rooming for them

Examples:

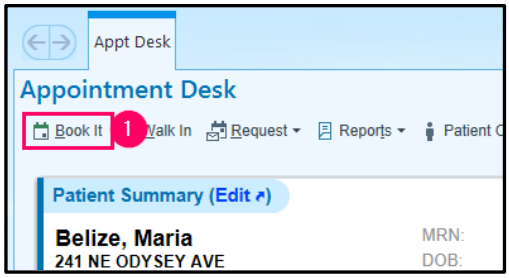
For OB Check visits, Clinicians need their schedule booked **and** their Medical Assistant (Clinical Resource) booked to be able to room the patient for the visit. Both template selections will be in 1 visit card on the Book It window.

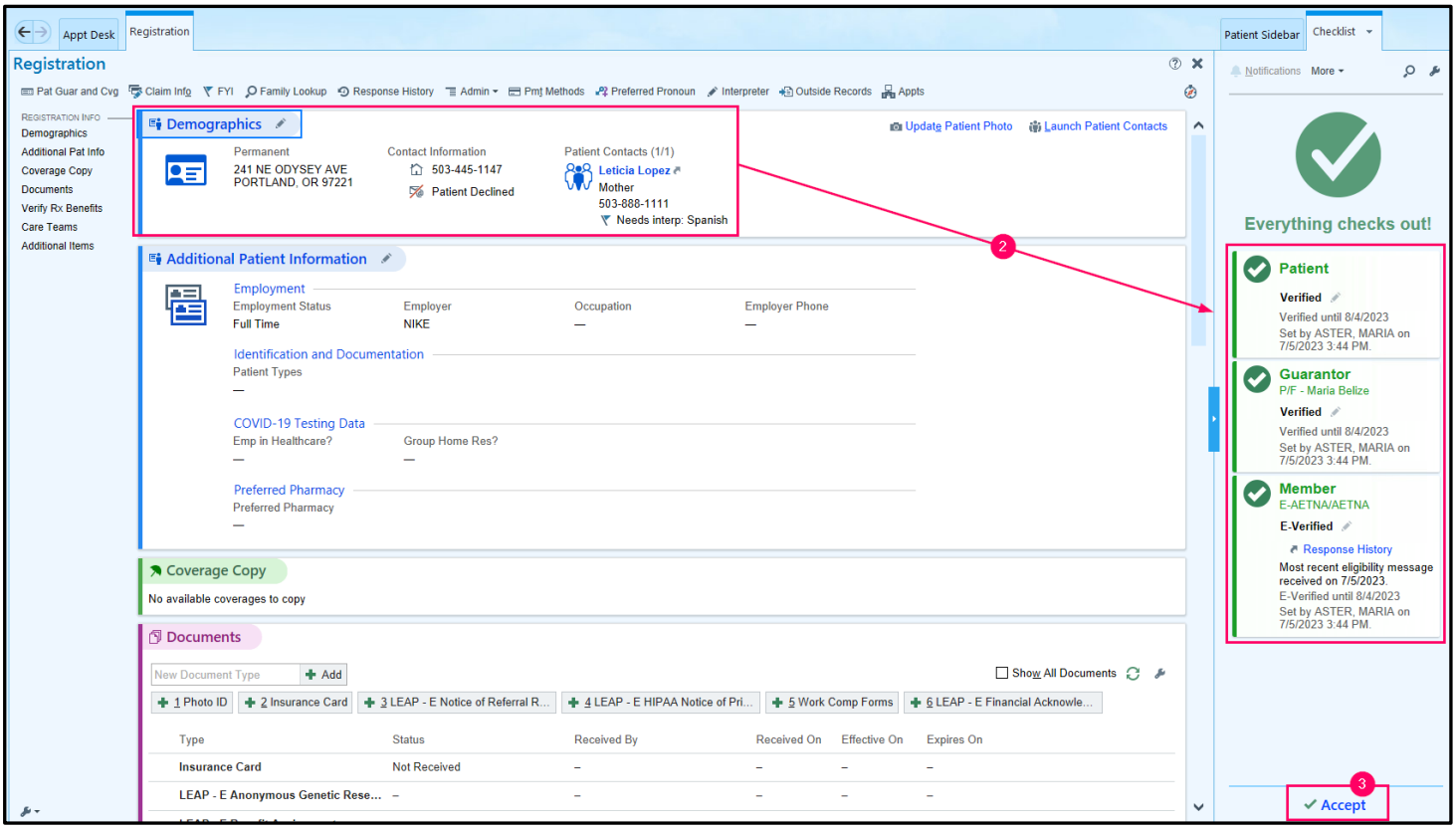


For Colposcopy visits it requires a Medical Assistant (Clinical Resource) booked to room the patient, the Clinician booked to do the visit, and a Procedure booked for that entire time (Clinical Resource start time to Clinician end time) to have a place to do the visit. All 3 template selections will be in 1 visit card on the Book It window. See chapter with a step by step guide here



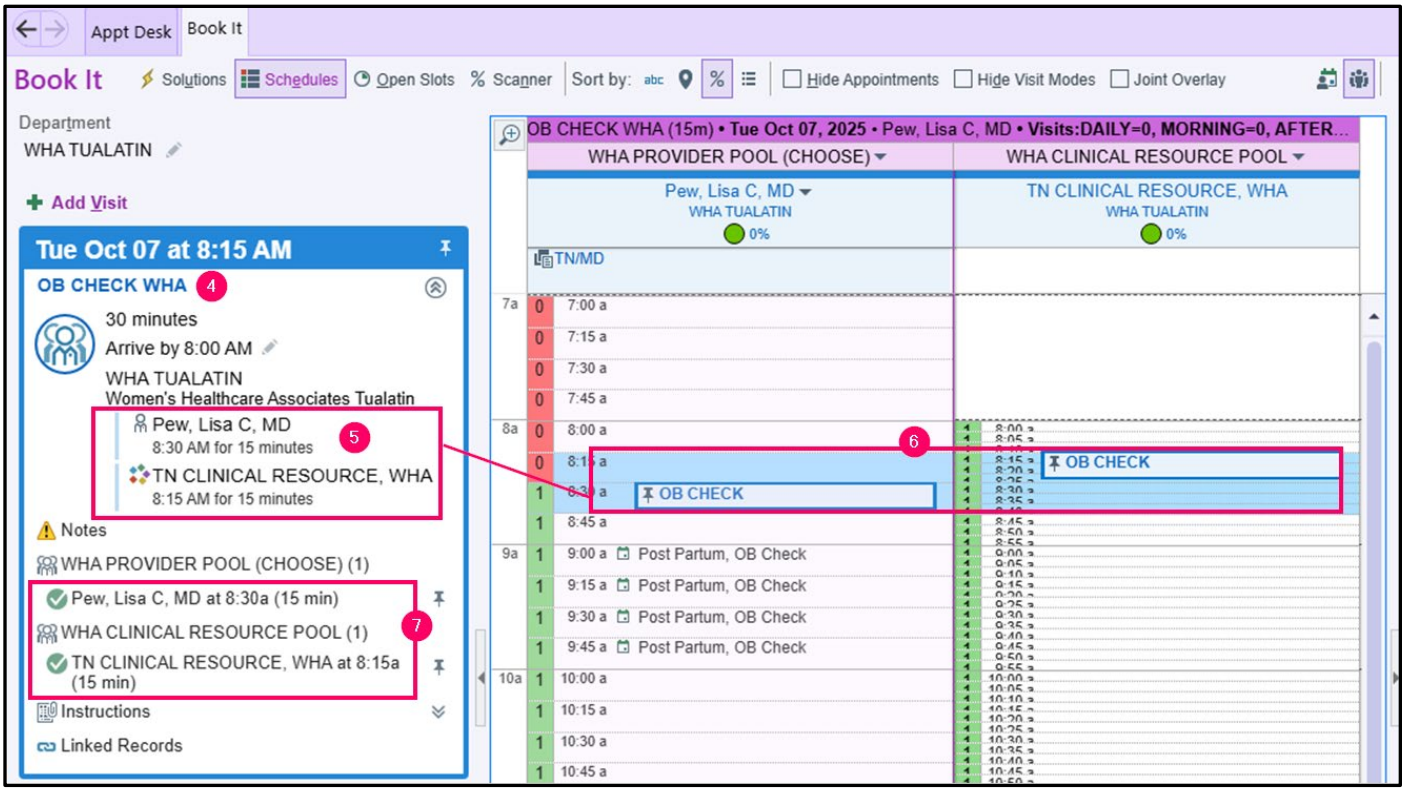
Step by step guide of how to schedule in Book It:





1. Click on **Book It**
2. Verify patient's address, billing address (guarantor account) and insurance then **Verify** the Checklist by clicking any  - Fill out/Complete any missing information before moving onto Scheduling
3. Click **Accept**

Advanced Visits are designed to pull in all visit Clinicians/Resources required based on the initial visit you enter. You can use Schedules or Solutions for this type of Scheduling



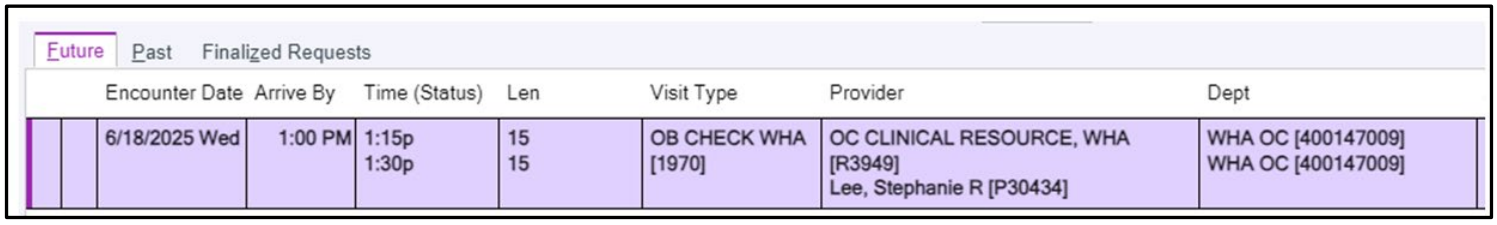
4. Choose the visit type in the Visit card

5. There will be a red hard stop sign indicating what needs to be completed in this Advanced visit card. For the example above, Dr. Pew was selected as the Clinician and the TN Clinical Resource

6. Choose the different start times you need for both provider & resource on the template

7. You will know if you selected both successfully, if there is a green check mark next to both listings in the visit card

This is an example of what a scheduled Advanced visit would look like on the patient's Appt Desk:



Future | Past | Finalized Requests

Encounter Date	Arrive By	Time (Status)	Len	Visit Type	Provider	Dept
6/18/2025 Wed	1:00 PM	1:15p 1:30p	15 15	OB CHECK WHA [1970]	OC CLINICAL RESOURCE, WHA [R3949] Lee, Stephanie R [P30434]	WHA OC [400147009] WHA OC [400147009]

Scheduling – Visits with a Procedure Room

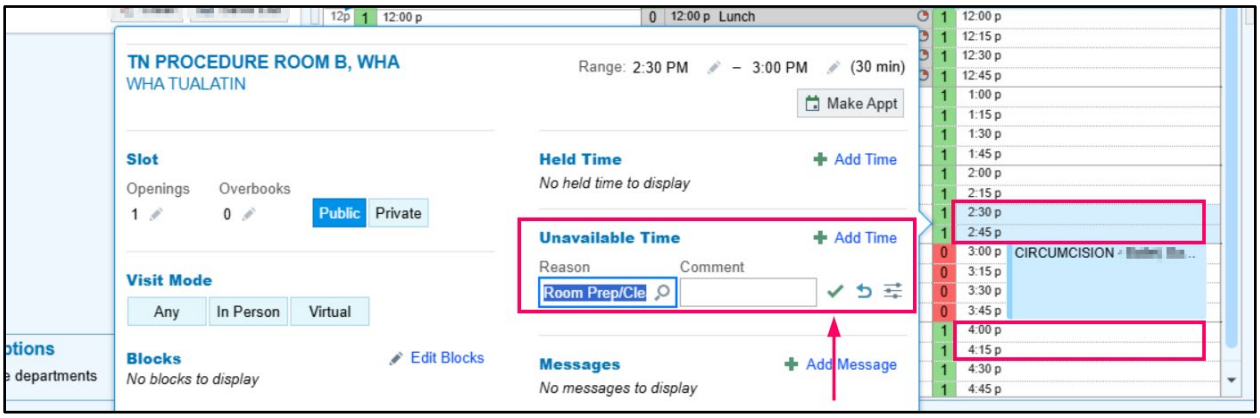
Procedure Visit Types are designed to pull in all clinicians/resources required based on the initial visit you enter. Scheduling Procedures that have Procedure Rooms attached are optimially scheduled within the Schedules view in Book It

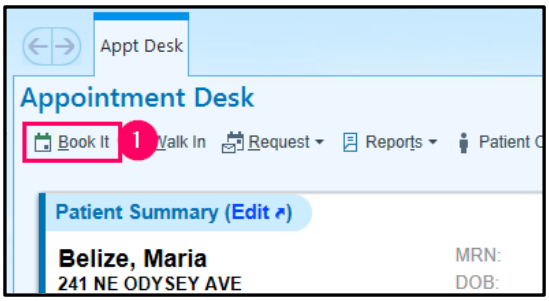
The Procedure Room needs to start from the start of the Clinical Resource (MA Rooming time) to the end of the Clinician visit so that the team can utilize the room the entire time slot. You may need to edit the Procedure Room time length within the visit card to accomplish this.

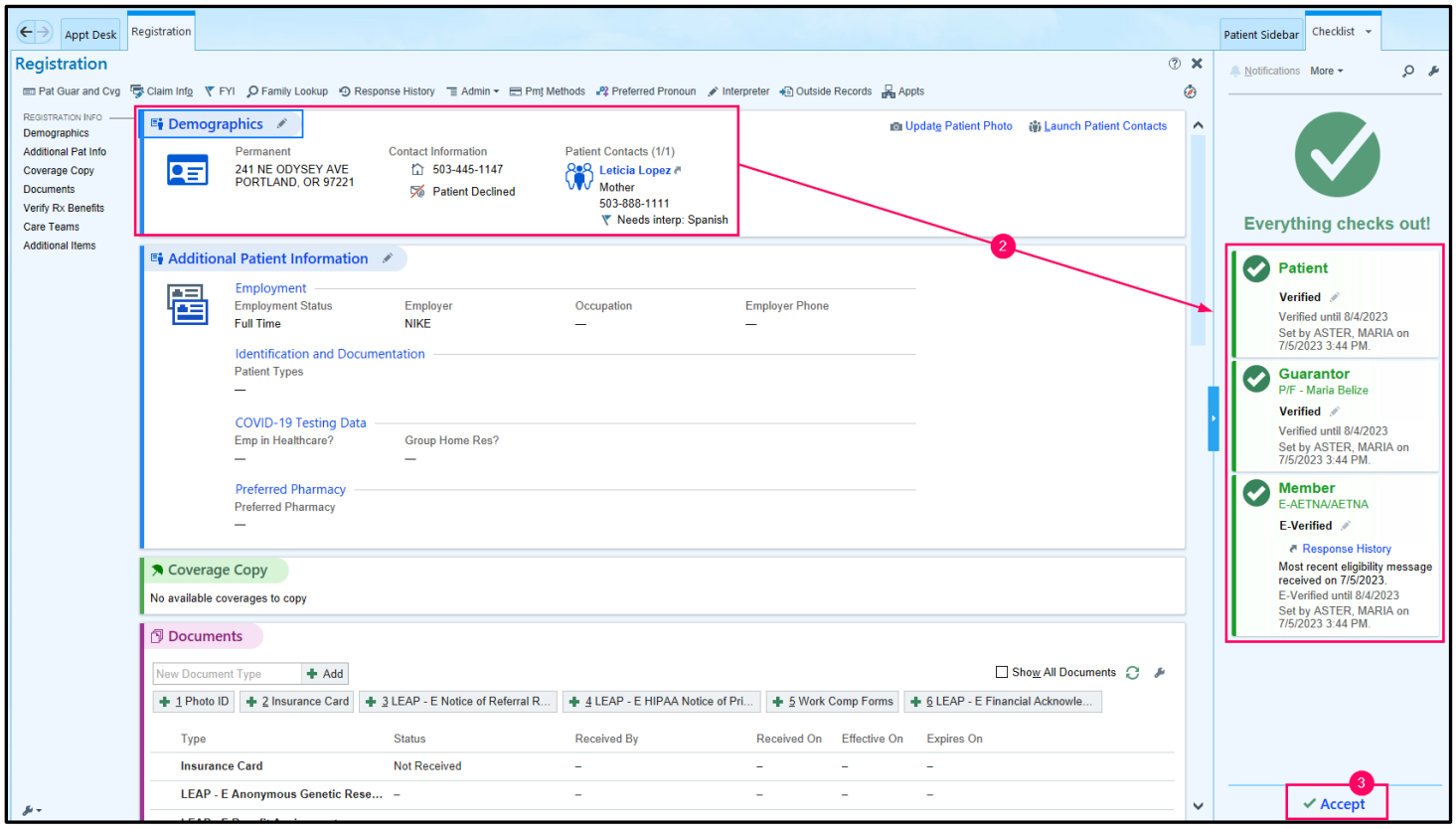
Example, for a Colposcopy Procedure Booking:

If the start time of Clinical Resource is 10am, then the Clinician visit would start at 10:15am and the Procedure room would need to be booked from 10am to 10:45am.

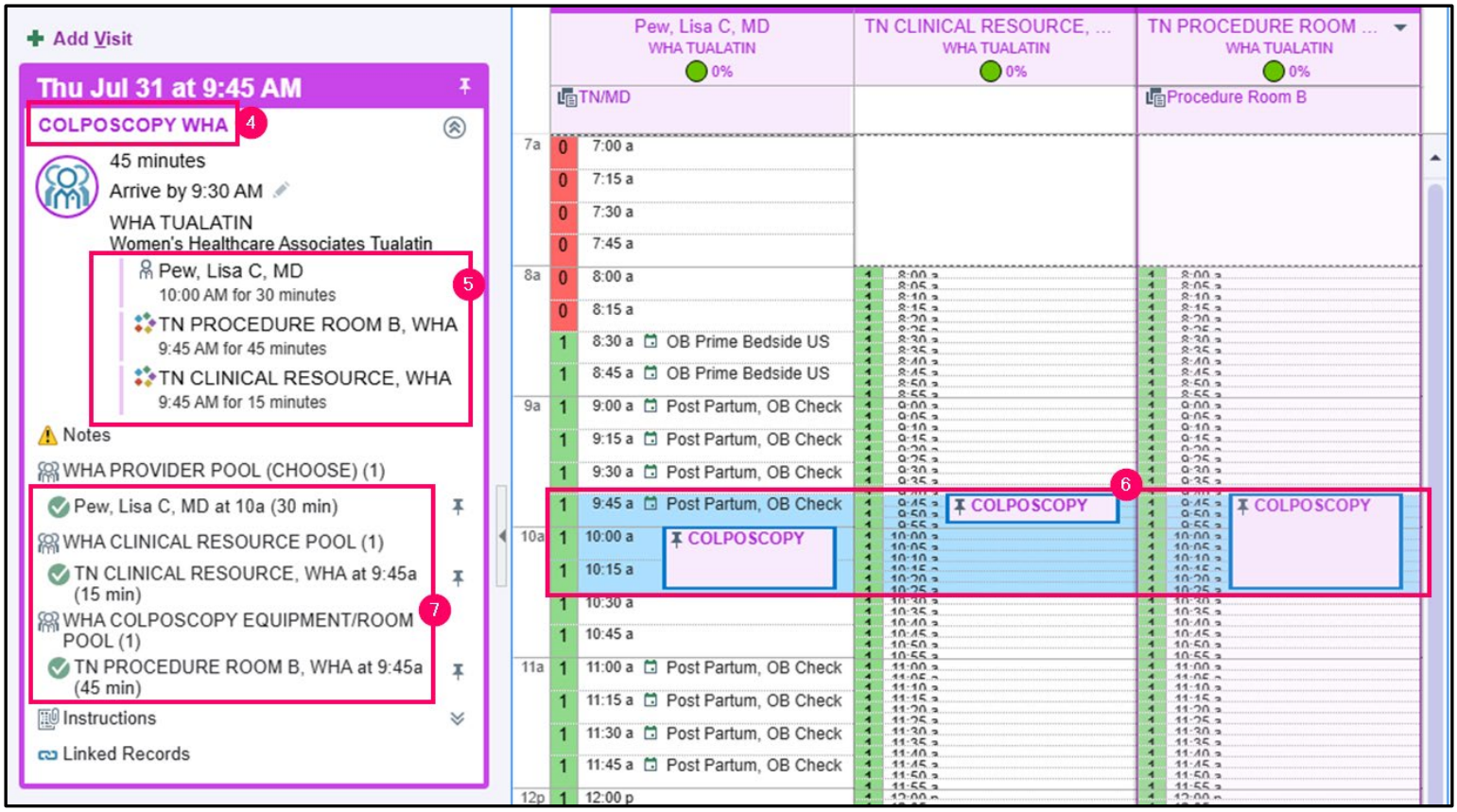
Note: Adding Prep / Clean Up Unavailable Time is required for every visit booked on the Procedure Room – the MA Rooming Clinical Resource does not replace the Prep / Clean Up because it is two separate resources being booked (the room and the person). It should be 15 – 20 minutes before **and** 15 – 20 minutes after (depending on how your clinic's template is built).







1. Click on **Book It**
2. Verify patient's address, billing address (guarantor account) and insurance then **Verify** the Checklist by clicking any  - Fill out/Complete any missing information before moving onto Scheduling
3. Click **Accept**



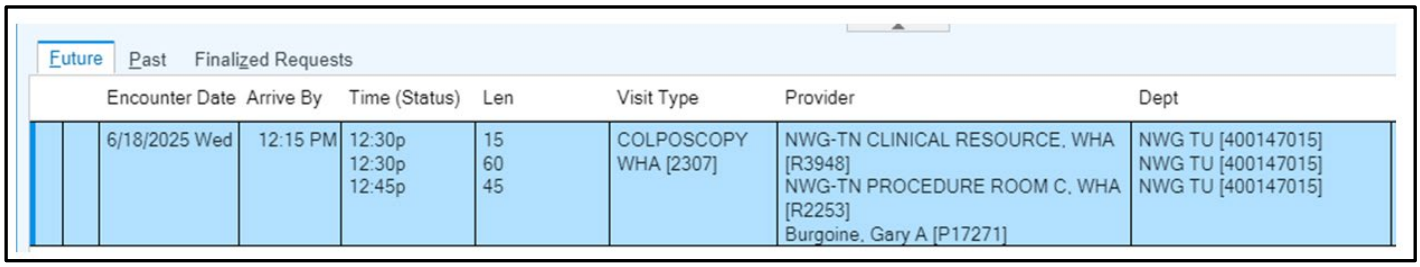
4. Choose the visit type in the Visit card

5. There will be a red hard stop sign indicating what needs to be completed in this Advanced visit card. For the example above, Dr. Pew was selected as the Clinician and the TN Clinical Resource

6. Choose the different start times you need for the provider & both resources on the template

7. You will know if you selected both successfully, if there is a green check mark next to all 3 listings in the visit card

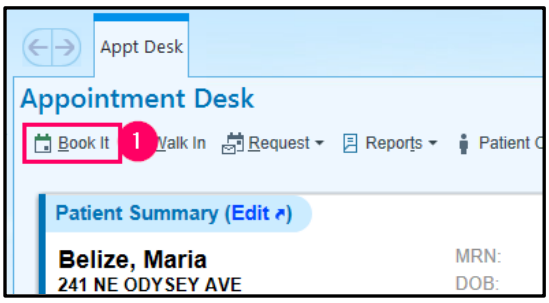
This is an example of what a scheduled Procedure visit would look like on the patient's Appt Desk:

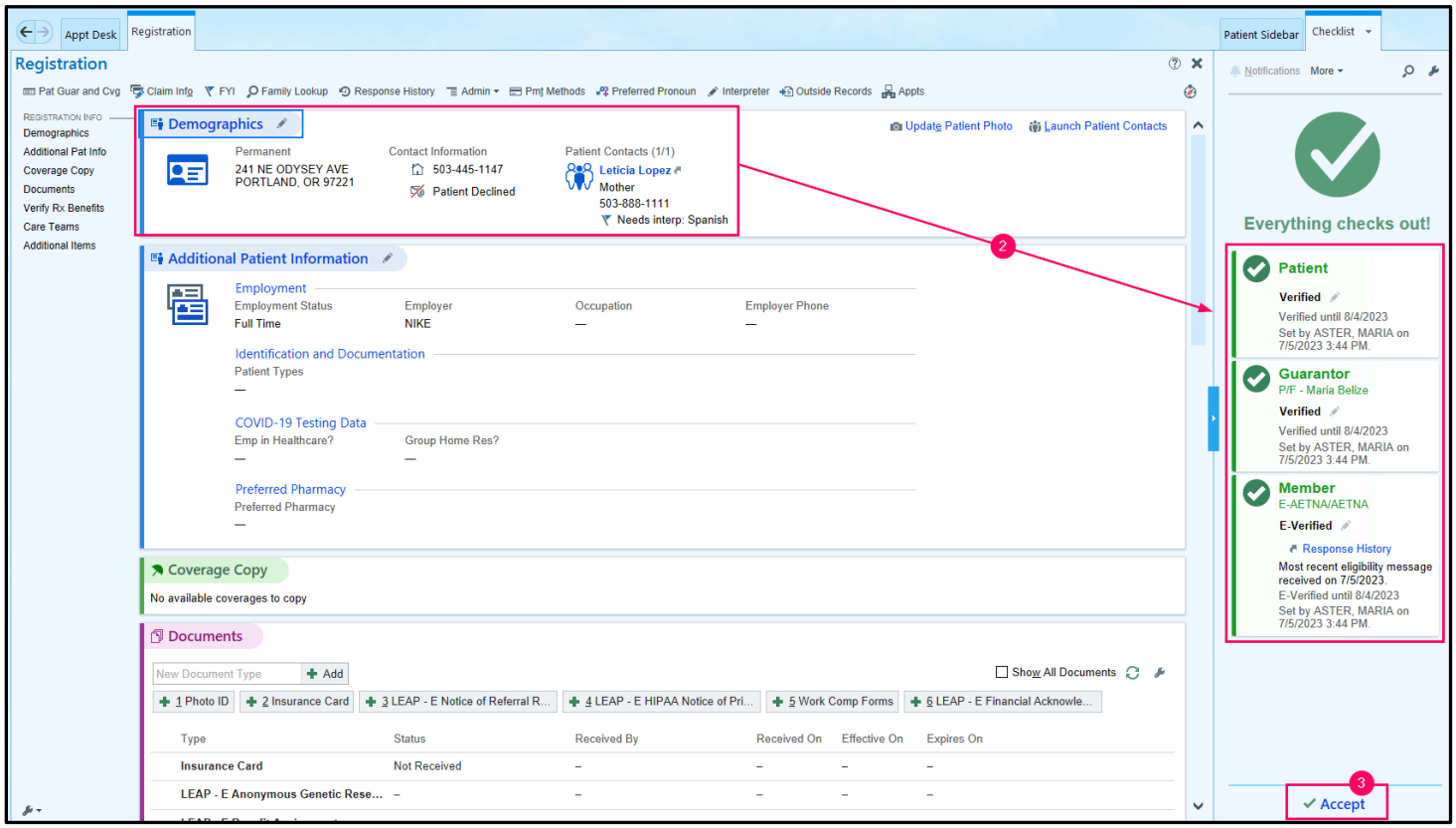


Future | Past | Finalized Requests

Encounter Date	Arrive By	Time (Status)	Len	Visit Type	Provider	Dept
6/18/2025 Wed	12:15 PM	12:30p 12:30p 12:45p	15 60 45	COLPOSCOPY WHA [2307]	NWG-TN CLINICAL RESOURCE, WHA [R3948] NWG-TN PROCEDURE ROOM C, WHA [R2253] Burgoine, Gary A [P17271]	NWG TU [400147015] NWG TU [400147015] NWG TU [400147015]

Scheduling – Visits with Pools

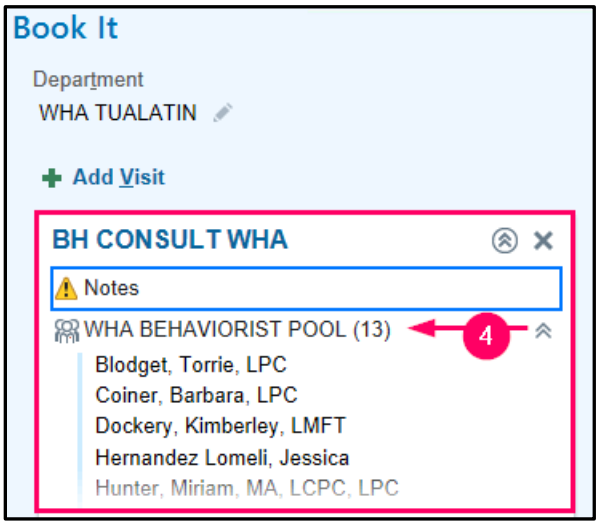




1. Click on **Book It**
2. Verify patient's address, billing address (guarantor account) and insurance then

   Verify the Checklist by clicking any  - Fill out/Complete any missing information before moving onto Scheduling
3. Click **Accept**

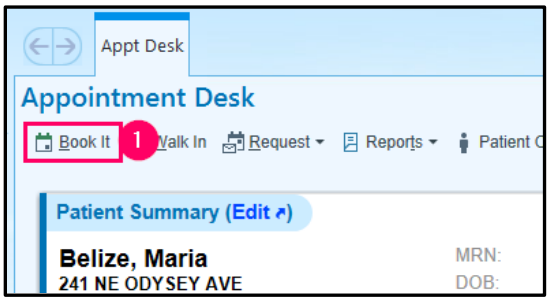
Advanced Visits with Pools are designed to pull in all clinicians/resources that could possibly be chosen based on the initial visit you enter. For example, an Ultrasound visit type will pull in all of the Ultrasound rooms.

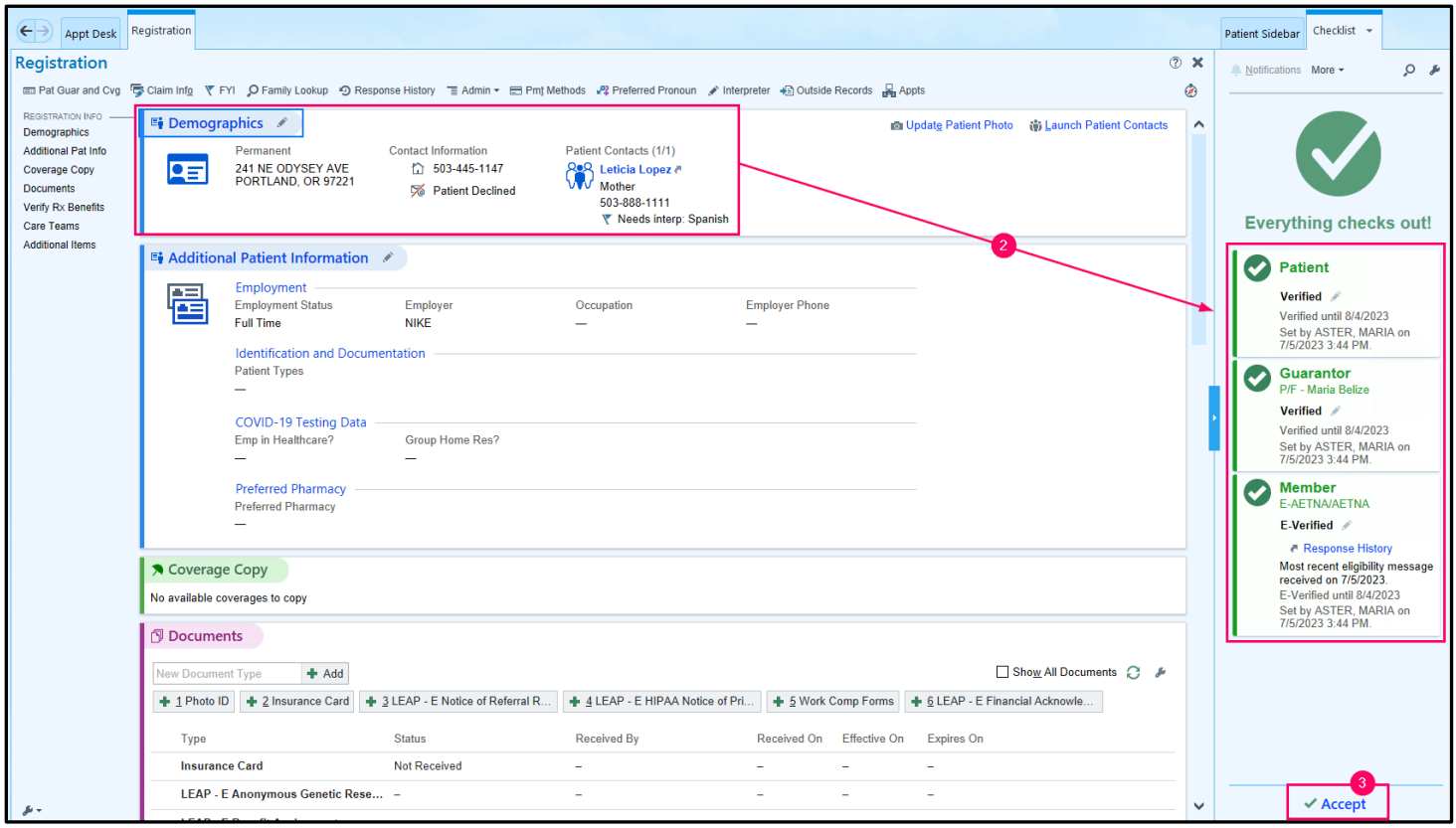


4. Choose which visit type you need to schedule within the Visit Card

   Advanced Visit Types pull in a "Pool" of providers or resources that do that particular appointment. You can leave them all listed to see the group options, or choose one/multiple to look at certain schedules

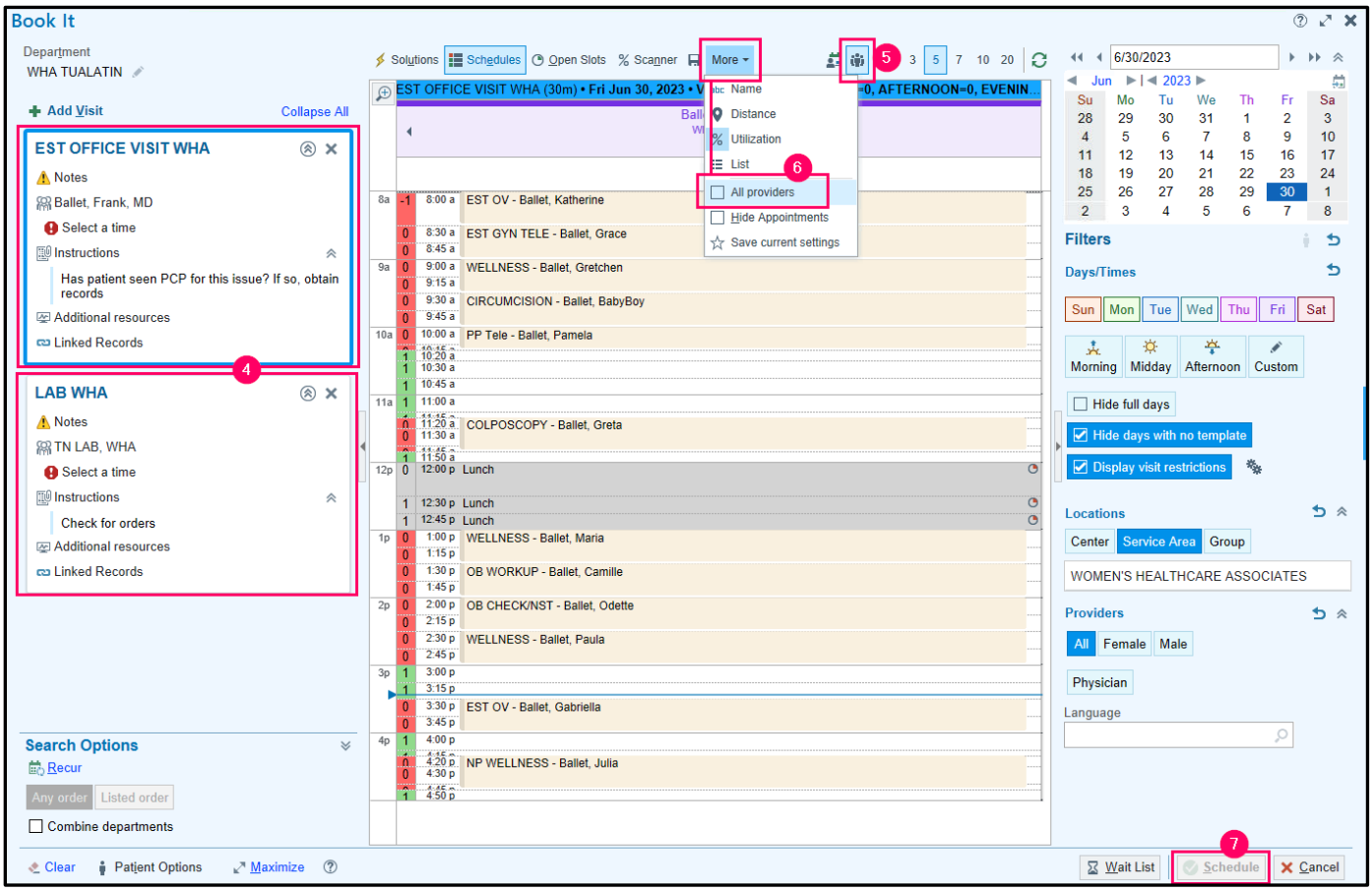
Scheduling – Series of Appointments





1. Click on **Book It**
2. Verify patient's address, billing address (guarantor account) and insurance then

   Verify the Checklist by clicking any  - Fill out/Complete any missing information before moving onto Scheduling
3. Click **Accept**



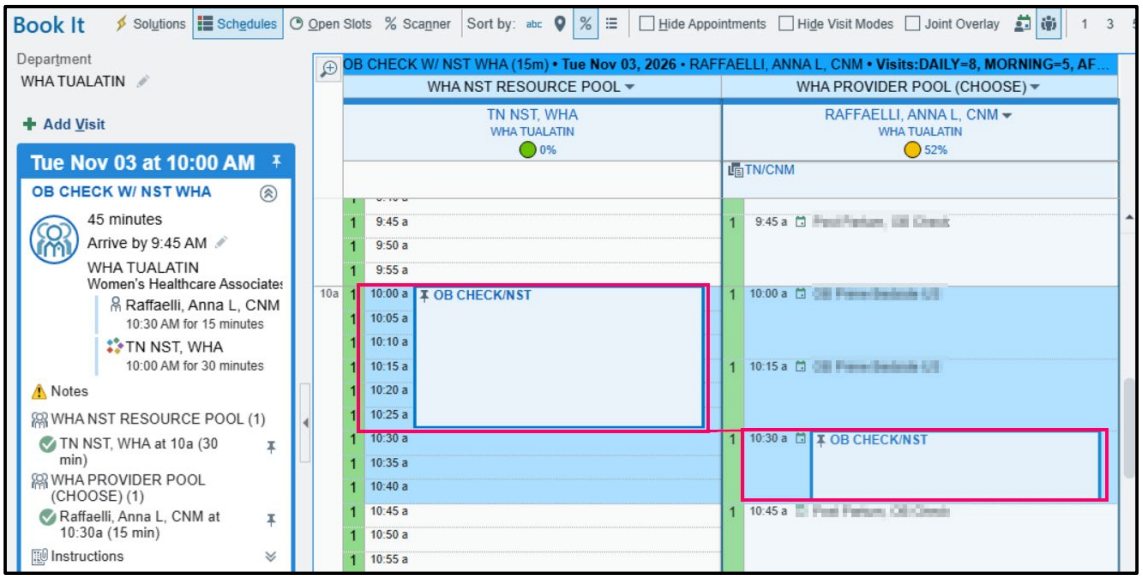
4. Add and complete all Visit Cards needed for the patient
5. Choose the multiple provider(s)/resource(s) layout view
6. Remember if you can't see all of the provider(s)/resource(s) you chose in the Visit Cards, click the **More** drop down menu and choose **All Providers**
7. Click **Schedule** once you have chosen an appointment for each Visit Card

Scheduling – NST Visits

NSTs, or Non-Stress Tests, are screenings done during the third trimester of a patient's pregnancy that checks a baby's health by tracking their heart rate and how it responds to movement.

Scheduling Best Practices:

They are scheduled in combination with a patient's OB Check visit (NST first and OB Check directly afterwards to review the results)

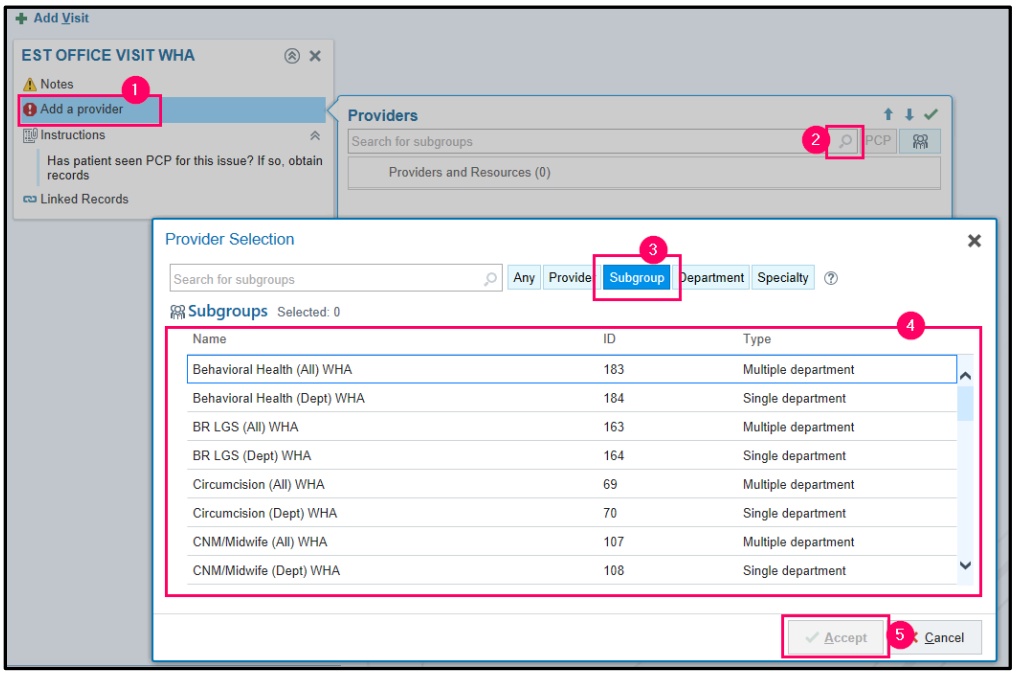


If the Clinician indicates that the patients need to be seen twice weekly for an NST/OB Check they should be 3 days apart following either appts on **Monday & Thursday** or **Tuesday & Friday**

If indicated that a weekly NST/OB Check is needed, these should be scheduled as close to weekly as possible (example: if scheduled Tuesday this week, next week's visit should be on Tuesday if possible)

Scheduling – Visit Types with Subgroups

How to find Subgroups within the Scheduling screen:



The following are some visit types that use a subgroup:

Behavioral Health
Circumcision
Colposcopy
Cryotherapy
Endometrial Biopsy (EMB)
IUD
LEEP
Nexplanon
Pessary Fitting
Sonohysterogram (SIS)

Additional Subgroups available to search for:

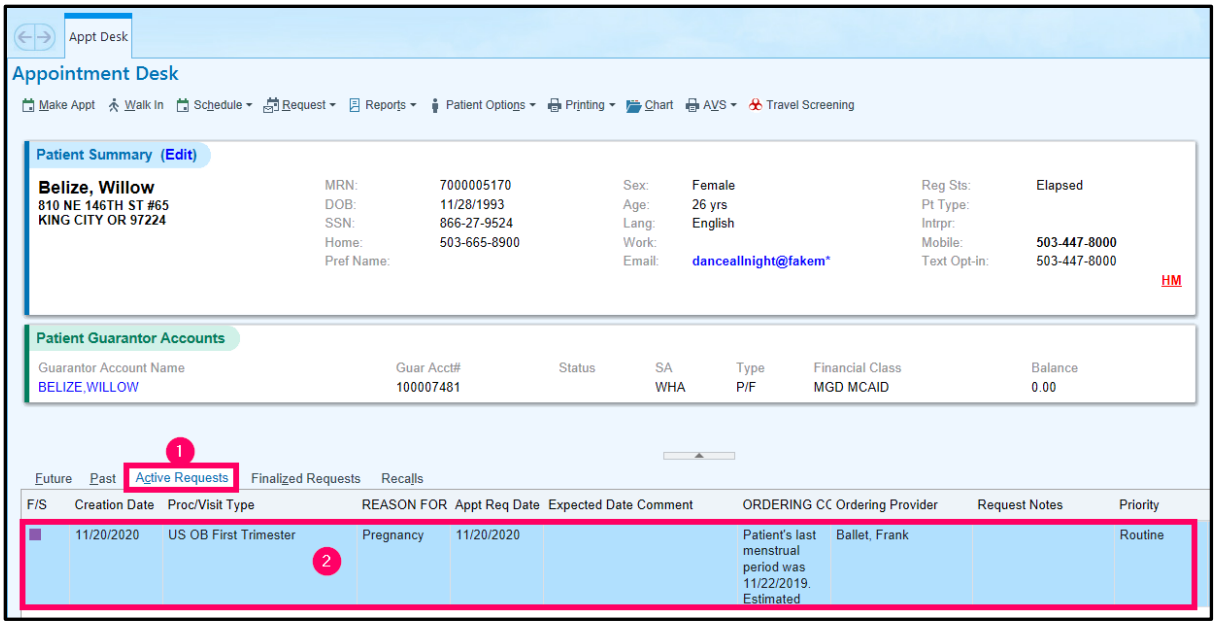
Call Groups (For specific WHA sites – Will show all providers that work call for a specific hospital)
Spanish speaking providers
Provider credentials (MD's, CNM's, etc.)

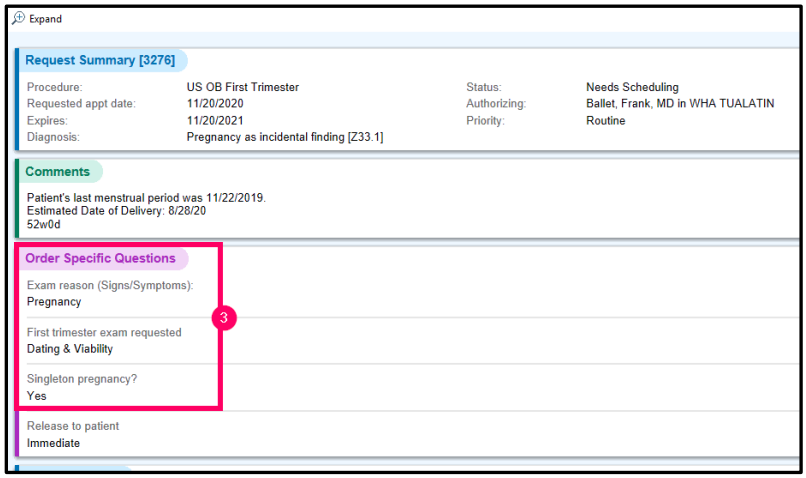
Note: When using Subgroups, it can be best to use the selection that shows (*Dept*) vs (*All*).

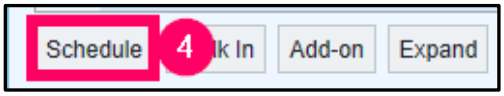
- Using (*Dept*) will look for providers at the selected WHA Department
- Using (*All*) will look for all providers at any WHA location

Colposcopy (All) WHA
Colposcopy (Dept) WHA

Scheduling – From an Active Request

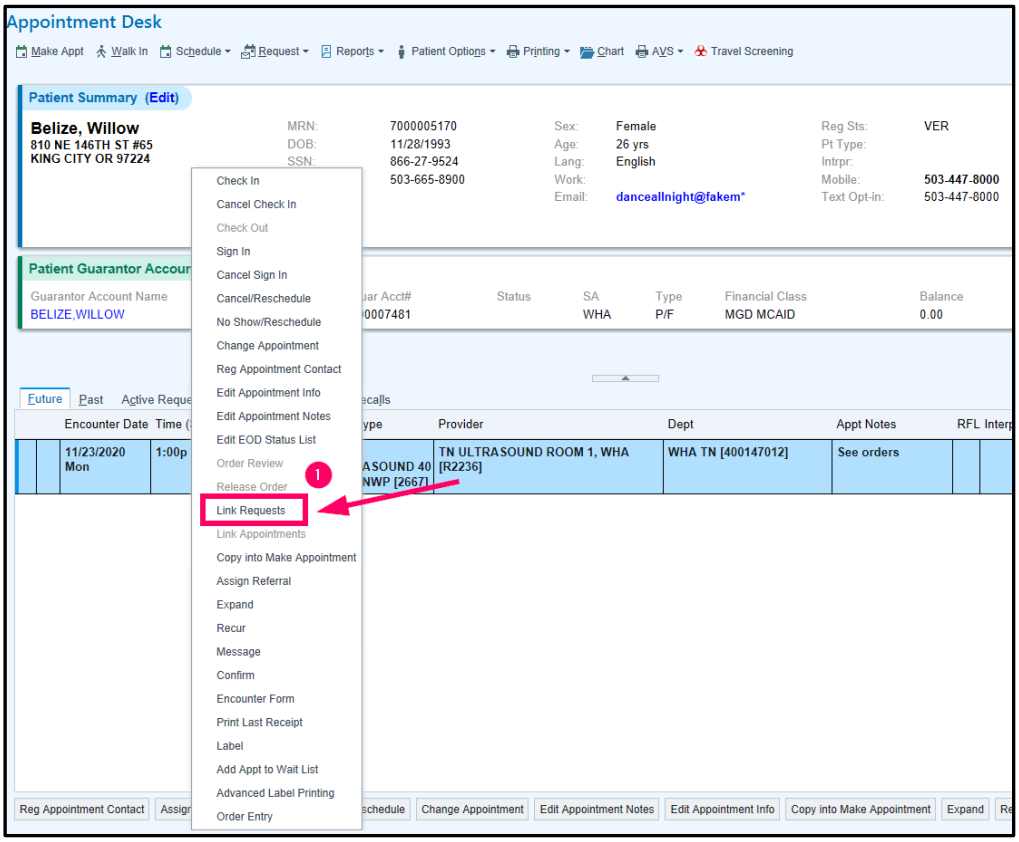


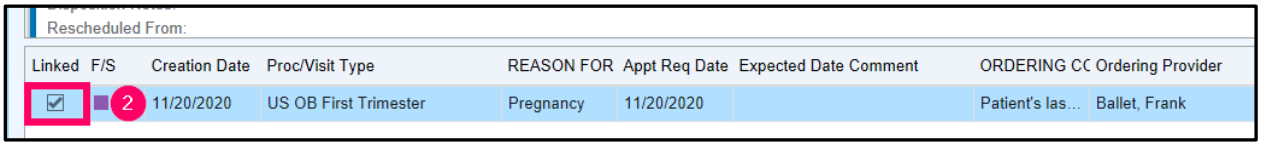




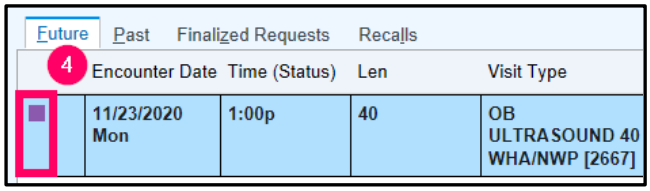
1. Click on the **Orders** tab on the patient's Appointment Desk
2. To see the detail of the order, double click on it or click **Expand**
3. This **Order Specific Questions** section is where the details of the US order is
4. Close the Expand window and Click on **Schedule from order** and continue the scheduling process

Scheduling – Manually Linking Active Requests to a scheduled visit





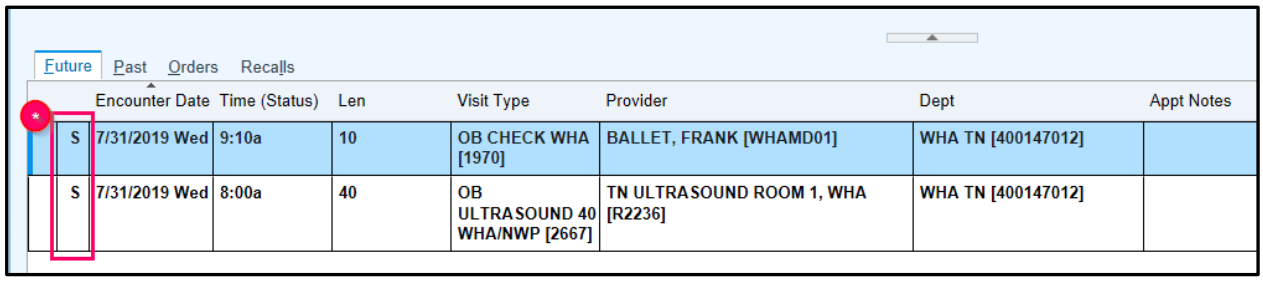




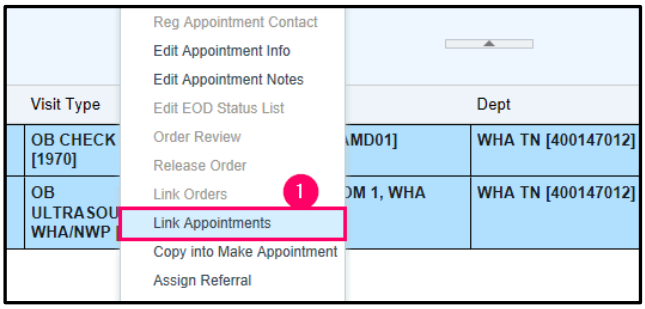
It is always best practice to schedule from an order but if you are in a situation where you are unable to, the next steps show how to link a Request to the scheduled visit…

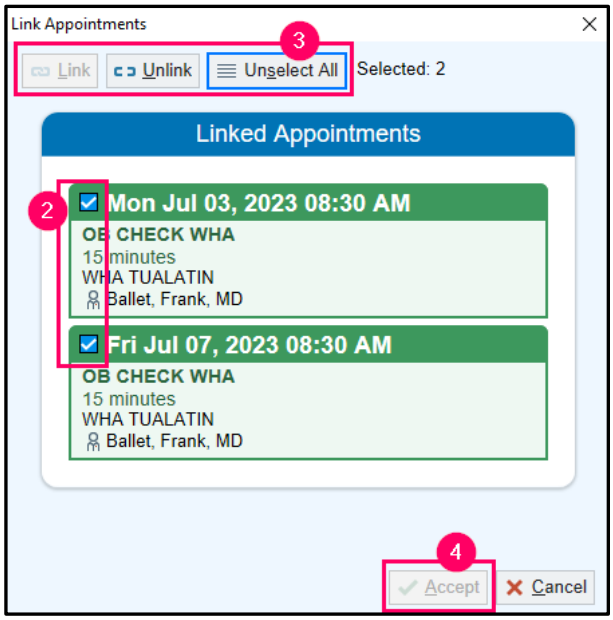
1. **Right-Click** on the scheduled visit from the Appointment Desk. Select **Link Requests** from the menu.
2. **Checkmark** the box next to the Request (*Note: Make sure you choose the correct Request/Order if there are multiple*)
3. Click **Accept**
4. The **purple square** icon indicates the Request is now linked

Scheduling – Linking and Unlinking Appointments



* The appointments are set as a Series of Appointments





*If you ever notice appointments scheduled on the same day are not linked or if you scheduled multiple appts for different days but are accidentally linked, you can manually edit them

1. Select all the appointments you would like to link, right click on them and select **Link Appointments**
2. Check or Uncheck the boxes attached to the desired appointments
3. Click **Link** or **Unlink**
4. **Click Accept**

ER Follow Ups

Determine if a WHA provider was specifically on call during the patient's ER visit:

- If WHA provider was on call at time of visit, follow the next steps in this workflow
- If a WHA provider was not on call at time of visit, use the following scripting:
    - *"We were not the office on call at the time of your ER visit. We would be happy to see you in our office, however we also wanted to make you aware that a different office was on call in the event you want to seek care with them"*
    - If patient would like to proceed with scheduling, follow the next steps in this workflow
    - If patient would like to schedule with the non-WHA provider, advise them to contact that provider's office. No further action needed

OB patients (patient is currently pregnant)

- Update patient registration or register completely if patient is new. Registration update must be completed before moving on to the next step
- **If patient has not** already established OB care for this pregnancy with WHA, start by completing an **OB Intake** telephone encounter, then route to the appropriate OB Coordinator Pool
- Whether the patient has established OB care with WHA or not, create a separate Telephone Encounter. This should include the following:

- Reason for Conversation = ER Follow Up
- Utilize Smart Phrase = **.WHAEDFOLLOWUP**
- Route to the Advice Pool at the site where the patient desires to be seen**.** Let the patient know that a nurse will be calling them back to discuss

GYN Patients

Patient Services:

- If a patient calls stating that they were seen in the ER and they would like to schedule a follow-up visit:
  - Update patient registration or register completely if patient is new. Registration update must be completed before moving on to the next step

- Create Telephone Encounter. This should include the following:
  - Reason for Conversation = ER Follow Up
  - Utilize Smart Phrase = **.WHAEDFOLLOWUP**
  - Route to the Advice Pool at the site where the patient desires to be seen**.** Let the patient know that a nurse will be calling them back to discuss

Advice Nurse:

- Advice nurse reviews the patient's ER notes, and consult providers as needed. Advice nurse will call patient to discuss and further evaluate if needed

  - If no, call from advice nurses needed RN will route the telephone encounter to the appropriate site's PS pool for PS team member indicating when to schedule patient for their visit. **Please ensure you are scheduling patient within the correct call group**

- If Advice evaluates and decides it is needed to speak with patient, Advice will schedule the patient during the time of their conversation. **Please ensure you are scheduling patient within the correct call group**

Scheduling Guidelines:

- MD visit only – exception to this approved if there is specific approval from APC acquired. If approval acquired Advice Nurse to document in telephone encounter
  - Should be scheduled within the appropriate call group (for example, if patient was seen at LMPH patient should be seen by TN call group)

- Team member who schedules will add the following appointment notes:
  - Hospital; Date of ER visit; Reason

ER Follow-up Reschedules

- If **patient** rescheduling okay to shift to next available appointment with MD

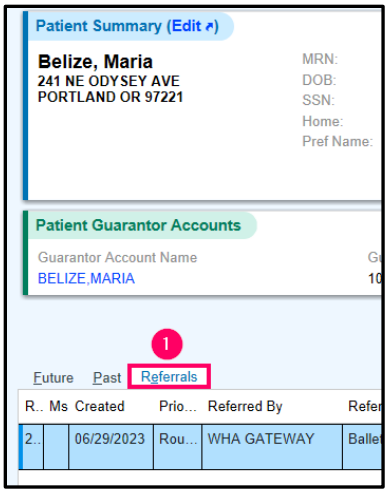
- If **WHA** is rescheduling and nothing available within documented time frame from telephone encounter, reach out to Advice Nurse or Supervisor for further assistance

ER Follow-up No Shows – Reference the GYN High Risk or OB Triage chapter of the No Show Guide here

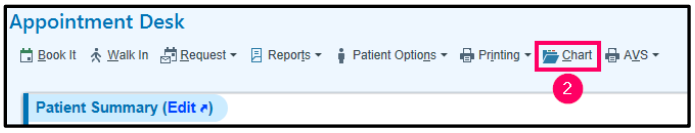
Referrals - Where to find Active Referrals on file

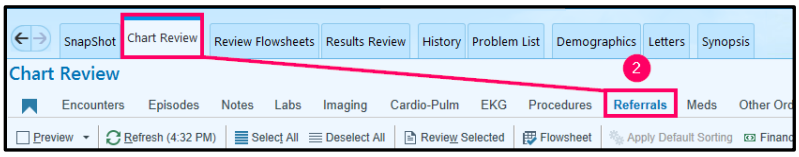
There are 3 places to find an active Referral:

1. From the patient's **Appt Desk**. If there is an active Referral, you will see a tab labeled Referrals next to the Future/Past tabs

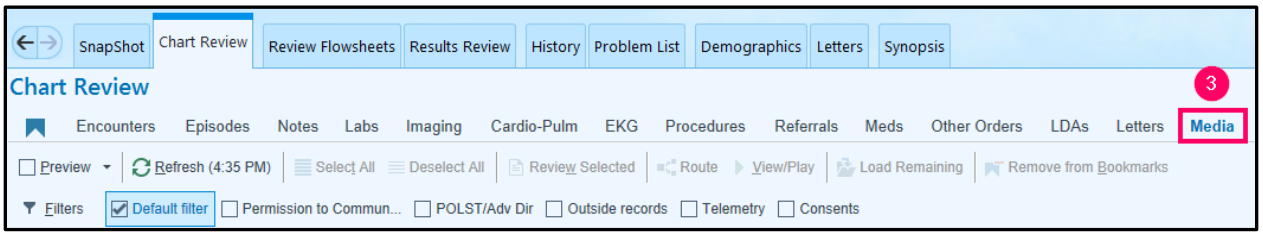


2. Within the patient's **Chart**. Go to Chart Review then the **Referrals tab**

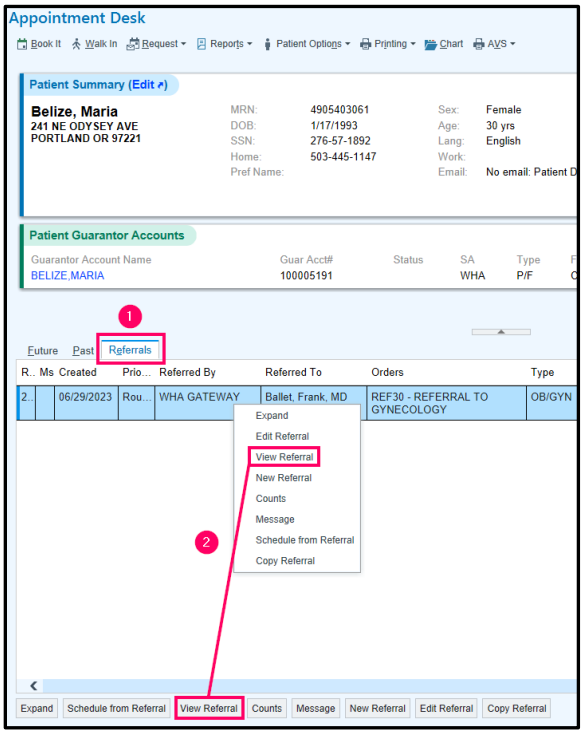


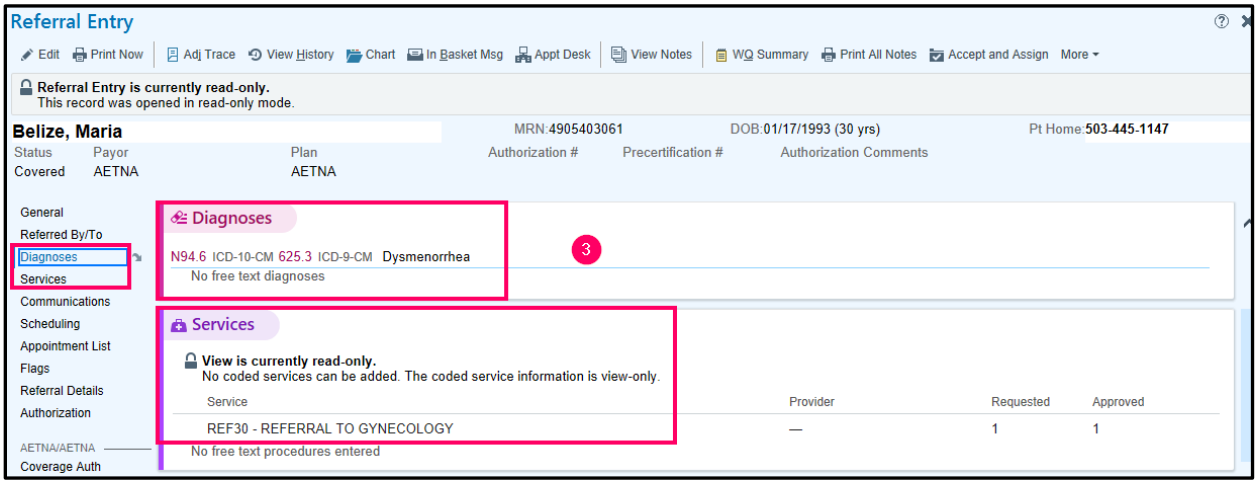


3. If an electronic referral is not found, try looking under the **Media** tab to see if we received a paper version that was scanned into EPIC

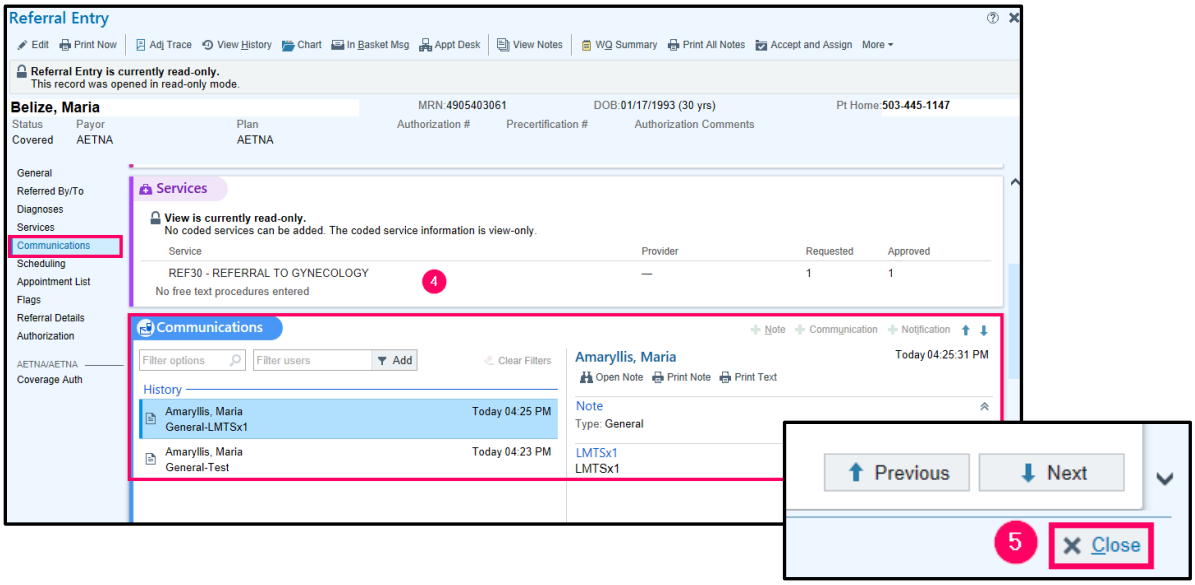


Referrals - Reading notes within a Referral

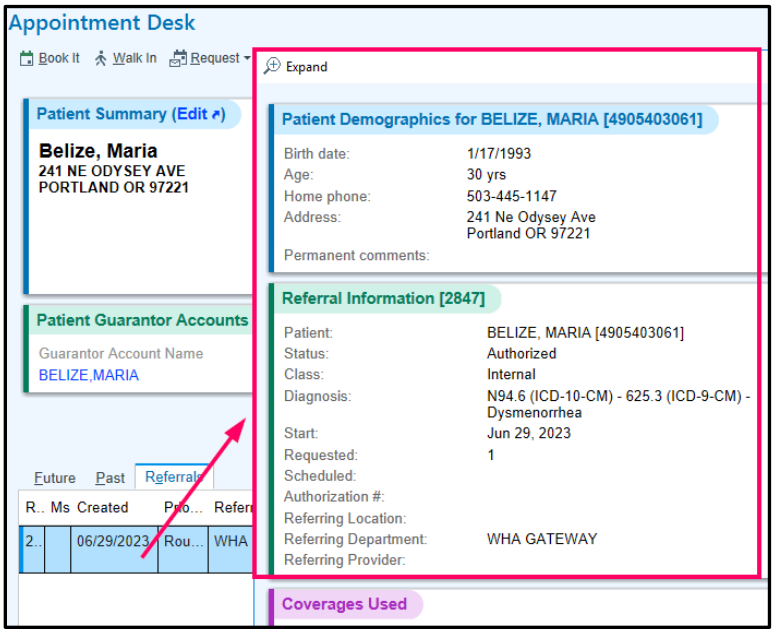




1. From the patient's Appt Desk, click on the **Referral** tab
2. Choose **View Referral** by either right clicking on the Referral or from the task bar below
3. Under **Diagnosis** & **Services,** you will be able to see the referred Diagnoses and Service to guide scheduling



4. Select the **Communications** tab to see any notes from the Referral Navigator regarding this specific Referral (*Example: If they have already called to schedule*)
5. Click **Close** to go back to the Appt Desk screen

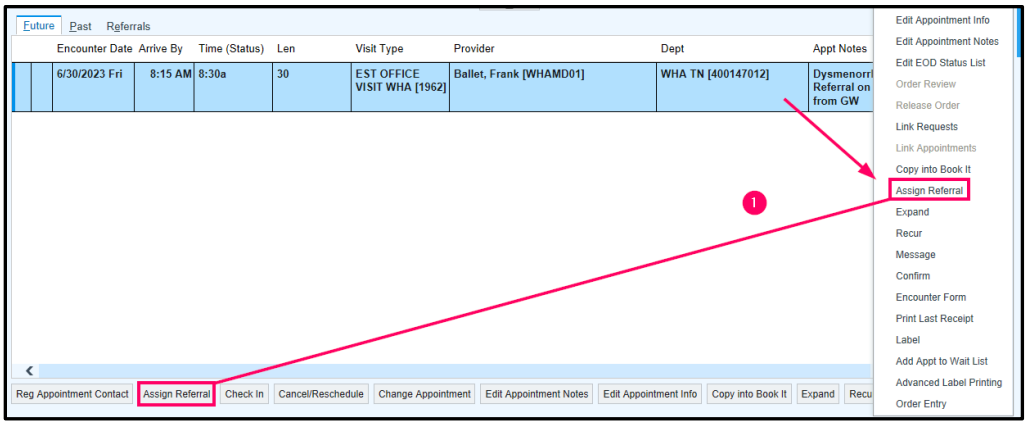


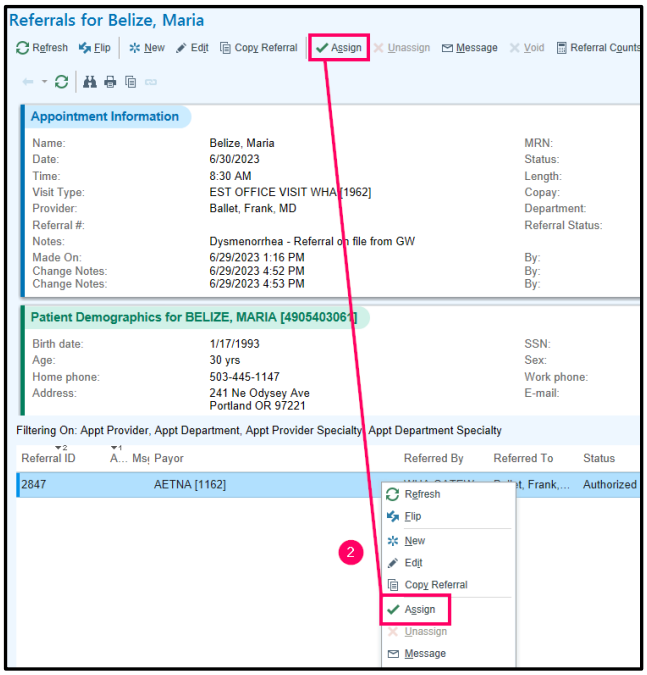
Quick Tip:

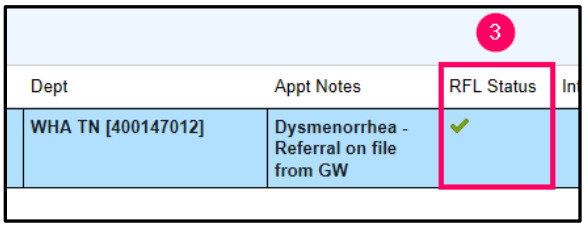
Within the Appt Desk, you can double click the Referral to expand the view. This will let you see the Diagnoses to help with scheduling

Referrals - Assigning a Referral and FAQs

Only Assign a Referral if it has a status of **Authorized**, which you can find within the Referral Shell. If it says "In Progress" or "Pending Review" **do not** Assign Referral







1. Choose **Assign Referral** by either right clicking on the Referral or from the task bar below
2. Choose **Assign** by either right clicking on the Referral or in the top banner
3. Once back on the Appt Desk, there is a **green check mark** attached to the visit to indicate that the Referral was successfully assigned

Referrals – FAQ's

What is the difference between an Insurance Referral and a Regular Referral?

A: An insurance referral a referral requested by the patient that goes through the patient's PCP for approval for insurance tracking purposes. A regular referral is clinician/clinic referred.

Can I schedule a patient if I cannot find a Referral on file?

A: Yes, but there are certain insurance plans that require a Referral before being seen (scheduled is different than being seen). If the patient has one of these insurance plans, the appointment needs to be **scheduled at least a week out.** This gives our Financial team some time to obtain a Referral.

How do I know what insurance plans require a Referral?

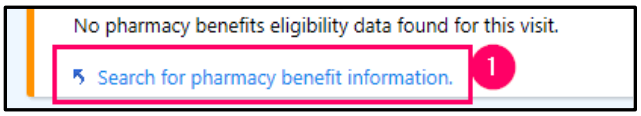
A: Your Financial Coordinator has a general list of the insurance plans that need a Referral on file before the patient can be seen. There is also an insurance matrix on SharePoint (Location: Patient Financial Services tab > Insurance Info)

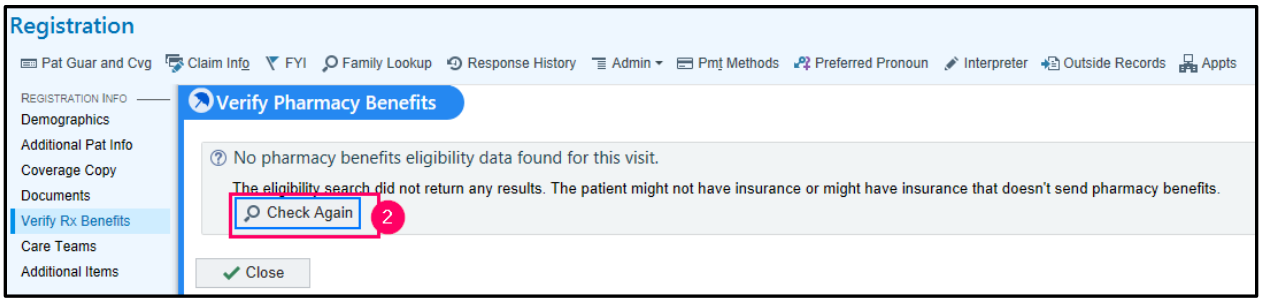
Some of the main ones are: Providence OHP, Providence Medicare, Providence Choice, Providence Connect, Champ VA, Regence BlueAdvantage HMO. Also, any OHP plans (it is diagnoses based) and any HMO plans

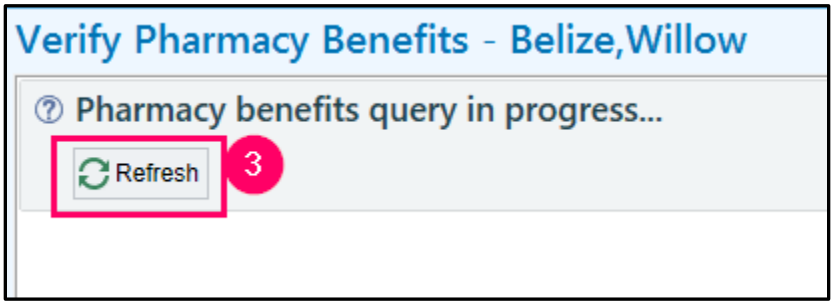
What if I need more assistance with a specific Referral?

A: Remember, your Referral Navigator is here to help answer any immediate Referral related questions, but you can send a **Staff Message** to the Referral Navigator Pool or the Financial Coordinator Pool for non-urgent needs

Prescription Benefit Verification

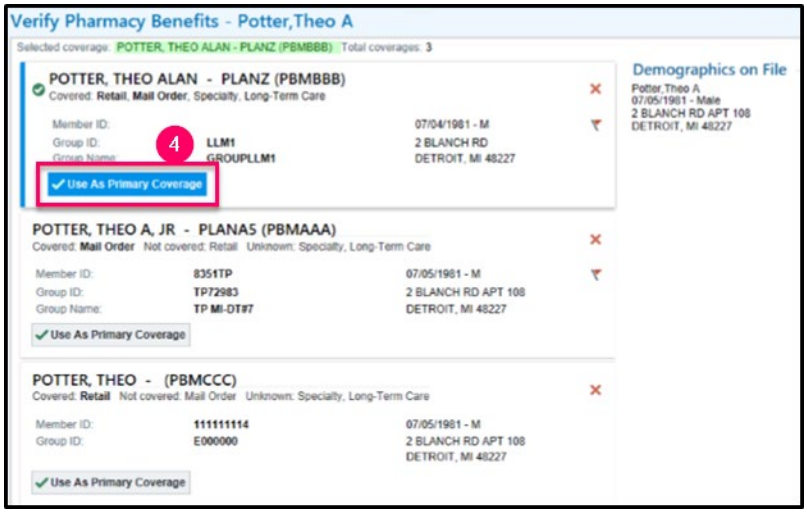


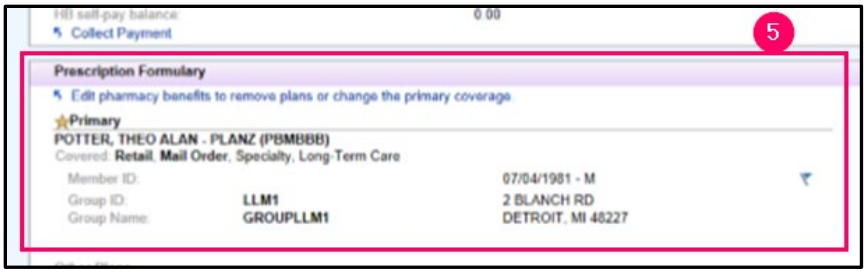




This step will **only** need to be done for Walk-In appointments or Same Day Add-Ons

1. In Registration under Verify Rx Benefits, select **Search for pharmacy benefit information**
2. If no benefits found, click the **Check Again** button to search for coverage
3. Epic will start a query for the Pharmacy Benefits





4. If a patient has more than one prescription benefit listed, clarify with the patient which benefit they prefer to use at their pharmacy. Click **Use As Primary Coverage**
5. The Prescription Benefit will display in the Prescription Formulary in Registration

   Epic automatically send eligibility requests the night before with previously scheduled visits

List of Visit Types

NWP
AMNIO ONLY NWP
AMNIOCENTESIS NWP
AMNIO PANEL NWP 1.GENETIC CONSULT NWP 2.AMNIO NWP
CVS NWP
CVS PANEL NWP 1.GENETIC CONSULT NWP 2.CVS NWP
GENETIC CONSULT NWP (Advanced and Panel options) – Currently inactive (use OB TELEHEALTH VISIT WHA)
GENETIC CONSULT ONLY NWP – Currently inactive (use OB TELEHEALTH VISIT WHA)
GENETIC CON W/ OB US FYI NWP – Currently inactive (use OB TELEHEALTH VISIT WHA)
GENETIC CON W/ MFM CON NWP – Currently inactive (use OB TELEHEALTH VISIT WHA)
GYN ULTRASOUND NWP
IV INFUSION NWP – Currently inactive
LAB NWP
MFM CONSULT NWP
MOD RISK CONSULT NWP
MOD RISK FOLLOW UP NWP
MOD RISK FOLLOW UP W/ NST WHA
MOD RISK POSTPARTUM NWP
NST NWP
NURSE VISIT NWP
OB CHECK NWP
OB CHECK W/ NST NWP
OB ULTRASOUND NWP
OB ULTRASOUND W/ FYI NWP

OB ULTRASOUND W/ LAB NWP
OB US W/ MOD RISK CONSULT NWP
OB US W/ MOD RISK FOLLOW UP NWP
OB ULTRASOUND W/ MFM CON NWP (PANEL) 1. OB ULTRASOUND 2. MFM CONSULT
OB ULTRASOUND W/ OB CHECK NWP (PANEL) 1. OB ULTRASOUND 2. OB CHECK
OFFSITE SURGERY NWP
PEDS CARDIOLOGY NWP
PEDS CARDIO W/ MFM CONSULT NWP
PEDS URO W/ MFM CONSULT NWP (PANEL) – Currently inactive 1. PEDS UROLOGY 2. MFM CONSULT
PEDS URO W/ OB CHECK NWP (PANEL) – Currently inactive 1. PEDS UROLOGY 2. OB CHECK
PEDS UROLOGY NWP – Currently inactive
PEDS URO W/ FYI NWP – Currently inactive
PEDS UROLOGY W/ MFM FYI NWP – Currently inactive
POSTPARTUM NWP

Pathways
P BREASTFEEDING WHA – Currently inactive
P BRSTFD & CONTRACEPTION WHA – Currently inactive
P C-SECTION PREP WHA – Currently inactive
P DELIVERY PREP WHA – Currently inactive
P EMOTIONAL WELLNESS WHA – Currently inactive
P FAMILY DYNAMICS WHA – Currently inactive
P FITNESS AND NUTRITION WHA – Currently inactive
P MEAL PREP WHA – Currently inactive
P PATHWAYS INTAKE WHA – Currently inactive
P PELVIC FLOOR HEALTH WHA – Currently inactive
P POSTPARTUM & NB CARE WHA – Currently inactive
P POSTPARTUM WHA – Currently inactive
P PREG COMFORTS & CONCERNS 1 WHA – Currently inactive
P PREG COMFORTS & CONCERNS 2 WHA – Currently inactive
P STRETCH AND RELAXATION WHA – Currently inactive

WHA
BH CASE MANAGEMENT TELEHEALTH WHA
BH CONSULT WHA *(this is the intake visit)*
BH RETURN VISIT WHA
BH TRIAGE VISIT TELEHEALTH WHA
BH WARM HANDOFF PHONE WHA
BH WARM HANDOFF WHA
BLADDER INSTILLATION WHA
BONE DENSITY WHA – Currently inactive
CHILDBIRTH EDUCATION WHA – Currently inactive
CIRCUMCISION WHA
COAPTITE WHA – Currently inactive **(?)**

COLPOSCOPY WHA
CRYOTHERAPY WHA
CYSTOSCOPY WHA
ENDOMETRIAL BIOPSY WHA
ER FOLLOW UP WHA
EST COMMON CONCERNS WHA – Currently inactive
EST OFFICE VISIT WHA
EST WELLNESS VISIT WHA
GYN TRIAGE WHA
GYN ULTRASOUND WHA
HPV INJECTION WHA
IN OFFICE SURGERY WHA
INCISION AND DRAINAGE WHA
INJECTION WHA
IUD INSERTION WHA
IUD REMOVAL WHA
IUD REPLACEMENT WHA
LAB WHA
LEEP WHA
MAMMOGRAM WHA
MBC MEET AND GREET WHA - Currently inactive
MED MANAGEMENT TELEHEALTH WHA
MYHEALTH BIRTH CONTROL WHA
MYHEALTH WELLNESS WHA
MYHEALTH MEET AND GREET WHA - Currently inactive
MYHEALTH OFFICE VISIT WHA
MYHEALTH MAMMOGRAM 20 WHA
MYHEALTH MAMMOGRAM 40 WHA
MYHEALTH TELEHEALTH VISIT WHA

NEWBORN HOME VISIT WHA - Currently inactive
NEXPLANON WHA (Insertion, Removal, and Reinsertion)
NP COMMON CONCERNS WHA – Currently inactive
NP OB TRIAGE TELEHEALTH WHA
NP OFFICE VISIT WHA
NP WELLNESS VISIT WHA
NST WHA
NURSE VISIT WHA
OB CHECK WHA
OB CHECK W/ NST WHA
OB PRIME CONSULT TELEHEALTH WHA
OB TRIAGE WHA
OB TRIAGE W/ NST WHA
OB TRIAGE TELEHEALTH WHA
OB ULTRASOUND W/OB CHECK WHA
OB ULTRASOUND W/OB WORKUP WHA
OB ULTRASOUND WHA
OB WORKUP WHA
OFFSITE SURGERY WHA/NWP
OPEN SCHED COMMON CONCERNS WHA – Currently inactive
PELLETS WHA
PESSARY FITTING WHA
PHLEBECTOMY WHA – Currently inactive
POST-OPERATIVE WHA
POSTPARTUM WHA
POSTPARTUM TRIAGE WHA
PP DOULA HOME VISIT WHA – Currently inactive
PP DOULA MEET AND GREET WHA – Currently inactive
PP DOULA OFFICE VISIT WHA – Currently inactive

PP DOULA PHONE VISIT WHA – Currently inactive
PRE OP WHA
PSYCMED CONSULT WHA - Currently inactive
PSYCMED RETURN WHA - Currently inactive
REPEAT PAP WHA
SCLEROTHERAPY WHA – Currently inactive
SIS 70 WHA / SIS 100 WHA
TRIGGER POINT INJECTION WHA
URODYNAMICS WHA
UROGYNECOLOGY WHA
VEIN ABLATION WHA – Currently inactive
VEIN ULTRASOUND WHA – Currently inactive
VULVAR INTAKE WHA

WHA Telehealth
OB TELEHEALTH VISIT WHA
NP GYN TELEHEALTH VISIT WHA
EST GYN TELEHEALTH VISIT WHA
PP TELEHEALTH VISIT WHA
BH CONSULT TELEHEALTH WHA *(this is the intake visit)*
BH RETURN VISIT TELEHEALTH WHA
PSYCMED CONSULT TELEHEALTH WHA *(this is the intake visit)* - Currently inactive
PSYCMED RETURN TELEHEALTH WHA - Currently inactive
OB INTAKE TELEHEALTH VISIT WHA
WELLNESS TELEHEALTH VISIT WHA